

MOUNT SINAI HEALTH SYSTEM, INC.

Consolidated Financial Statements and Supplementary Information

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Report of Independent Auditors

Management and the Board of Trustees
Mount Sinai Health System, Inc.

Report on the Audit of the Financial Statements

Opinion

We have audited the consolidated financial statements of the Mount Sinai Health System, Inc. (the System), which comprise the consolidated statement of financial position as of December 31, 2024 and the related consolidated statements of operations, changes in net assets and cash flows for the year then ended, and the related notes (collectively referred to as the “financial statements”).

In our opinion, the accompanying financial statements present fairly, in all material respects, the consolidated financial position of the System at December 31, 2024, and the consolidated results of its operations, changes in its net assets and its cash flows for the year then ended in accordance with accounting principles generally accepted in the United States of America.

Basis for Opinion

We conducted our audit in accordance with auditing standards generally accepted in the United States of America (GAAS) and the standards applicable to financial audits contained in *Government Auditing Standards*, issued by the Comptroller General of the United States (*Government Auditing Standards*). Our responsibilities under those standards are further described in the Auditor’s Responsibilities for the Audit of the Financial Statements section of our report. We are required to be independent of the System and to meet our other ethical responsibilities in accordance with the relevant ethical requirements relating to our audit. We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our audit opinion.

Responsibilities of Management for the Financial Statements

Management is responsible for the preparation and fair presentation of the financial statements in accordance with accounting principles generally accepted in the United States of America, and for the design, implementation, and maintenance of internal control relevant to the preparation and fair presentation of financial statements that are free of material misstatement, whether due to fraud or error.



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In preparing the financial statements, management is required to evaluate whether there are conditions or events, considered in the aggregate, that raise substantial doubt about the System's ability to continue as a going concern for one year after the date that the financial statements are issued.

Auditor's Responsibilities for the Audit of the Financial Statements

Our objectives are to obtain reasonable assurance about whether the financial statements as a whole are free of material misstatement, whether due to fraud or error, and to issue an auditor's report that includes our opinion. Reasonable assurance is a high level of assurance but is not absolute assurance and therefore is not a guarantee that an audit conducted in accordance with GAAS and *Government Auditing Standards* will always detect a material misstatement when it exists. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control. Misstatements are considered material if there is a substantial likelihood that, individually or in the aggregate, they would influence the judgment made by a reasonable user based on the financial statements.

In performing an audit in accordance with GAAS and *Government Auditing Standards*, we:

- Exercise professional judgment and maintain professional skepticism throughout the audit.
- Identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, and design and perform audit procedures responsive to those risks. Such procedures include examining, on a test basis, evidence regarding the amounts and disclosures in the financial statements.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the System's internal control. Accordingly, no such opinion is expressed.
- Evaluate the appropriateness of accounting policies used and the reasonableness of significant accounting estimates made by management, as well as evaluate the overall presentation of the financial statements.
- Conclude whether, in our judgment, there are conditions or events, considered in the aggregate, that raise substantial doubt about the System's ability to continue as a going concern for a reasonable period of time.

We are required to communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit, significant audit findings, and certain internal control-related matters that we identified during the audit.



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Supplementary Information

Our audit was conducted for the purpose of forming an opinion on the financial statements as a whole. We have not performed any procedures with respect to the audited financial statements or the accompanying Financial Responsibility Supplemental Schedules Related to the U.S. Department of Education Title IV Regulations (the Financial Responsibility Supplemental Schedules) as of and for the year ended December 31, 2024 subsequent to May 30, 2025. The accompanying Financial Responsibility Supplemental Schedules as of and for the year ended December 31, 2024, as required by the U.S. Department of Education, and the accompanying Schedule of Expenditures of Federal Awards for the year ended December 31, 2024, as required by Title 2 U.S. *Code of Federal Regulations* Part 200, *Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards*, are presented for purposes of additional analysis and are not a required part of the financial statements. Such information is the responsibility of management and was derived from and relates directly to the underlying accounting and other records used to prepare the financial statements. The information has been subjected to the auditing procedures applied in the audit of the financial statements and certain additional procedures, including comparing and reconciling such information directly to the underlying accounting and other records used to prepare the financial statements or to the financial statements themselves, and other additional procedures in accordance with GAAS. In our opinion, the information is fairly stated, in all material respects, in relation to the financial statements as a whole.

Other Reporting Required by *Government Auditing Standards*

In accordance with *Government Auditing Standards*, we also have issued our report dated May 30, 2025, on our consideration of the System's internal control over financial reporting and on our tests of its compliance with certain provisions of laws, regulations, contracts, and grant agreements and other matters. The purpose of that report is solely to describe the scope of our testing of internal control over financial reporting and compliance and the results of that testing, and not to provide an opinion on the effectiveness of the System's internal control over financial reporting or on compliance. That report is an integral part of an audit performed in accordance with *Government Auditing Standards* in considering the System's internal control over financial reporting and compliance.

Ernst & Young LLP

May 30, 2025, except for our report on the Schedule of Expenditures of Federal Awards for which the date is September 30, 2025.

MOUNT SINAI HEALTH SYSTEM, INC.
Consolidated Statement of Financial Position
(In thousands)
December 31, 2024

Assets

Current assets:

Cash and cash equivalents	\$ 1,224,215
Short-term investments	263,702
Current portion of assets limited or restricted as to use	77,222
Patient accounts receivable, net	1,193,089
Current portion of professional liabilities insurance recoveries receivable	68,212
Other current assets	620,654
Total current assets	3,447,094

Assets limited or restricted as to use, net of current portion	26,559
Investments	2,499,795
Beneficial interest in self-insurance trust	553,250
Property, plant, and equipment, net	5,800,778
Right-of-use assets	832,398
Professional liabilities insurance recoveries receivable, net of current portion	189,531
Other assets	728,668
Total assets	\$ 14,078,073

Liabilities and Net Assets

Current liabilities:

Accounts payable and accrued expenses	\$ 1,067,961
Accrued salaries and related liabilities	625,343
Current portion of long-term debt	68,667
Current portion of operating lease liabilities	55,294
Current portion of professional insurance liabilities	68,212
Other current liabilities	219,161
Total current liabilities	2,104,638

Long-term debt, net of current portion	3,516,582
Operating lease liabilities, net of current portion	876,395
Professional insurance liabilities, net of current portion	189,531
Self-insurance liabilities	553,250
Accrued pension and postretirement liability	40,916
Other liabilities	1,364,008
Total liabilities	8,645,320

Net assets:

Without donor restrictions	3,903,400
With donor restrictions	1,529,353
Total net assets	5,432,753
Total liabilities and net assets	\$ 14,078,073

See accompanying notes to consolidated financial statements.

MOUNT SINAI HEALTH SYSTEM, INC.

Consolidated Statement of Operations

(In thousands)

Year Ended December 31, 2024

Operating revenue:

Patient service revenue	\$	8,649,832
Other revenue		3,204,446
Total operating revenue		<u>11,854,278</u>

Operating expenses:

Salaries and wages		5,929,343
Employee benefits		1,545,363
Supplies and other		4,002,291
Interest		116,265
Depreciation and amortization		526,420
Total operating expenses		<u>12,119,682</u>
Loss from operations		<u>(265,404)</u>

Other items:

Net change in unrealized gains and losses on investments		372,958
Other, net		(21,010)
Excess of revenue over expenses		<u>86,544</u>

Other changes:

Pension and postretirement changes other than net periodic benefit cost		1,678
Net assets released from restrictions for capital		43,842
Other, net		(5,325)
Increase in net assets without donor restrictions	\$	<u><u>126,739</u></u>

See accompanying notes to consolidated financial statements.

MOUNT SINAI HEALTH SYSTEM, INC.

Consolidated Statement of Changes in Net Assets

(In thousands)

Year Ended December 31, 2024

	<u>Without donor restrictions</u>	<u>With donor restrictions</u>	<u>Total net assets</u>
Net assets at January 1, 2024	\$ 3,776,661	\$ 1,518,892	\$ 5,295,553
Changes in net assets:			
Excess of revenue over expenses	86,544	–	86,544
Pension and postretirement changes other than net periodic benefit cost	1,678	–	1,678
Donor restricted contributions	–	125,088	125,088
Donor restricted realized and unrealized investment returns	–	36,239	36,239
Net assets released from restrictions for operations	–	(115,360)	(115,360)
Net assets released from restrictions for capital	43,842	(43,842)	–
Other, net	(5,325)	8,336	3,011
Change in net assets	<u>126,739</u>	<u>10,461</u>	<u>137,200</u>
Net assets at December 31, 2024	<u>\$ 3,903,400</u>	<u>\$ 1,529,353</u>	<u>\$ 5,432,753</u>

See accompanying notes to consolidated financial statements.

MOUNT SINAI HEALTH SYSTEM, INC.

Consolidated Statement of Cash Flows

(In thousands)

Year Ended December 31, 2024

Cash flows from operating activities:

Change in net assets	\$	137,200
Adjustments to reconcile change in net assets to net cash used in operating activities:		
Pension and postretirement changes other than net periodic benefit cost		(1,678)
Depreciation and amortization expense		526,420
Amortization of bond financing costs, premiums, and discounts		4,137
Net change in unrealized gains and losses on investments		(372,958)
Net realized gains and losses on sales of securities		(188,931)
Gain on sale of assets		(10,136)
Donor-restricted contributions and realized and unrealized investment returns		(161,327)
Changes in operating assets and liabilities:		
Patient accounts receivable, net		(169,056)
Accounts payable, accrued expenses, accrued salaries and related liabilities		157,095
Other assets and liabilities, net		(139,183)
Accrued pension and post-retirement liability		(12,054)
Net cash used in operating activities		<u>(230,471)</u>

Cash flows from investing activities:

Acquisition of property, plant, and equipment		(590,091)
Decrease in investments and assets limited or restricted as to use, net		1,367,049
Funding of self-insurance trust		(92,350)
Proceeds from sale of assets		37,893
Net cash provided by investing activities		<u>722,501</u>

Cash flows from financing activities:

Repayments of long-term debt		(85,545)
Donor-restricted contributions and restricted realized and unrealized investment returns		161,327
Net cash provided by financing activities		<u>75,782</u>

Net increase in cash and cash equivalents		567,812
Cash, cash equivalents, and restricted cash at beginning of year		840,751
Cash, cash equivalents, and restricted cash at end of year	\$	<u><u>1,408,563</u></u>
Cash and cash equivalents	\$	1,224,215
Restricted cash included in assets limited or restricted as to use and investments		184,348
Total cash, cash equivalents, and restricted cash	\$	<u><u>1,408,563</u></u>

See accompanying notes to consolidated financial statements.

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

(1) Organization and Summary of Significant Accounting Policies

(a) Organization

Mount Sinai Health System, Inc. (MSHS) and its controlled subsidiaries and other affiliated entities (collectively, the System) is an integrated health care delivery system in the New York metropolitan area, with facilities located in New York City and Nassau County, New York. MSHS is incorporated under New York State Not-for-Profit Corporation law and is a tax-exempt entity under the provisions of Section 501(a) of the Internal Revenue Code (the Code). The System provides patient care, teaching, research, and other health care related services.

MSHS is the sole member of Mount Sinai Hospitals Group, Inc. (MSHG). MSHG is a 501(c)(3) tax exempt entity incorporated under New York State Not-for-Profit Corporation law and was formed to be the sole member of the System's hospitals. The member hospitals of MSHG, all of which are 501(c)(3) tax exempt entities incorporated under New York State Not-for-Profit Corporation law, include:

- The Mount Sinai Hospital (MSH), including Mount Sinai Hospital of Queens
- Beth Israel Medical Center and Affiliates (BIMC), including Mount Sinai Phillips School of Nursing and locations at Mount Sinai Brooklyn, Mount Sinai Union Square and Mount Sinai Behavioral Health Center
- The New York Eye and Ear Infirmary and Affiliates (NYEEI)
- South Nassau Communities Hospital and Subsidiaries (SNCH)
- The St. Luke's-Roosevelt Hospital Center and Affiliates (SLR), comprised of Mount Sinai West and Mount Sinai Morningside

The principal subsidiaries and other affiliated entities of the System also include:

- The Mount Sinai Medical Center, Inc. (MSMC), a 501(c)(3) not-for-profit corporation, – maintains a pooled investment portfolio for the benefit of other System entities
- Icahn School of Medicine at Mount Sinai (ISMMS), a 501(c)(3) education corporation – teaching and research institution with faculty and community physician practice plans
- 8 East 102nd Street Manager, LLC (the Manager) – real estate holdings and related services
- MSMC Residential Realty LLC (MSMCRR) – real estate holdings and related services
- MSMC Realty Corporation (MSMCRC) – real estate holdings and related services

In October 2023, BIMC submitted a closure plan to the New York State Department of Health (NYSDOH) for its 16th Street campus, a component of its downtown Manhattan, New York campus. The closure plan was submitted due to a decreasing census, outdated infrastructure, the shift in patient care to outpatient, significant increases in labor and supply costs, and significant financial losses at the 16th Street campus. In November 2023, an amended closure plan was submitted to the NYSDOH. On April 2, 2024, the NYSDOH requested additional information, which MSHS provided in a revised closure plan submitted in May 2024, and the revised plan was approved by the NYSDOH in July 2024. On February 7, 2024, community groups commenced an action seeking to stop BIMC from closing the 16th Street campus and NYEEI from closing its facility. On February 24, 2025, the action brought by the community groups was dismissed by the New York State Supreme Court. In the days following the dismissal, the inpatient service of the 16th Street campus was closed. On February 28, 2025, the plaintiffs appealed and the appellate court agreed to hear an appeal of the community groups' action and

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

imposed a stay on closing further services. On April 8, 2025 the appeal was dismissed and on April 9, 2025 the 16th Street campus was closed. BIMC intends to continue operating Mount Sinai Brooklyn, its ambulatory care locations, the Mount Sinai Phillips School of Nursing and the Mount Sinai Behavioral Health Center.

Change in Reporting Entity

During the year ended December 31, 2024, the System transitioned from reporting standalone financial statements for MSH, BIMC, NYEEI, SNCH, SLR, ISMMS, MSMC, MSMCRRRC, MSMCRC, and the Manager to consolidated financial statements of MSHS and its controlled subsidiaries and affiliates. Other controlled entities included in the accompanying consolidated financial statements did not previously issue standalone financial statements; however, those entities are not significant to the System's consolidated financial position or its operating results. The change in reporting entity was made to provide more relevant and comprehensive financial information to stakeholders, including a more complete representation of the consolidated financial position and performance of the System and its subsidiaries. The accompanying consolidated financial statements of the System reflect the accounting for prior acquisitions of BIMC, NYEEI, and SLR (each in 2013) and SNCH (in 2018) in accordance with Accounting Standards Codification 805, *Business Combinations*.

Although the System and the other entities described above have been consolidated for financial statement reporting purposes under applicable accounting requirements, there may be limitations on the use of one entity's funds by another member of the consolidated group resulting from the charitable nature of some of the entities or other factors.

(b) Principles of Consolidation

The accompanying consolidated financial statements have been prepared on the accrual basis of accounting and include all subsidiaries and other affiliated entities for which operating control is exercised by the System. Intercompany balances and transactions are eliminated in consolidation.

(c) Use of Estimates

The preparation of financial statements in accordance with accounting principles generally accepted in the United States of America (GAAP) requires management to make estimates and assumptions that affect the reported amounts of assets and liabilities and the disclosure of contingent assets and liabilities at the date of the financial statements. Estimates also affect the reported amounts of revenue and expenses during the reporting period. In the accompanying consolidated financial statements, estimates principally relate to the valuation of patient accounts receivable, net, amounts due from and to third-party payors, estimated professional and self-insurance liabilities and related insurance recoveries receivable, benefit plan assumptions, and the carrying value of alternative investments. Actual results could differ from those estimates.

(d) Cash and Cash Equivalents

The System considers highly liquid financial instruments purchased with a maturity of three months or less to be cash equivalents. The System has balances in financial institutions that exceed Federal depository insurance limits. It is the System's policy to monitor the financial strength of these institutions. Management does not believe the credit risk related to these deposits to be significant. The System does

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

not hold any money market funds with significant liquidity restrictions that would require the funds to be excluded from cash equivalents.

Amounts within restricted cash include cash held within investments and assets limited or restricted as to use and represent funds set aside within investments based on management's policy or contractual arrangements.

(e) *Patient Accounts Receivable, Net and Patient Service Revenue*

Patient accounts receivable, net and patient service revenue result from the health care services provided by the System and are reported at the amount that reflects the consideration to which the System expects to be entitled in exchange for providing patient care. These amounts are due from patients, third-party payors (including health insurers and government programs), and others and include variable consideration in determination of the transaction price.

(f) *Investments and Investment Return*

Investments consist of cash and cash equivalents, U.S. government obligations, mutual funds, equity securities, and interests in alternative investments. All investments are classified as trading investments and, excluding interests in alternative investments, are carried at fair value as described in Note 4.

Alternative investments (nontraditional, not readily marketable securities) are measured based upon net asset values (NAV) derived from the application of the equity method of accounting. Financial information used to evaluate alternative investments is provided by the respective investment manager or general partner and includes fair value valuations (quoted market prices and values determined through other means) of underlying securities and other financial instruments held by the investee and estimates that require varying degrees of judgment. The financial statements of the investee companies are audited annually by independent auditors, although the timing for reporting the results of such audits for certain investees does not coincide with the System's annual financial statement reporting. There is uncertainty in determining values of alternative investments arising from factors such as lack of active markets (primary and secondary), lack of transparency into underlying holdings, time lags associated with reporting by the investee companies and the subjective evaluation of liquidity restrictions. As a result, the estimated values might differ from the values that would have been used had a ready market for the alternative investment interests existed and there is at least a reasonable possibility that estimates will change.

Alternative investments may consist of equity, debt, and derivatives both within and outside the U.S. in multi-strategy hedge funds, event-driven strategies, global investment mandates, distressed securities, and private funds. Alternative investment interests generally are structured such that the System holds a limited partnership interest or an interest in an investment management company. Alternative investments often have liquidity restrictions under which the pooled investment capital may be divested only at specified times. Liquidity restrictions may apply to all or portions of a particular invested amount.

A substantial portion of the System's investments are pooled for management purposes amongst the System entities (pool participants). MSMC has custody of the pooled investments. For purposes of the consolidating financial statements, interests in pooled investments are reported by pool participants using unitized net asset value based on their respective share of the investment pool and are reported as investments in the statement of financial position. Investment income from the investment pool is

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

allocated to investment pool participants based on their pro rata share and is calculated using the market value unit method. An annual spending rate for donor related pooled funds is approved by the Board of Trustees annually (see Note 12). Realized gains and losses from the sale of securities are computed using the average cost method.

The investment pool's financial risk is limited to the carrying amount reported for each investee, in addition to any unfunded capital commitment.

In the absence of donor restrictions, investment income, including realized gains and losses, is reflected in the accompanying consolidated statement of operations within other revenue. The net change in unrealized gains and losses on investments and the change in value of alternative investments, arising from pooled investments, are reported as other items.

(g) Inventories

Inventories (included in other current assets) are stated at the lower of cost or net realizable value.

(h) Assets Limited or Restricted as to Use

Assets so classified represent assets whose use is restricted by donors, bond indenture agreements, internally designated for specific purposes, or under terms of other agreements. These assets consist primarily of U.S. Treasury obligations held in the bond trustees' accounts and money market funds.

(i) Property, Plant, and Equipment, Net

Property, plant, and equipment are stated at cost. Those acquired through acquisition of businesses, gifts and bequests are stated at appraised or fair value established at the date of the transaction. The carrying amounts of assets and the related accumulated depreciation and amortization are removed from the accounts when such assets are disposed of and any resulting gain or loss is included in operating results.

Annual provisions for depreciation are made based upon the straight-line method using a half-year convention over the estimated useful lives of the assets, ranging from 3 to 40 years.

(j) Impairment of Long-Lived Assets

Long-lived assets are reviewed for impairment whenever events or changes in circumstances indicate that the carrying amount of assets, or a related group of assets, may not be recoverable. Long-lived assets are grouped with other assets and liabilities at the lowest level for which identifiable cash flows are largely independent of the cash flows of other assets and liabilities. The recoverability of assets to be held and used is measured by a comparison of the carrying amount of an asset to the estimated undiscounted future cash flows expected to be generated by the underlying tangible assets. When the carrying value of an asset exceeds its estimated recoverability, an asset impairment charge is recognized for the difference between the fair value and carrying value of the asset.

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

(k) Leases

The System determines if an arrangement is a lease at inception. Operating leases are included in right-of-use (ROU) assets and operating lease liabilities in the accompanying consolidated statement of financial position. ROU assets and liabilities are recognized based on the present value of the future minimum lease payments over the lease term using a risk-free discount rate determined using a period comparable with that of the lease term. The ROU asset also includes any prepaid rent while excluding lease incentives and initial direct costs incurred. Lease expense for operating minimum lease payments is recognized on a straight-line basis over the full lease term.

Finance leases are included in property, plant, and equipment, net and long-term debt in the accompanying consolidated statement of financial position. Finance lease assets and liabilities are recognized based on the present value of the future minimum lease payments over the lease term using the explicit interest rate, when available. If an explicit interest rate is not available, the System applies a risk-free discount rate determined using a period comparable with that of the lease term. Finance lease assets are amortized on a straight-line basis over the lease term with such amortization presented in depreciation and amortization in the accompanying consolidated statement of operations. Interest on finance leases is calculated using the effective interest method and presented in interest expense in the accompanying consolidated statement of operations.

(l) Benefits and Insurance

The System is self-insured for medical, dental, hospitalization, workers' compensation and prescription drug benefits provided to employees and has a stop-loss arrangement to limit exposure for these self-insured benefits. These costs are included in employee benefits expense in the accompanying statement of operations.

(m) Net Assets Without Donor Restrictions

Net assets that are not subject to donor-imposed restrictions may be expended for any purpose in performing the primary objectives of the System. These net assets may be used at the discretion of the System's management and the Board of Trustees.

(n) Net Assets With Donor Restrictions

Net assets with donor restrictions are those whose use by the System has been limited by donors to a specific time period or purpose. Some donor restrictions are temporary in nature; those restrictions will be met by actions of the System or by the passage of time. Other donor restrictions are perpetual in nature, whereby the donor has stipulated the funds be maintained in perpetuity.

Donor restricted contributions are reported as increases in net assets with donor restrictions. When a restriction expires, net assets are reclassified as net assets released from restrictions for operations or as net assets released from restrictions for capital purposes in the accompanying consolidated statements of operations and changes in net assets.

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

(o) Other Revenue

Other revenue includes income from federal and non-federal grants and contracts, pharmacy sales, royalty revenue, investment income, affiliation revenue from New York City Health and Hospitals Corporation, tuition and fee revenue, contribution revenue, net assets released from restrictions for operations, and other miscellaneous revenue items.

(p) Grants and Contracts Revenue

Grants and contracts revenue represents reimbursement of costs incurred in direct support of federally and non-federally funded sponsored research and clinical trials and other sponsored activities of the System. The System does not record conditional grants or contracts revenue until performance of the conditions or when the barriers on which they depend, as stated in the contracts, have been substantially met. Exchange transactions are recognized as revenue at the amount that reflects the consideration to which the System expects to be entitled in exchange for performance under the contract.

(q) Contributions and Unconditional Promises to Give

Contributions, including unconditional promises to give cash and other assets (pledges), are reported at fair value on the date received. The gifts are reported as net assets with donor restrictions if they are received with donor-imposed stipulations that limit the use of the donated assets. Donor restricted contributions, whose restrictions and conditions are met within the same year as the contributions are received, are recorded as other revenue.

Conditional contributions are recognized as revenue when the conditions or barriers on which they depend have been substantially met. Pledges are recorded net of estimated uncollectible amounts and promises to give that are due in future years are discounted to present value.

(r) Performance Indicator

The consolidated statement of operations includes the excess of revenue over expenses as the performance indicator. Changes in net assets without donor restrictions which are excluded from the excess of revenue over expenses include changes in pension and postretirement obligations other than net periodic benefit cost, net assets released from restrictions for capital, and other items.

The System differentiates its operating activities through the use of the loss from operations as an intermediate measure of operations. For the purposes of display, items which management does not consider components of the System's operating activities are excluded from this measure and reported as other items in the consolidated statement of operations.

(s) Tax Status

Most entities within the System are not-for-profit corporations as described in Section 501(c)(3) of the Code and are generally exempt from federal income taxes pursuant to Section 501(a) of the Code.

Certain entities of the System are for-profit entities. Provisions for income taxes and deferred taxes, where applicable, have been made for the taxable entities of the System. Taxable operations and the potential for income taxes from these entities and unrelated business income activities of the tax-exempt entities are not significant to the accompanying consolidated financial statements.

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

The System evaluates tax positions taken or expected to be taken in the course of preparing the System's tax returns to determine whether the tax positions are "more likely than not" of being sustained by the applicable tax authority based upon the technical merits of the position. The System recognizes the effect of tax positions only if they are more likely than not of being sustained. This evaluation had no impact on the operations of the System as of and for the year ended December 31, 2024.

(t) Related Organizations

Transactions among MSHS and its related organizations relate principally to the sharing of certain services, facilities, equipment, personnel and other shared services. Such transactions are allocated among the entities based on the approximate cost to provide the services and an agreed upon method which reflects the approximate level of usage by each entity. In addition, the MSHG hospitals purchase professional services from ISMMS for the clinical care of its patients, teaching and supervision of its residents, the performance of certain administrative functions, and various strategic initiatives (CARTS). As noted above, intercompany balances and transactions are eliminated in consolidation.

For the year ended December 31, 2024, MSH wrote-off approximately \$139.6 million for amounts considered uncollectable from SLR, which is included as a transfer to SLR in MSH's consolidated statement of operations.

Also, MSH transfers to affiliates of \$103.6 million for the year ended December 31, 2024 consisted of \$95.2 million for MSH's funding of ISMMS' community practice plan deficits and \$8.4 million in transfers to MSMCRRC.

MSH guarantees several leases entered into by BIMC and ISMMS with total rent payments remaining at December 31, 2024 of approximately \$466.8 million and \$171.0 million, respectively, and remaining lease terms varying from 18-30 years. MSH guarantees payment if BIMC were unable to make payments under an event of default. MSH guarantees payment up to a maximum of \$45.0 million if ISMMS were unable to make payments under an event of default.

(u) COVID-19 Update

The System applied for reimbursement for qualifying expenses under the Federal Emergency Management Agency (FEMA) Disaster Relief Fund. During 2024, the System recognized approximately \$183.6 million of obligated reimbursement grants from FEMA. The FEMA grants are reported within other revenue as they are for reimbursement of operating costs. The receivable related to FEMA obligated reimbursement payments was approximately \$183.6 million at December 31, 2024 and is recorded within other current assets in the accompanying consolidated statement of financial position.

(2) Patient Accounts Receivable, Net and Patient Service Revenue

Patient service revenue is reported at the amount that reflects the consideration for which the System expects to be entitled in exchange for providing patient care. The System uses a portfolio approach to account for categories of patient contracts as collective groups rather than recognizing revenue on an individual contract basis. The portfolio consists of major payor classes for inpatient revenue and outpatient revenue. Based on historical collection trends and other analyses, the System believes that revenue recognized by utilizing the portfolio approach approximates the revenue that would have been recognized if an individual contract approach were used.

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Full-time faculty members and non-faculty physicians of MSH, BIMC, SLR, SNCH, and NYEEI may participate in ISMMS's faculty practice plan or community practice plan. All of these activities are included within patient service revenue within the accompanying consolidated statement of operations. Plan participants are authorized to conduct a private practice and engage in professional consultation in accordance with established institutional guidelines. Professional service fee receipts are recorded and deposited in private practice funds established by ISMMS for each individual participant or group practice when received by ISMMS. Portions of these receipts are used to support ISMMS activities and to reimburse ISMMS for indirect costs incurred in supporting plan activities. The remaining amounts, after direct plan expenses, provide participant salary supplements and support ISMMS departmental activities.

The System's initial estimate of the transaction price for services provided to patients is determined by reducing the total standard charges related to the patient services provided by various elements of variable consideration, including contractual adjustments, discounts, implicit price concessions, and other reductions to the System's standard charges and include provisions for variable consideration for retroactive revenue adjustments, including adjustments due to the settlement of ongoing and future audits, reviews, and investigations. The System determines the transaction price associated with services provided to patients who have third-party payor coverage on the basis of contractual or formula-driven rates for the services rendered (see description of third-party payor payment programs below). The estimates for contractual allowances and discounts are based on contractual agreements, the System's discount policies and historical experience. For uninsured and under-insured patients who do not qualify for charity care, the System determines the transaction price associated with services rendered on the basis of charges reduced by implicit price concessions. Implicit price concessions included in the estimate of the transaction price are based on the System's historical collection experience for applicable patient portfolios.

Generally, the System bills patients and third-party payors after the services are performed and the patient is discharged. Patient service revenue is recognized as performance obligations are satisfied. Performance obligations are determined based on the nature of the services provided by the System. Patient service revenue for performance obligations satisfied over time is recognized based on actual charges incurred in relation to total charges. The System believes that this method provides a reasonable depiction of the transfer of services over the term of the performance obligation based on the services needed to satisfy the obligation. Generally, performance obligations satisfied over time relate to patients receiving inpatient acute care services or patients receiving services in the System's outpatient settings. The System measures the performance obligation from admission into the System or the commencement of an outpatient service to the point when it is no longer required to provide services to that patient, which is generally at the time of discharge or the completion of the outpatient visit.

Substantially all of the System's performance obligations relate to contracts with a duration of less than one year. Unsatisfied or partially unsatisfied performance obligations primarily relate to inpatient acute care services at the end of the reporting period for patients who remain admitted at that time (in-house patients). The performance obligations for in-house patients are generally completed when the patients are discharged, which for the majority of the System's in-house patients occurs within days or weeks after the end of the reporting period.

Subsequent changes to the estimate of the transaction price (determined on a portfolio basis when applicable) are generally recorded as adjustments to patient service revenue in the period of the change. For the year ended December 31, 2024, changes in the System's estimates of implicit price concessions, discounts, contractual adjustments or other reductions to expected payments for performance obligations

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satisfied in prior years were not significant. Portfolio collection estimates are updated based on collection trends. Subsequent changes that are determined to be the result of an adverse change in the patient's ability to pay based on current or future estimated credit losses (determined on a portfolio basis when applicable) are recorded as bad debt expense. Bad debt expense for the year ended December 31, 2024 was not significant.

The System has determined that the nature, amount, timing and uncertainty of revenue and cash flows are affected by the following factors: payors, lines of business and timing of when revenue is recognized. Tables providing details of these factors are presented below.

Patient service revenue disaggregated by payor comprises the following for the year ended December 31, 2024 (in thousands):

Medicare & Medicare managed care	\$	2,894,441
Medicaid & Medicaid managed care		1,372,777
Blue Cross		1,418,262
Managed care		1,897,665
Commercial and other		844,158
Self-pay		222,529
Total	\$	<u>8,649,832</u>

Deductibles, copayments and coinsurance under third-party payment programs which are the patient's responsibility are included within the respective primary payor category above.

Patient service revenue disaggregated by lines of service comprises the following for the year ended December 31, 2024 (in thousands):

Inpatient services	\$	4,305,373
Outpatient services		4,344,459
Total	\$	<u>8,649,832</u>

Patient accounts receivable, net is comprised of the following components at December 31, 2024 (in thousands):

Patient receivables	\$	1,124,363
Contract assets		68,726
Total	\$	<u>1,193,089</u>

Contract assets are related to in-house patients who were provided services during the reporting period but were not discharged as of the reporting date and for which the System may not have the right to bill.

MOUNT SINAI HEALTH SYSTEM, INC.

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(a) Third-Party Payment Programs

The System has agreements with third-party payors that provide for payment for services rendered at amounts different from its established rates. A summary of the payment arrangements with major third-party payors follows:

Medicare: Hospitals are paid for most Medicare patient services under national prospective payment systems and other methodologies of the Medicare program for certain other services. Federal regulations provide for adjustments to current and prior years' payment rates, based on industry-wide and Hospital-specific data.

Non-Medicare: In New York State, hospitals and all non-Medicare payors (including Medicare and Medicaid managed care plans), except Medicaid, workers' compensation and no-fault insurance programs, negotiate hospitals' payment rates. Payments to hospitals for Medicaid, workers' compensation and no-fault inpatient services are based on a statewide prospective payment system, with retroactive and/or prospective adjustments. Outpatient services also are paid based on a statewide prospective system. Medicaid rate methodologies are subject to approval at the Federal level by the Centers for Medicare and Medicaid Services (CMS), which may routinely request information about such methodologies prior to approval. Revenue related to specific rate components that have not been approved by CMS is not recognized until the System is reasonably assured that such amounts are realizable. Adjustments to the current and prior years' payment rates for those payors will continue to be made in future years.

Other Third-Party Payors: The System also has entered into payment agreements with certain commercial insurance carriers and health maintenance organizations. The basis for payment to the System under these agreements includes prospectively determined rates per discharge or days of hospitalization and discounts from established charges.

Medicare cost reports, which serve as the basis for final settlement with the Medicare program, have been audited by the Medicare fiscal intermediary and settled through 2018 for MSH and through 2018 for the other System hospitals, although revisions to final settlements or other retroactive changes could be made in subsequent years. Certain cost report years and various issues remain open for audit and settlement, as are numerous issues related to the New York State Medicaid program for prior years. As a result, there is at least a reasonable possibility that recorded estimates will change by a material amount when open years are settled, audits are completed and additional information is obtained.

Settlements with third-party payors (see description of third-party payor payment programs above) for cost report filings and retroactive adjustments due to ongoing and future audits, reviews or investigations are considered variable consideration and are included in the determination of the estimated transaction price for providing patient care. These settlements are estimated based on the terms of the payment agreement with the payor, correspondence from the payor and the System's historical settlement activity (for example, cost report final settlements or repayments related to recovery audits), including an assessment to ensure that it is probable that a significant reversal in the amount of cumulative revenue recognized will not occur when the uncertainty associated with the retroactive adjustment is subsequently resolved. Such estimates are determined through either a probability-weighted estimate or an estimate of the most likely amount, depending on the circumstances related to a given estimated settlement item. Estimated settlements are adjusted in future periods as adjustments become known (that is, new

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information becomes available), or as years are settled or are no longer subject to such audits, reviews, and investigations. For the year ended December 31, 2024, the net effect of the System's revisions to prior year estimates was an increase to patient service revenue of approximately \$44.4 million.

Laws and regulations concerning government programs, including Medicare and Medicaid, are complex and subject to varying interpretation. As a result of investigations by governmental agencies, various health care organizations have received requests for information and notices regarding alleged noncompliance with those laws and regulations, which, in some instances, have resulted in organizations entering into significant settlement agreements. Compliance with such laws and regulations may also be subject to future government review and interpretation as well as significant regulatory action, including fines, penalties, and potential exclusion from the related programs. There can be no assurance that regulatory authorities will not challenge the System's compliance with these laws and regulations, and it is not possible to determine the impact (if any) such claims or penalties would have upon the System. The System is not aware of any allegations of non-compliance that could have a material adverse effect on the accompanying consolidated financial statements and believes that it is in compliance in all material respects with all applicable laws and regulations. In addition, certain contracts the System has with commercial payors also provide for retroactive audit and review of claims.

There are various proposals at the federal and state levels that could, among other things, significantly change payment rates or modify payment methods. The ultimate outcome of these proposals and other market changes, including the potential effects of or revisions to health care reform that has been or will be enacted by the federal and state governments, cannot be determined presently. Future changes in the Medicare and Medicaid programs and any reduction of funding could have an adverse impact on the System. Additionally, certain payors' payment rates for various years have been appealed by the System. If the appeals are successful, additional income applicable to those years could be realized.

The System grants credit without collateral to its patients, most of whom are insured under third-party payor agreements. Significant concentrations of patient accounts receivable, net at December 31, 2024, are as follows:

Medicare & Medicare managed care	15%
Medicaid & Medicaid managed care	19
Blue Cross	14
Managed care and other	49
Self-pay	3
Total	<u>100%</u>

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(b) Uncompensated Care

For patients who are deemed eligible for charity care and patients who apply and qualify for financial aid under the System's financial aid policies, care given but not paid for is classified as charity care. For the year ended December 31, 2024, the estimated cost of charity care was approximately \$79.0 million. The estimated cost of charity care includes the direct and indirect cost of providing charity care services and is estimated by utilizing a ratio of cost to standard charges applied to the uncompensated charges associated with providing charity care.

(c) Vital Access Provider Safety Net Program and Medicaid Enhanced Rates

In September 2015, MSHG entered into a Vital Access Provider/Safety Net Program Temporary Medicaid Rate Adjustment Agreement (the TRA Agreement) with the NYSDOH to participate in the Vital Access Provider/Safety Net Program (VAP). MSHG was awarded approximately \$81.4 million in VAP funding over three years. The NYSDOH had also agreed to provide MSH, BIMC, and NYEEI with a temporary Medicaid rate enhancement for three years. The enhanced Medicaid rates were paid to MSH, BIMC, and NYEEI directly by the Medicaid program or Medicaid managed care payors as patient services were rendered. As an express condition of receipt of the Medicaid rate enhancement, MSHG committed to complete a full asset merger (FAM) of MSH, BIMC, SLR and NYEEI.

On February 1, 2022, MSHG entered into a Medicaid Liability Repayment Agreement (the MLRA Agreement) with the NYSDOH, whereby MSH, BIMC, and NYEEI collectively agreed to implement a schedule for phased recoupment of the Medicaid temporary rate adjustment provided to MSHG pursuant to the TRA Agreement as MSHG did not consummate the FAM. MSHG and the NYSDOH have agreed to the repayment of approximately \$96.9 million (the TRA Amount) to be paid in ten equal annual installments that commenced on March 15, 2022. At December 31, 2024, approximately \$9.7 million is included in other current liabilities with the remaining obligation of \$52.2 million included within other liabilities at the present value. The allocation of the total recoupment amount to MSHG member hospitals was based on revenue recognized as a result of the Medicaid rate enhancement for each member hospital, respectively.

Upon repayment of the TRA Amount, MSHG will have satisfied all obligations under the MLRA Agreement and shall have no obligation to undertake the FAM. In the event that MSHG effectuates the FAM prior to January 1, 2031, there will be no further obligated recoupment payments. The likelihood that a FAM will be completed prior to January 1, 2031 is considered to be remote and, as such, MSH, BIMC, and NYEEI have recognized liabilities for the present value of the estimated recoupment payments. No repayment amounts have been allocated to SLR and SNCH as they were not a party to the original TRA Agreement.

MOUNT SINAI HEALTH SYSTEM, INC.

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(3) Investments and Assets Limited or Restricted as to Use

Investments, as well as cash and cash equivalents, are maintained at December 31, 2024 as follows (in thousands):

Cash and cash equivalents	\$ 1,224,215
Pooled investments	2,127,146
Non-pooled investments	636,351
Assets limited or restricted as to use	103,781
Total	\$ <u>4,091,493</u>

The following table summarizes the composition of investments at December 31, 2024 (in thousands):

	<u>Pooled investments</u>	<u>Non-pooled investments</u>
Cash and cash equivalents	\$ 107,831	\$ 76,517
Mutual funds	7,039	164,740
U.S. equities	164,774	221,193
Other investments	—	173,901
Alternative investments:		
Hedge funds:		
Long-only equity ^(a)	417,847	—
Long/short equity ^(b)	105,204	—
Long/short credit ^(c)	44,388	—
Hedged/market neutral ^(d)	335,373	—
Private investments:		
Equity ^(e)	672,820	—
Credit/distressed ^(f)	144,150	—
Real assets ^(g)	127,720	—
Total	\$ <u>2,127,146</u>	\$ <u>636,351</u>

(a) Investments, consisting of publicly traded equity holdings with long positions.

(b) Investments, consisting primarily of publicly traded equity holdings with both long and short positions.

(c) Investments, consisting primarily of publicly traded credit holdings with both long and short positions.

(d) Investments with a balanced mix of asset exposures and strategies. Underlying exposures primarily include publicly traded equity and credit positions in relative value and various arbitrage strategies. Investments may reflect a tilt towards equity or credit with hedging.

(e) Investments targeting buyout, growth equity, and venture opportunities that require time to reach realization.

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(f) Investments in structured credit, claims, distressed positions of either a minority or controlling interest that require time to reach realization.

(g) Real estate, natural resources, and asset backed royalty investments that require time to reach realization.

The total return on investments recorded in the accompanying consolidated statement of operations comprises the following for the year ended December 31, 2024 (in thousands):

Interest and dividend and other income	\$	26,162
Net realized gains and losses on sales of securities		188,931
Fees and other expenses		(5,312)
Subtotal (included in other revenue in the consolidated statement of operations)		<u>209,781</u>
Net change in unrealized gains and losses on investments		<u>372,958</u>
Total	\$	<u>582,739</u>

Assets limited or restricted as to use, consist mainly of cash and cash equivalents and fixed income securities consist of the following at December 31, 2024 (in thousands):

Donor-restricted funds, net	\$	939
Funds held by bond trustees under bond indenture agreements		61,605
Internally designated funds for specific use		32,837
Other limited use funds		<u>8,400</u>
	\$	<u>103,781</u>

Assets held under bond indenture agreements are maintained for the following purposes at December 31, 2024 (in thousands):

Capital project funds	\$	42,307
Debt service fund		<u>19,298</u>
	\$	<u>61,605</u>

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

(4) Fair Values of Financial Instruments

For assets and liabilities requiring fair value measurement, the System measures fair value based on the price that would be received to sell an asset or paid to transfer a liability in an orderly transaction between market participants at the measurement date. The System follows a fair value hierarchy based upon the transparency of inputs to the valuation of an asset or liability as of the measurement date. The three levels are defined as follows:

Level 1 – Quoted prices (unadjusted) in active markets that are accessible at the measurement date for identical assets or liabilities. The fair value hierarchy gives the highest priority to Level 1 inputs.

Level 2 – Observable inputs that are based on inputs not quoted in active markets but corroborated by market data.

Level 3 – Unobservable inputs are used when little or no market data is available. The fair value hierarchy gives the lowest priority to Level 3 inputs.

A financial instrument's categorization within the valuation hierarchy is based upon the lowest level of input that is significant to the fair value measurement.

In determining fair value, the System uses valuation techniques that maximize the use of observable inputs and minimize the use of unobservable inputs to the extent possible, as well as considers nonperformance risk in its assessment of fair value. Investments valued based upon NAV are not subject to the valuation hierarchy.

Financial assets carried at fair value by the System as of December 31, 2024 are classified in the table below in one of the three categories described above (in thousands):

	<u>Level 1</u>	<u>Level 2</u>	<u>Level 3</u>	<u>Total</u>
Cash and cash equivalents and money market funds	\$ 1,408,563	\$ –	\$ –	\$ 1,408,563
Equity securities	385,967	–	–	385,967
Mutual funds	269,967	–	–	269,967
Other investments	55,683	10,252	–	65,935
Total	<u>\$ 2,120,180</u>	<u>\$ 10,252</u>	<u>\$ –</u>	<u>\$ 2,130,432</u>

The table does not include investments measured at NAV as a practical expedient of \$1.8 billion and other investments that are not carried at fair value of approximately \$113.6 million at December 31, 2024.

MOUNT SINAI HEALTH SYSTEM, INC.

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The following is a summary of investments (by major category) with restrictions to redeem the investments at the measurement date, any unfunded capital commitments, and investment strategies of the investees as of December 31, 2024:

Description of investment	Carrying value	Unfunded commitments	Redemption frequency	Redemption notice period	Funds availability
Hedge funds:					
Long-only equity	\$ 311,842	\$ —	Quarterly/annual	45-90 days	14-30 days
Long-only equity	106,005	—	Rolling 3 years	90 days	14 days
Long/short equity	105,204	—	Quarterly	60 to 90 days	30 days
Long/short credit	44,388	—	Quarterly	90 days	120 days
Hedged/market neutral	205,771	—	Quarterly	60 to 65 days	30 days
Hedged/market neutral	129,602	—	Rolling 2 years	90 days	90 days
Private investments:					
Equity	672,820	218,453	*	*	*
Credit/distressed	144,150	26,538	*	*	*
Real assets	127,720	5,248	*	*	*
Total	<u>\$ 1,847,502</u>	<u>\$ 250,239</u>			

* MSMC liquidity restrictions range from several months to greater than 10 years for certain private investments. Liquidity restrictions may apply to all or a portion of a particular invested amount.

(5) Liquidity and Available Resources

Financial assets available for general expenditure within one year of the consolidated statement of financial position date consist of the following at December 31, 2024 (in thousands):

Financial assets:

Cash and cash equivalents	\$ 1,224,215
Short-term investments	263,702
Patient accounts receivables, net	1,193,089
Rent receivables	7,686
Assets limited or restricted as to use - internally designated	32,837
Investments	<u>1,066,276</u>
Total financial assets available to meet cash needs for general expenditures within one year	<u>\$ 3,787,805</u>

The System has certain internally designated assets limited or restricted as to use which are available for general expenditure within one year in the normal course of operations. Accordingly, these assets have been included in the information above. The System has other assets limited or restricted to use for donor-restricted purposes and debt service. These assets limited or restricted as to use, which are more fully described in Note 3, are not available for general expenditure within the next year and are not reflected in the amounts above. Certain entities maintain lines of credit which are disclosed in Note 7, which provide an additional source of liquidity and are not reflected in the amounts above.

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(6) Property, Plant, and Equipment

A summary of property, plant, and equipment, net is as follows at December 31, 2024 (in thousands):

Land and improvements	\$	865,425
Leasehold		220,308
Buildings and improvements		6,496,041
Fixed equipment		1,372,272
Movable equipment		3,804,658
		12,758,704
Less: accumulated depreciation and amortization		(7,457,866)
Add: construction in progress		499,940
Total	\$	<u>5,800,778</u>

Assets under finance leases approximate \$910.9 million at December 31, 2024, and are included in buildings and improvements and movable equipment (\$754.0 million at December 31, 2024, net of accumulated amortization).

The System capitalizes costs incurred in connection with the development of internal use software or purchased software modified for internal use. In 2024, approximately \$83.0 million was capitalized.

In 2024, the System wrote off approximately \$11.2 million of fully depreciated assets that were no longer in use.

Substantially all property, plant, and equipment have been pledged as collateral under various debt agreements (see Note 7).

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Notes to the Consolidated Financial Statements Year Ended December 31, 2024

(7) Long-Term Debt

Each entity as noted below is the primary obligor for its long-term debt, although certain debt instruments have been guaranteed by other System entities. A summary of long-term debt is as follows at December 31, 2024 (in thousands):

MSH Series 2013 bonds ^(a)	\$	78,400
MSH Series 2017 bonds ^(b)		382,000
MSH Series 2019 bonds ^(c)		500,000
MSH Series 2020 bonds ^(d)		400,000
ISMMS Series 2015A bonds ^(e)		410,580
SNCH Series 2018 bonds ^(f)		90,000
SNCH Series 2022 bonds ^(g)		81,322
102nd Street Series 2010A bonds ^(h)		135,690
102nd Street Series 2010B bonds ^(h)		5,310
MSH note payable ⁽ⁱ⁾		87,416
MSH loan ^(j)		9,516
59 th Street mortgage loan ^(k)		19,025
A&J and MSMCRRC mortgage loan ^(l)		218,263
SLR mortgage loan ^(m)		102,370
SNCH mortgage loan ⁽ⁿ⁾		5,917
Finance leases with various interest rates (Note 8)		<u>1,053,524</u>
Total long-term debt	\$	3,579,333
Add: unamortized bond premium		21,425
Less: unamortized bond discount		(450)
Less: deferred financing costs, net		(15,059)
Less: current portion		<u>(68,667)</u>
Long-term portion	\$	<u><u>3,516,582</u></u>

(a) In December 2013, Build NYC Resource Corporation issued \$112.0 million of tax-exempt bonds (MSH Series 2013 bonds) on behalf of MSH. In June 2020, MSH refinanced the bonds and the term was extended by twelve years to 2032 at an interest rate of 1.88%. The bonds were privately placed with TD Bank.

(b) In December 2017, MSH issued \$382.0 million of taxable bonds (MSH Series 2017 bonds) with interest rates ranging from 3.83% to 3.98%. The bonds are structured with interest only payments until 2031 and two bullet maturities: one in 2035 and the second in 2048. There are mandatory annual sinking payments beginning in 2031.

(c) In November 2019, MSH issued \$500.0 million of taxable bonds (MSH Series 2019 bonds) with an interest rate of 3.74%. The bonds are structured with interest only payments through 2044 and one bullet maturity in 2049. Annual sinking fund payments begin in 2045.

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- (d) In September 2020, MSH issued \$400.0 million of taxable bonds (MSH Series 2020 bonds) with an interest rate of 3.39%. The MSH Series 2020 bonds are structured with interest only payments through 2049 and one bullet maturity in 2050.
- (e) In August 2015, the Dormitory Authority of the State of New York issued on behalf of ISMMS \$512.4 million of tax-exempt bonds (ISMMS Series 2015A bonds) with interest rates ranging from 2.50% to 5.00%. The principal on the bonds is payable on July 1 of every year through July 1, 2045. Interest payments are due on January 1 and July 1 of each year.
- (f) In February 2018, SNCH issued \$90.0 million of Series 2018 taxable bonds (SNCH Series 2018 bonds). The principal of \$90.0 million is due on the maturity date of August 1, 2048. Interest payments are due on February 1 and August 1 of each year, at a fixed rate of 4.65%.
- (g) In October 2022, SNCH issued \$90.0 million of Series 2022 tax-exempt bonds (SNCH Series 2022 bonds). The principal on the bonds is payable on July 1 of every year through July 1, 2047. Interest payments are due on January 1 and July 1 of each year, at a fixed rate of 2.62%.
- (h) In September 2010 and October 2011, the New York Housing Finance Agency, issued \$135.7 million of variable rate 2010 Series A tax-exempt bonds (102nd Street Series 2010A bonds) and \$8.0 million of variable rate 2010 Series B taxable bonds (102nd Street Series 2010B bonds) on behalf of the Manager. Principal and interest for the Series 2010A and Series 2010B bonds are due monthly through May 1, 2044 and the interest rates at December 31, 2024 were 3.6% and 4.5%, respectively. The Manager maintains an irrevocable direct pay letter of credit for \$143.7 million to support the 102nd Street Series 2010A and 2010B bonds. The letter of credit expires on September 27, 2027. MSH is the guarantor of the letter of credit.
- (i) In August 2014, MSH purchased a leasehold condominium interest financed through the issuance of a promissory note payable (MSH note payable) with a principal amount of \$110.1 million, with interest at a rate of 8%, and payments beginning in June 2015 and ending in March 2046.
- (j) In January 2022, MSH entered into a fifty month loan (MSH loan) with GE Healthcare Financial Services with installment payments for the acquisition of certain equipment and related costs.
- (k) In April 2008, 425 West 59th Street Condominium, LLC, an affiliate of SLR, entered into a \$44.5 million mortgage (59th Street mortgage) for commercial condominiums with a twenty-year term and an interest rate of 6.22% per annum. Payment of principal and interest are due monthly and the mortgage is collateralized by the condominiums.
- (l) In November 2018, Augustus & James Corporation (A&J), an affiliate of SLR, and MSMCRRC refinanced their then outstanding mortgage loans with a ten-year mortgage loan (A&J and MSMCRRC mortgage loan). The A&J and MSMCRRC mortgage loan has an interest rate of 4.10% and payments of principal and interest are due quarterly with a final principal payment of approximately \$196.2 million due in November 2028.
- (m) In November 2005, SLR refinanced an existing Federal Housing Administration (FHA) Section 241 Loan with an FHA Section 223 Loan (SLR mortgage loan), which now bears interest at a fixed rate of 3.56%. Payments of principal and interest are due monthly through May 2030.

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(n) In December 2016, SNCH entered into a mortgage loan note with TD Bank (SNCH mortgage loan) for \$8.5 million with an interest rate of 4.03% and is payable in monthly installments through December 2026, with a final payment of \$5.1 million in January 2027.

The terms of the various financing arrangements between the System's subsidiaries and their respective lenders require the System's subsidiaries to maintain certain financial and non-financial covenants. The System and its subsidiaries were in compliance with all financial ratio covenants as of December 31, 2024. As security for its various obligations under the debt agreements, the System has pledged as collateral certain property, plant, and equipment, certain gross receipts and revenues, and limitations on use of certain assets.

SNCH maintains two revolving lines of credit totaling \$40.0 million. The lines are set to expire on June 30, 2025 and July 1, 2025, respectively. The lines of credit allow repayment at any time during the term and require that any remaining unpaid balance be repaid at the expiration date. Outstanding loans bear interest at the Bloomberg Short-Term Bank Yield Index rate plus 0.75% or Prime Floating rate with a floor of 2.75% or Term AMERIBOR with a floor of 2.75%. SNCH had no outstanding balance on the lines of credit at December 31, 2024.

MSH maintains a revolving line of credit with a bank totaling \$100.0 million. The line is set to expire on March 19, 2027. The line of credit allows repayment at any time during the term and requires any remaining unpaid balance be repaid at the expiration date. Outstanding loans bear interest at the Secured Overnight Financing Rate (SOFR) plus 0.80%. MSH had no outstanding balance on the line of credit at December 31, 2024.

Principal payments on long-term debt, excluding finance leases (see Note 8), subsequent to December 31, 2024 are as follows (in thousands):

2025	\$	56,625
2026		64,659
2027		67,487
2028		153,987
2029		51,705
Thereafter		2,131,346
Total	\$	<u>2,525,809</u>

In 2024, the System capitalized net interest of approximately \$11.8 million relating to construction activity in progress.

(8) Leases

The System leases certain property and equipment under finance and operating leases, the classification of which is based on the underlying terms of the agreement and certain criteria, such as lease term relative to useful life and total lease payments compared to fair value, among others. Finance leases result in an accounting treatment similar to an acquisition of the asset.

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Year Ended December 31, 2024

For operating leases, the System has made an accounting policy election not to separate lease components and non-lease components in contracts when determining ROU assets and operating lease liabilities for real estate leases and to separate lease components and non-lease components for equipment leases. The System has also made an election not to record leases with an initial term of less than one year as ROU assets and operating lease liabilities.

For leases with initial terms greater than one year, the System records the related ROU assets and liabilities at the present value of the lease payments to be paid over the life of the related lease. The System's leases may include variable lease payments and renewal options. Variable lease payments are excluded from the amounts used to determine the ROU assets and liabilities unless the variable lease payments depend on an index or rate or are in substance fixed payments. Lease payments related to periods subject to renewal options are also excluded from the amounts used to determine the ROU assets and liabilities unless the System is reasonably certain to exercise the option to extend the lease.

The System's operating leases are primarily for real estate, including office space and parking, typically with initial terms between 5 and 15 years. The System's finance leases are primarily for real estate, typically with initial terms between 25 and 40 years.

The following schedule summarizes information related to the lease assets and liabilities as of and for the year ended December 31, 2024 (in thousands):

Components of lease balances	Classification in consolidated balance sheets	
Assets:		
Operating lease assets	Right-of-use assets	\$ 832,398
Finance lease assets	Property, plant, and equipment, net	754,017
Total leased assets		<u>\$ 1,586,415</u>
Liabilities:		
Operating lease liabilities:		
Current	Current portion of operating lease liabilities	\$ 55,294
Long-term	Operating lease liabilities, net of current portion	876,395
Total operating lease liabilities		<u>\$ 931,689</u>
Finance lease liabilities:		
Current	Current portion of long-term debt	\$ 12,042
Long-term	Long-term debt, net of current portion	1,041,482
Total finance lease liabilities		<u>1,053,524</u>
Total lease liabilities		<u>\$ 1,985,213</u>

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements
Year Ended December 31, 2024

Components of lease expense	Classification in consolidated statements of operations	
Operating lease expense	Supplies and other	\$ 110,509
Finance lease expense:		
Amortization of leased assets	Depreciation and amortization	35,745
Interest on lease liabilities	Interest	30,001
Total finance lease expense		<u>\$ 65,746</u>

Lease activity such as variable lease cost and short-term lease cost and sublease income is not considered to be significant.

The following table summarizes the operating cash flows from leases as well as the weighted average remaining lease terms and weighted average discount rates used to determine the lease obligations as of December 31, 2024 (amounts in thousands):

Financing cash flows from finance leases	\$ 17,252
Operating cash flows from finance leases	30,001
Operating cash flows from operating leases	93,715
Weighted average remaining lease term – finance leases	27 years
Weighted average remaining lease term – operating leases	15 years
Weighted average discount rate – finance leases	2.96%
Weighted average discount rate – operating leases	2.92%

Future maturities of lease liabilities at December 31, 2024 are presented in the following table (in thousands):

	Finance leases	Operating leases
2025	\$ 47,164	\$ 84,359
2026	47,482	80,198
2027	47,589	77,004
2028	46,683	73,852
2029	45,268	77,264
Thereafter	1,355,929	804,767
Total lease payments	<u>1,590,115</u>	<u>1,197,444</u>
Less: imputed interest	(531,188)	(261,784)
Less: lease incentives	(5,403)	(3,971)
Total lease obligation	<u>1,053,524</u>	<u>931,689</u>
Less: current portion	(12,042)	(55,294)
Long-term lease obligation	<u>\$ 1,041,482</u>	<u>\$ 876,395</u>

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

As of December 31, 2024, the System has entered into lease agreements with undiscounted lease payments totaling approximately \$476.7 million which will commence when the premises are ready for intended use with terms ranging from 20 to 36 years.

(9) Professional Liabilities Insurance Program

The System's estimates of professional liabilities are based upon complex actuarial calculations, which utilize factors such as historical claims experience for the System and related industry factors, trending models, estimates for the payment patterns of future claims, and present value discount factors. As a result, there is at least a reasonable possibility that recorded estimates will change by a material amount in the near term. Revisions to estimated amounts resulting from actual experience differing from projected expectations are recorded in the period the information becomes known or when changes are anticipated.

Professional Insurance Liabilities

The System's subsidiaries maintain coverage through various captive and commercial insurance companies. Professional liabilities representing the amount of the System's medical malpractice liabilities that are reinsured from commercial and captive insurance companies are included in professional insurance liabilities in the accompanying consolidated statement of financial position at the actuarially determined present value of approximately \$257.7 million, based on discount rates ranging from 0.0%–2.75%, at December 31, 2024. The System has recorded related insurance recoveries receivable of approximately \$257.7 million at December 31, 2024, in consideration of the expected insurance recoveries. The current portion of professional insurance liabilities and the related insurance recoveries receivable represent an estimate of expected settlements and insurance recoveries over the next twelve months.

(a) MSH, BMC and ISMMS Professional Liability

Primary coverage of professional and general liability incidents was provided through participation in a pooled program with certain other health care facilities affiliated with Healthcare Risk Advisors (previously affiliated with the Federation of Jewish Philanthropies of New York). This occurrence-basis insurance coverage participation was with captive insurance companies and commercial insurance companies.

As of December 31, 2024, the System retains ownership interest of 50% in two captive insurance companies. The System follows the equity method of accounting for its investment in the captive insurance companies and has recognized its allocated share of a portion of the program's accumulated surplus. This investment is recorded in investments and is approximately \$48.0 million at December 31, 2024.

(b) SLR Professional Liability

Effective July 1, 2004, SLR joined with other unrelated not-for-profit institutions to form a segregated cell captive structure through CCC Insurance Company (CCCI). CCCI formed CCC Insurance Company SCC and implemented a "cell captive" structure which replaced the previous coverage structure. Under the cell captive program, primary coverage for individual claims for each participating hospital is provided through a commercial insurance carrier on an occurrence basis. The first layer of excess coverage under the program utilizes individual cells for each participating hospital, under which invested assets and insurance related liabilities are segregated for each participant and there is no shared risk among the entities. Effective January 1, 2015, the SLR segregated cell went into run off and no longer writes premiums. Subsequent to that date, SLR purchases commercial insurance.

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

(c) NYEEI Professional Liability

Since June 2003, NYEEI has purchased professional liability insurance on an occurrence-basis from a commercial carrier, with current policy limits of \$1.3 million per occurrence/\$3.9 million in aggregate as of December 31, 2024.

(d) SNCH Professional Liability

Since August 1, 2013, SNCH has purchased claims-made excess professional liability insurance from a commercial carrier. Under the current policy, the retention limit is approximately \$4.0 million per occurrence/\$20.0 million in aggregate. As of December 31, 2024, the self-insured retentions maintained by SNCH were approximately \$19.6 million and \$51.1 million included in other current liabilities and other liabilities, respectively.

Self-Insurance Trust

Effective January 1, 2018, the Mount Sinai Health System Self-Insurance Trust (the Self-Insurance Trust) was established to provide coverage in excess of existing insurance program limits. Currently, MSH, BIMC, SLR, and NYEEI participate in the Self-Insurance Trust, which is irrevocable. Amounts funded into the Self-Insurance Trust are based upon actuarially determined liabilities. These participating members record beneficial interest assets and liabilities relating to the Self-Insurance Trust, which are reflected in the accompanying consolidated financial statements. Excess coverage of the Self-Insurance Trust is extended to ISMMS by the aforementioned members.

As of December 31, 2024, the Self-Insurance Trust held investments of \$588.1 million on behalf of the System, net of a receivable of \$34.8 million, which is included as beneficial interest in self-insurance trust in the accompanying consolidated statement of financial position.

As of December 31, 2024, the System had actuarially determined liabilities for the Self-Insurance Trust of approximately \$553.3 million, discounted at 4%, which is included as self-insurance liabilities in the accompanying consolidated statement of financial position.

(10) Pensions and Similar Plans

The System's subsidiaries sponsor various qualified defined benefit pension plans and defined contribution plans covering certain groups of employees. The benefits provided are based on age, years of service, and compensation as stipulated by the respective plan documents. The System's policy is to fund the pension benefits with at least the minimum amounts required by the Employee Retirement Income Security Act of 1974. The System's defined benefit pension plans, which are sponsored by SLR, NYEEI, and SNCH, were each previously frozen to new participants and benefit accruals.

Certain subsidiaries of the System maintain Section 457(b) plans. Contributions are made solely by the employees through payroll deductions. The contributions are maintained in individual participant accounts held by a custodian and remain an asset of the employer until the participant retires. A corresponding liability is also recorded for these amounts, which is reduced upon the participant's retirement. The balances recorded at December 31, 2024 in other assets and other long-term liabilities in the accompanying statement of financial position were approximately \$241.7 million.

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements

Year Ended December 31, 2024

Certain System subsidiaries sponsor postretirement plans which provided medical and life insurance benefits to eligible employees; most of the benefit provisions under such plans have been curtailed.

The System recognizes the funded status (i.e., the difference between the fair value of plan assets and the projected benefit obligations) of its defined-benefit pension and postretirement benefits plans, with a corresponding adjustment to net assets without donor restrictions for the portion of the unfunded obligations that have not been recognized as pension and postretirement cost. The adjustment to net assets without donor restrictions represents the net unrecognized actuarial losses and unrecognized prior service cost, which will be subsequently recognized as a component of net periodic pension and postretirement cost through amortization.

The following table provides a reconciliation of the changes in the pension obligation and a statement of the funded status of the plans as of December 31, 2024 (in thousands):

	Defined-benefit pension plans
Change in benefit obligation:	
Benefit obligation at beginning of year	\$ 433,733
Service cost	1,288
Interest cost	21,125
Actuarial gain	(20,745)
Benefits paid	(30,230)
Benefit obligation at end of year	<u>405,171</u>
Change in plans' assets:	
Fair value of assets at beginning of year	398,299
Actual return on plans' assets	361
Employer contributions	11,750
Benefits paid	(29,526)
Fair value of plans' assets at end of year	<u>380,884</u>
Funded status	\$ <u>(24,287)</u>

Pension plan liabilities and pension plan assets are recorded in accrued pension and postretirement liability and other assets in the accompanying consolidated statement of financial position, respectively.

Included in net assets without donor restrictions at December 31, 2024 are the following amounts that have not yet been recognized in net periodic pension cost: unrecognized actuarial losses of approximately \$36.5 million. Unrecognized actuarial losses represent unexpected changes in the projected benefit obligation and plan assets over time, primarily due to changes in assumed discount rates and investment experience.

The actuarial gain in 2024 resulted in an overall decrease in projected benefit obligation at December 31, 2024 of \$20.7 million which was primarily attributable to changes in the discount rate assumptions.

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements

Year Ended December 31, 2024

The projected pension benefit obligations, accumulated benefit obligations and fair value of the plans' assets were as follows at December 31, 2024 (in thousands):

Projected benefit obligation	\$	405,171
Accumulated benefit obligation		405,033
Fair value of plan assets		380,884

The net periodic benefit cost for the year ended December 31, 2024 is as follows (in thousands):

	<u>Defined-benefit pension plans</u>	
Service cost	\$	1,288
Interest cost		13,457
Expected return on plan assets		(15,651)
Amortization of prior service credit		111
Recognized net actuarial gain (loss)		(428)
Total net periodic benefit cost	\$	<u>(1,223)</u>

Weighted-average assumptions used in determining the benefit obligation and net periodic benefit cost as of December 31, 2024 were as follows:

	<u>Defined-benefit pension plans</u>
Weighted average assumptions used to determine benefit obligation:	
Discount rate	5.15 - 5.65%
Rate of compensation increase	0 - 3%
Weighted average assumptions used to determine net benefit cost:	
Discount rate	5.04 - 5.60%
Expected long-term rate of return on plan assets	5.75 - 6.5%
Rate of compensation increase	0 - 3%

The expected long-term rate of return on plan assets for each plan is based on the portfolio as a whole and not the sum of the returns on the individual asset categories. The return is based exclusively on historical returns, without adjustments.

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements

Year Ended December 31, 2024

The future cash flows of the System relative to retirement benefits are expected to be as follows (in thousands):

December 31:

2025	\$	33,373
2026		33,107
2027		33,416
2028		33,040
2029		32,130
2030 to 2034		153,023

The following table presents the System's fair value hierarchy for those pension plan assets measured at fair value as of December 31, 2024 (in thousands):

	<u>Level 1</u>	<u>Level 2</u>	<u>Level 3</u>	<u>Total</u>
Cash and cash equivalents and money market funds	\$ 26,132	\$ —	\$ —	\$ 26,132
Equity securities	12,987	—	—	12,987
Mutual funds	137,193	4,991	—	142,184
Fixed income	90,148	—	—	90,148
Total	<u>\$ 266,460</u>	<u>\$ 4,991</u>	<u>\$ —</u>	<u>271,451</u>
Investments measured at NAV as a practical expedient				109,433
			\$	<u>380,884</u>

Investments measured at NAV as a practical expedient mainly consist of alternative investments related to hedge funds and private investments.

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

The weighted-average allocation of the System's pension plan assets at December 31, 2024 was as follows:

Asset category	Target allocation 2025 (varies by Plan)	Percentage of assets (Aggregate all plans)
Cash and cash equivalents	9%	7%
Equity securities	3	3
Mutual funds	36	37
Fixed income	23	24
Alternative investments	29	29
	<u>100%</u>	<u>100%</u>

(11) Multi-Employer Defined Benefit Plans

The System contributes to various multiemployer defined-benefit pension plans under the terms of collective bargaining agreements that cover its union-represented employees. The risks of participating in these multiemployer plans are different from single employer plans in the following aspects:

- (a) Assets contributed to the multiemployer plan by one employer may be used to provide benefits to employees of other participating employers.
- (b) If a participating employer stops contributing to the plan, the unfunded obligations of the plan may be borne by the remaining participating employers.
- (c) If an employer chooses to stop participating in some of its multiemployer plans, the employer may be required to pay those plans an amount based on the underfunded status of the plan, referred to as a withdrawal liability.

The System's participation in these plans for the year ended December 31, 2024 is outlined in the table below. The "EIN Number" column provides the Employer Identification Number (EIN). Unless otherwise noted, the most recent Pension Protection Act (PPA) zone status available in 2024 is for a plan's year-end at December 31, 2023. The zone status is based on information that the System received from the plans and is certified by the plans' actuaries. Among other factors, plans in the red zone are generally less than 65% funded, plans in the yellow zone are less than 80% funded, and plans in the green zone are at least 80% funded. The "FIP/RP Status Pending/Implemented" column indicates plans for which a financial improvement plan (FIP) or a rehabilitation plan (RP) is pending or has been implemented. The last column lists the expiration dates of the collective bargaining agreements to which the plans are subject. The System's contributions to the multiemployer plans during the year ended December 31, 2024 are included in the following table (in thousands):

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

Pension fund	EIN number	Plan number	Pension Protection Act	FIP/RP	Contributions by MSHS 2024	Surcharge imposed	Expiration date of collective- bargaining agreement
			Zone status 2024	Status pending implemented			
New York State Nurses Association Pension Plan	13-6604799	1	Green	No	\$ 58,940	No	12/31/2025
1199 SEIU Health Care Employees Pension Fund	13-3604862	1	Green	No	106,723	No	9/30/2026

The System was listed in the New York State Nurses Association Pension Plan and 1199 SEIU Health Care Employees Pension Fund Forms 5500 as providing more than 5% of the total contributions during each of the plan's 2023 plan years. At the date the System's consolidated financial statements were issued, Forms 5500 were not yet available for any of the multiemployer plans for the plan year ended in 2024.

(12) Net Assets With Donor Restrictions

Net assets with donor restrictions include endowments that have been restricted by donors to be maintained in perpetuity and invested by the System, with the income expendable to support program activities as stipulated by donors.

The System follows the requirements of the New York Prudent Management of Institutional Funds Act (NYPMIFA) as it relates to its permanently restricted contributions and net assets.

The System has interpreted NYPMIFA as requiring the preservation of the fair value of the original gift as of the gift date of the donor-restricted endowment fund absent explicit donor stipulations to the contrary. As a result of this interpretation, the System classifies within net assets with donor restrictions the original value of the gifts donated to the permanent endowment and the original value of subsequent gifts to the permanent endowment. Accumulations to the permanent endowment are used in accordance with the direction of the applicable donor gift. The remaining portion of the donor-restricted endowment fund is also classified in net assets with donor restrictions until the amounts are appropriated for expenditure in accordance with a manner consistent with the standard of prudence prescribed by NYPMIFA. In accordance with NYPMIFA, the System considers the following factors in making a determination to appropriate or accumulate donor-restricted endowment funds:

(1) The duration and preservation of the fund; (2) the purposes of the System and the donor-restricted endowment fund; (3) general economic conditions; (4) the possible effect of inflation and deflation; (5) where appropriate and circumstances would otherwise warrant, alternatives to expenditure of the endowment fund, giving due consideration to the effect that such alternatives may have on the institution; (6) the expected total return from income and the appreciation of investments; (7) other resources of the System; and (8) the investment and spending policies of the System. The System's policies provide the guidelines for setting the annual spending rate of 4% and the treatment of any investment returns in excess of the annual spending rate. The endowment spend rate is calculated on the average three-year rolling market value of each endowed fund. Any excess investment returns beyond the spending rate, to the extent available, are added to the endowed fund and classified as net assets with donor restrictions, unless also appropriated for expenditure. The System expends the income distributed from certain restricted assets on an annual basis in support of health care services.

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements **Year Ended December 31, 2024**

The System has adopted investment and spending policies for endowment assets that attempt to provide a predictable stream of funding to programs supported by its endowment. Endowment assets are invested in a manner to provide that sufficient assets are available as a source of liquidity for the intended use of the funds, achieve the optimal return possible within the specified risk parameters, prudently invest assets in a high-quality diversified manner, and adhere to the established guidelines.

To satisfy its long-term rate of return objectives, the System relies on a total return strategy in which investment returns are achieved through both capital appreciation (realized and unrealized) and current yield (interest and dividends). The System targets a diversified asset allocation that places a greater emphasis on equity-based investments to achieve its long-term return objectives within prudent risk constraints.

From time to time, the fair value of assets associated with individual donor-restricted endowment funds may fall below the level of the original principal donation. These deficiencies could result from unfavorable market fluctuations that occur shortly after the investment of new donor restricted contributions and continued appropriation for certain programs deemed prudent by the Board of Trustees. There were no deficiencies of this nature that are reported in net assets without donor restrictions as of December 31, 2024.

Net assets with donor restrictions that are temporary in nature are available to support program activities as stipulated by donors. Net assets with donor restrictions that are temporary in nature are restricted as follows at December 31, 2024:

Category:

Research	\$	110,593
Education		90,050
Capital projects		96,432
Other specific-purpose funds		578,860
Total	\$	<u>875,935</u>

Net assets with donor restrictions that are perpetual in nature are restricted as follows at December 31, 2024 (in thousands):

Investments to be held in perpetuity, the income from which is restricted for research and other purposes	\$	378,384
Investments to be held in perpetuity, the income from which is unrestricted as to use		<u>275,034</u>
Total	\$	<u>653,418</u>

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements

Year Ended December 31, 2024

Changes in restricted endowment assets for the year ended December 31, 2024 were as follows (in thousands):

	Return on endowment assets with restrictions	Endowment assets
Restricted endowment assets at January 1, 2024	\$ 136,558	\$ 629,662
Total investment return	48,609	—
Contributions	8	15,292
Appropriation of assets for expenditure	(18,345)	—
Restricted endowment assets at December 31, 2024	<u>\$ 166,830</u>	<u>\$ 644,954</u>

Donor restricted endowment assets with permanent restrictions presented in the table above exclude ISMMS's pledges receivable with permanent donor restrictions of \$8.5 million as of December 31, 2024, which is included within other assets in the accompanying consolidated statement of financial position. Approximately \$5.0 million of ISMMS's donor restricted earnings were appropriated for spending from endowment assets but not released from restrictions in 2024.

Investments to be held in perpetuity are included in pooled investments in the accompanying consolidated statement of financial position.

Net assets were released from restrictions in satisfaction of the following restrictions for the year ended December 31, 2024 (in thousands):

Instruction	\$ 10,672
Capital projects	43,842
Hospital operations	38,056
Research	66,632
Total	<u>\$ 159,202</u>

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

(13) Functional Expenses

The System provides inpatient and outpatient health care and related services, including graduate medical education, to patients from throughout the world. Expenses related to its services for the year ended December 31, 2024 were as follows:

	Healthcare services	Sponsored research	Basic and clinical science	General and administrative	Total
For the year ended December 31, 2024:					
Salaries and wages	\$ 4,486,186	\$ 306,330	\$ 420,589	\$ 716,238	\$ 5,929,343
Employee benefits	1,164,278	85,304	109,722	186,059	1,545,363
Supplies and other	3,285,580	244,795	84,794	387,122	4,002,291
Interest	63,348	—	—	52,917	116,265
Depreciation and amortization	336,060	—	—	190,360	526,420
Total	<u>\$ 9,335,452</u>	<u>\$ 636,429</u>	<u>\$ 615,105</u>	<u>\$ 1,532,696</u>	<u>\$ 12,119,682</u>

The financial statements report certain expense categories that are attributable to more than one healthcare service or support function. Therefore, these expenses require an allocation on a reasonable basis that is consistently applied. Costs not directly attributable to a function are allocated on a units of service basis or are otherwise allocated based on revenue.

(14) Commitments and Contingencies

(a) *Litigation*

The System is a defendant in various legal actions arising out of the normal course of its operations, the final outcome of which cannot presently be determined. Management is of the opinion that the ultimate liability, if any, with respect to all of these matters will not have a material adverse effect on the System's consolidated financial position.

(b) *Collective Bargaining Agreements*

Approximately 45% of the System's employees are members of various unions who are covered under the terms of various collective bargaining agreements, of which approximately 30% of the covered employees are under a contract that expires on December 31, 2025.

(c) *Other*

The System is self-insured, based on individual employees' elections, for medical, dental, and pharmaceutical benefits. The System also is self-insured for unemployment benefits and worker's compensation. Liabilities have been accrued at December 31, 2024 based on expected future payments (included in accrued salaries and related liabilities).

MOUNT SINAI HEALTH SYSTEM, INC.Notes to the Consolidated Financial Statements
Year Ended December 31, 2024**(15) Other Revenue**

Other revenue includes operating revenues that are not directly related to the System's patient services. Other revenue consisted of the following (in thousands):

Federal grants and contracts	\$	689,469
Other grants and contracts		219,332
FEMA revenue (see Note 1)		183,626
Pharmacy sales		871,930
Tuition and fee revenue		43,244
Distributions from Healthfirst		58,588
Rental income		68,543
Contribution revenue		71,915
Affiliation revenue from New York City Health and Hospitals Corporation		332,725
Royalty revenue		54,205
Investment income		209,781
Net assets released from restrictions for operations		115,360
Other		285,728
	\$	<u>3,204,446</u>

(16) Pledges Receivable

Pledges receivable, representing unconditional promises to give to the System, recorded net of a present value discount (based on a range of interest rates of 2.36% to 4.68%) and valuation allowance, are reported within other assets and consist of the following at December 31, 2024 (in thousands):

Unconditional promises to give before discount to present value and valuation allowance	\$	187,881
Less: present value discount and valuation allowance		<u>(25,048)</u>
Net pledges receivable	\$	<u><u>162,833</u></u>

Pledges receivable are due to be collected over the following periods at December 31, 2024 (in thousands):

Within one year	\$	62,002
One to five years		99,609
More than five years		<u>1,222</u>
Total pledges receivable	\$	<u><u>162,833</u></u>

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements Year Ended December 31, 2024

ISMMS is party to a pledge agreement among ISMMS, MSH, Carl C. Icahn (the Donor) and The Icahn Medical Research Foundation (the Icahn Foundation). The pledge will be paid by the Donor to the Icahn Foundation solely for use by the Icahn Institute of Medical Research at Mount Sinai LLC (the MRO), a single member limited liability company of which the Icahn Foundation is the sole member, to conduct research in conjunction with ISMMS and MSH pursuant to terms of a collaboration agreement. The purpose of the collaboration agreement and the establishment of the MRO is to enable ISMMS, MSH, the Icahn Foundation and the MRO to closely cooperate in a joint effort to conduct research in the fields of genomics, multi-scale biology and related matters.

(17) Agreements With the New York City Health and Hospitals Corporation

Pursuant to various agreements with New York City Health and Hospitals Corporation (HHC), ISMMS provides professional, medical and other services for the operations of Elmhurst Hospital Center (Elmhurst) and Queens Hospital Center (Queens). For services provided under the agreements, ISMMS is reimbursed for costs incurred, plus overhead, but not in excess of amounts specified in the agreements. Certain costs are funded by the operations of faculty practice group arrangements at Elmhurst and Queens, which are independent of other School programs, under a letter of understanding with HHC. The agreement expired on June 30, 2020, and was extended an additional six months. Since the expiration of the six-month extension, the contract continues to be extended on a month-to-month basis. HHC and ISMMS are negotiating a new five-year contract. A fiscal year budget is proposed and agreed upon before the start of each fiscal year during this period of negotiation.

The agreements with HHC do not permit the accrual of vacation and retirement benefits. ISMMS would be liable for such benefits only upon termination of the agreements; however, ISMMS's liability would be limited to amounts due based on benefits policies in effect at that time upon termination of the agreements.

ISMMS's arrangements with HHC are subject to final settlements based on future audits; however, ISMMS anticipates that the effects of future final settlements will not be material.

(18) Employee Relocation Loan Program

The System maintains an employee relocation loan program whereby ISMMS participates in a portion of the financing of the primary residence mortgage of eligible faculty members. All taxes, insurance and repair and maintenance costs of the residence are the responsibility of the faculty members. If the faculty member's employment is terminated before the mortgage is repaid, the faculty member may purchase ISMMS's share of the equity and assume the remainder of ISMMS's mortgage obligation on the property, or the residence will be sold in accordance with the employee relocation loan program agreement and ISMMS will be entitled to a portion of the proceeds. ISMMS's participation in this program aggregated approximately \$89.1 million at December 31, 2024.

(19) Sponsored Research

Research grants and contracts generally provide for the recovery of direct and indirect costs. The System recognizes revenue associated with direct costs as conditional barriers are met and the related costs are incurred. Recovery of related indirect costs is recorded using facilities and administrative rates negotiated with the federal government and other sponsors. Direct expenditures under federal research projects totaled approximately \$474.1 million and is recorded as a component of federal grants and contracts within other revenue in the accompanying consolidated statement of operations for the year ended December 31, 2024.

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements

Year Ended December 31, 2024

The System's principal source of federal research funds is the U.S. Department of Health and Human Services. Federal and other research grants and contracts awarded but for which expenditures have not yet been incurred or cash has not been received approximated \$512.5 million as of December 31, 2024.

(20) BIMC Phillips School of Nursing

Summary of financial information for the Mount Sinai Phillips School of Nursing for the year ended December 31, 2024 is included in the table below (in thousands):

Total revenue	\$ 11,886
Salaries and wages	3,633
Employee benefits	899
Supplies and other	1,252
Depreciation and amortization	2,827
Total expenses	\$ 8,611

(21) Department of Education Title IV

U.S. Department of Education regulations require additional disclosures deemed necessary to calculate certain ratios for determining sufficient financial responsibility under Title IV regulations. The information through the year ended December 31, 2024 is as follows:

	BIMC	ISMMS
Assets		
Property, plant and equipment, net at January 1, 2024	\$ 754,540	\$ 1,371,318
Debt-financed property, plant and equipment, net acquired during 2024	—	48,988
Property, plant and equipment, net acquired without debt during 2024	(46,931)	(5,180)
Net change in construction in progress	21,015	(41,148)
Property, plant and equipment, net at December 31, 2024	728,624	1,373,978
Secured and unsecured related party receivable	18,255	18,449
Unsecured related party receivable	18,255	18,449
Right-of-use assets at January 1, 2024	344,319	734,777
Right-of-use assets at December 31, 2024	322,241	715,035

MOUNT SINAI HEALTH SYSTEM, INC.

Notes to the Consolidated Financial Statements
Year Ended December 31, 2024

	BIMC	ISMMS
Liabilities		
Long-term debt, pre-implementation	\$ —	\$ 430,415
Long-term debt, post-implementation	—	554,238
	<u>—</u>	<u>984,653</u>
Lease liabilities at January 1, 2024	441,234	950,087
Lease liabilities at December 31, 2024	387,146	941,487
Net Assets		
Net assets with donor restrictions: purpose and time restrictions	20,414	634,435
Net assets with donor restrictions: restricted in perpetuity	12,234	459,177
Revenues without donor restrictions		
Total operating revenue	1,020,170	4,312,105
Net change in net unrealized gains and losses on investments	4,470	255,173
Net assets released from restriction for capital	—	5,597
Transfer from affiliate	—	1,708
Other items	5,492	395
	<u>\$ 1,030,132</u>	<u>\$ 4,574,978</u>

(22) Subsequent Events

For purposes of the accompanying consolidated financial statements, the System has considered for accounting and disclosure events that occurred through May 30, 2025, the date the consolidated financial statements were issued.

There were no subsequent events or transactions that either resulted in recognition in the accompanying consolidated financial statements or required additional disclosure except for the closure of the 16th Street campus disclosed in Note 1.

Supplementary Information

MOUNT SINAI HEALTH SYSTEM, INC.

Financial Responsibility Supplemental Schedule – Beth Israel Medical Center and Affiliates

(In thousands)

Year Ended December 31, 2024

Reference to Financial Statements and/or Notes	Ratio Element	Amount
Primary reserve ratio		
Expendable net assets:		
Consolidating Statement of Financial Position	Net assets without donor restrictions	\$ (230,458)
Consolidating Statement of Financial Position	Net assets with donor restrictions	32,648
Note 21. Department of Education Title IV	Secured and unsecured related party receivable	18,255
Note 21. Department of Education Title IV	Unsecured related party receivable	18,255
Note 21. Department of Education Title IV	Property, plant and equipment, net at January 1, 2024	754,540
Note 21. Department of Education Title IV	Property, plant and equipment, net acquired with debt during 2024	–
Note 21. Department of Education Title IV	Property, buildings and equipment, net acquired without debt during 2024	(46,931)
Note 21. Department of Education Title IV	Net change in construction in progress	21,015
Note 21. Department of Education Title IV	Total property, plant and equipment, net at December 31, 2024	728,624
Note 21. Department of Education Title IV	Right-of-use assets at January 1, 2024	344,319
Note 21. Department of Education Title IV	Right-of-use assets at December 31, 2024	322,241
Not applicable	Intangible assets	–
Not applicable	Accrued pension and postretirement liability	–
Not applicable	Long-term debt – for long term purposes pre-implementation	–
Not applicable	Long-term debt – for long term purposes post-implementation	–
Note 21. Department of Education Title IV	Lease liabilities at January 1, 2024	441,234
Note 21. Department of Education Title IV	Lease liabilities at December 31, 2024	387,146
Not applicable	Annuities with donor restrictions	–
Note 21. Department of Education Title IV	Purpose and time restriction	20,414
Note 21. Department of Education Title IV	Restricted in perpetuity	12,234
Consolidating Statement of Financial Position	Total net assets with donor restrictions	32,648
Total expenses and losses:		
Consolidating Statement of Operations	Total expenses without donor restrictions	1,179,090
Not applicable	Non-operating and net investment (loss)	–
Not applicable	Net investment losses	–
Not applicable	Pension and postretirement changes other than net periodic benefit costs	–
Equity ratio		
Modified net assets:		
Consolidating Statement of Financial Position	Net assets without donor restrictions	(230,458)
Consolidating Statement of Financial Position	Net assets with donor restrictions	32,648
Not applicable	Intangible assets	–
Note 21. Department of Education Title IV	Secured and unsecured related party receivable	18,255
Note 21. Department of Education Title IV	Unsecured related party receivable	18,255
Modified assets:		
Consolidating Statement of Financial Position	Total assets	1,254,803
Not applicable	Intangible assets	–
Note 21. Department of Education Title IV	Secured and unsecured related party receivable	18,255
Note 21. Department of Education Title IV	Unsecured related party receivable	18,255
Net income ratio		
Consolidating Statement of Operations	Change in net assets without donor restrictions	(148,958)
Note 21. Department of Education Title IV	Revenues without donor restrictions	1,030,132

MOUNT SINAI HEALTH SYSTEM, INC.

Financial Responsibility Supplemental Schedule – Icahn School of Medicine at Mount Sinai

(In thousands)

Year Ended December 31, 2024

Reference to Financial Statements and/or Notes	Ratio Element	Amount
Primary reserve ratio		
Expendable net assets:		
Consolidating Statement of Financial Position	Net assets without donor restrictions	\$ 448,862
Consolidating Statement of Financial Position	Net assets with donor restrictions	1,093,612
Note 21. Department of Education Title IV	Secured and unsecured related party receivable	18,449
Note 21. Department of Education Title IV	Unsecured related party receivable	18,449
Note 21. Department of Education Title IV	Property, plant and equipment, net at January 1, 2024	1,371,318
Note 21. Department of Education Title IV	Property, plant and equipment, net acquired with debt during 2024:	
	Finance leases	48,988
Note 21. Department of Education Title IV	Property, buildings and equipment, net acquired without debt during 2024	(5,180)
Note 21. Department of Education Title IV	Net change in construction in progress	(41,148)
Note 21. Department of Education Title IV	Total property, plant and equipment, net at December 31, 2024	1,373,978
Note 21. Department of Education Title IV	Right-of-use assets at January 1, 2024	734,777
Note 21. Department of Education Title IV	Right-of-use assets at December 31, 2024	715,035
Not applicable	Intangible assets	–
Consolidating Statement of Financial Position	Accrued pension and postretirement liability	3,227
Note 21. Department of Education Title IV	Long-term debt – for long term purposes pre-implementation	430,415
Note 21. Department of Education Title IV	Long-term debt – for long term purposes post-implementation	554,238
Note 21. Department of Education Title IV	Total long-term debt	984,653
Note 21. Department of Education Title IV	Lease liabilities at January 1, 2024	950,087
Note 21. Department of Education Title IV	Lease liabilities at December 31, 2024	941,487
Not applicable	Annuities with donor restrictions	–
Note 21. Department of Education Title IV	Purpose and time restriction	634,435
Note 21. Department of Education Title IV	Restricted in perpetuity	459,177
Consolidating Statement of Financial Position	Total net assets with donor restrictions	1,093,612
Total expenses and losses:		
Consolidating Statement of Operations	Total expenses without donor restrictions	4,312,326
Not applicable	Non-operating and net investment (loss)	–
Not applicable	Net investment losses	–
Not applicable	Pension and postretirement changes other than net periodic benefit costs	–
Equity ratio		
Modified net assets:		
Consolidating Statement of Financial Position	Net assets without donor restrictions	448,862
Consolidating Statement of Financial Position	Net assets with donor restrictions	1,093,612
Not applicable	Intangible assets	–
Note 21. Department of Education Title IV	Secured and unsecured related party receivable	18,449
Note 21. Department of Education Title IV	Unsecured related party receivable	18,449
Modified assets:		
Consolidating Statement of Financial Position	Total assets	4,416,059
Not applicable	Intangible assets	–
Note 21. Department of Education Title IV	Secured and unsecured related party receivable	18,449
Note 21. Department of Education Title IV	Unsecured related party receivable	18,449
Net income ratio		
Consolidating Statement of Operations	Change in net assets without donor restrictions	262,652
Note 21. Department of Education Title IV	Revenues without donor restrictions	4,574,978

Supplementary Information, Audit Reports and
Schedules Related to the Uniform Guidance

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards

Year Ended December 31, 2024

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Agriculture							
Agricultural Research Basic and Applied Research: Pass-through Research Corporation for Science Advancement (RCSA)	10.001	SA- MZT-2023-065C	\$ 55,031	\$ –	\$ –	\$ 55,031	\$ –
Specialty Crop Block Grant Program – Farm Bill: Pass-through New York Farm Viability Institute	10.170	SCG 22 005/SCG 23 009	53,457	–	–	53,457	–
Agriculture and Food Research Initiative (AFRI): Pass-through University of Missouri System	10.310	2024-67015-42402/USDA	24,644	–	–	24,644	–
WIC Special Supplemental Nutrition Program for Women, Infants, and Children: Pass-through Program from New York State Department of Health WIC Program – Administration	10.557	DOH01-C35478GG-3450000 DOH01-C38232GG-3450000	–	–	696,924	696,924	–
Pass-through Program from New York State Department of Health WIC Program – Food instruments	10.557	DOH01-C35478GG-3450000 DOH01-C38232GG-3450000	–	–	1,353,380	1,353,380	–
Total Special Supplemental Nutrition Program for Women, Infants, and Children			–	–	2,050,304	2,050,304	–
Environmental Quality Incentives Program	10.912		14,980	–	–	14,980	–
Total Department of Agriculture			148,112	–	2,050,304	2,198,416	–

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Defense							
Basic and Applied Scientific Research:							
Pass-through National Marrow Donor Program	12.300	590-RITN	\$ —	\$ —	\$ 8,835	\$ 8,835	\$ —
Pass-through University of California, Davis	12.300	A20-2957-S001	32,417	—	—	32,417	—
Total Basic and Applied Scientific Research			32,417	—	8,835	41,252	—
Scientific Research – Combating Weapons of Mass Destruction:							
Pass-through University of California, San Francisco	12.351	15045SC	120,771	—	—	120,771	—
Military Medical Research and Development:							
U.S. Army	12.420		10,787,221	—	—	10,787,221	2,733,816
Pass-through Bronx Veterans Medical Research Inc.	12.420	191709,W8XWH2010328 SUB-1	23,369	—	—	23,369	—
Pass-through Cold Spring Harbor Laboratory	12.420	61480112	7,109	—	—	7,109	—
Pass-through Columbia University	12.420	1(GG016845-01)	172	—	—	172	—
Pass-through Ecog-Acrin Medical Research	12.420	HT9425231057201MTSN1	122,354	—	—	122,354	—
Pass-through Georgia State University	12.420	SP00014255-01	32,789	—	—	32,789	—
Pass-through Oregon Health & Science University	12.420	1014941 MTSINAI	3,992	—	—	3,992	—
Pass-through Scripps Research Institute/Sanford Burnham Prebys Medical Discovery Institute	12.420	69867-13290-MSSM,1150-01	52,496	—	—	52,496	—
Pass-through Sloan-Kettering Cancer Center	12.420	C21859645	40,908	—	—	40,908	—
Pass-through University of California, San Diego	12.420	129688973	59	—	—	59	—
Pass-through University of Florida	12.420	SUB00004252	31,831	—	—	31,831	—
Pass-through University of Porto	12.420	W81XWH1920053	53,058	—	—	53,058	—

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Defense (DOD) (continued)							
Pass-through University of Southern California	12.420	SCON-00003699	\$ 3,939	\$ —	\$ —	\$ 3,939	\$ —
Pass-through Yale University	12.420	CON-80004790GR122504,	913,561	—	—	913,561	—
Total Military Medical Research and Development		CON-80004527	12,072,858	—	—	12,072,858	2,733,816
Basic Scientific Research:							
U.S. Army	12.431		530,314	—	—	530,314	—
Research and Technology Development:							
Defense Advanced Research Project	12.910		3,404,935	—	—	3,404,935	991,060
COVID-19 Pass-through University of California, San Diego	12.910	11961SC	113	—	—	113	—
Pass-through Harvard University	12.910	HR0011-20-2-0040,	14,226	—	—	14,226	—
Pass-through University of North Carolina	12.910	168032-5116457	665,980	—	—	665,980	—
Total Research and Technology Development		5117493	4,085,254	—	—	4,085,254	991,060
Direct Contracts	12.RD		5,581,357	—	—	5,581,357	1,179,080
Total Department of Defense			22,422,971	—	8,835	22,431,806	4,903,956
Department of Housing and Urban Development							
Mortgage Insurance Hospitals	14.128		—	—	119,206,932	119,206,932	—
Total Department of Housing and Urban Development			—	—	119,206,932	119,206,932	—

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Justice (DOJ)							
Sexual Assault Services Formula Program:							
Pass-through New York State Coalition Against Sexual Assault (NYS CASA)	16.017	SASP,SASP 22-23, SASP 23-24,RCP-15, RCP02	\$ —	\$ —	\$ 29,733	\$ 29,733	\$ —
Services for Trafficking Victims	16.320		—	—	357,861	357,861	—
National Institute of Justice Research, Evaluation, and Development Project Grants:							
National Institute of Justice	16.560		—	—	632,186	632,186	429,229
Crime Victim Assistance:							
Pass-through New York State Crime Victims Board	16.575	C100234/OVS01-C11000GG-1080200	—	—	579,136	579,136	—
Pass-through New York Department of Health	16.575	PDV01-T00026GG-1310000	—	—	23,236	23,236	—
		OVS01-C11208GG-1080200, OVS01-C11283GG-10802, OVS01-C11284GG-1080200, OVS01-C11283GG-1080200, OVS01-C10977GG-10802					
Pass-through New York State, Office of Victim Services	16.575		—	—	1,293,743	1,293,743	—
Total Crime Victim Assistance			—	—	1,896,115	1,896,115	—
Violence Against Women Formula Grants:							
Pass-through New York State, Division of Criminal Justice Services	16.588	DCJ01-C00355GG-1090000	—	—	97,717	97,717	—

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Justice (DOJ) (continued)							
Grants for Outreach and Services to Underserved Populations:							
Pass-through The Shalom Task Force	16.889	2016-UD-AX-0004	\$ —	\$ —	\$ 48,146	\$ 48,146	\$ —
Total Department of Justice			—	—	3,061,758	3,061,758	429,229
Department of Labor							
Direct Contracts	17.RD		3,200	—	—	3,200	—
Total US Department of Labor			3,200	—	—	3,200	—
National Aeronautics and Space Administration (NASA)							
Exploration:							
NASA	43.003		267,124	—	—	267,124	—
Pass-through University of Virginia	43.003	GG13377	180,914	—	—	180,914	—
Total Exploration			448,038	—	—	448,038	—
Direct Contract – National Aeronautics and Space Administration (NASA)	43.RD		484,347	—	—	484,347	—
Total National Aeronautics and Space Administration (NASA)			932,385	—	—	932,385	—
Promotion of the Arts Grants to Organizations and Individuals							
National Endowment for the Art	45.024		12,003	—	—	12,003	—
Total Promotion of the Arts Grants to Organizations and Individuals			12,003	—	—	12,003	—

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
National Science Foundation (NSF)							
Engineering:							
National Science Foundation	47.041		\$ 236,256	\$ —	\$ —	\$ 236,256	\$ —
Pass-through Emory University	47.041	A146712	116	—	—	116	—
Pass-through Research Foundation of CUNY	47.041	CM00007252-00	47,090	—	—	47,090	—
Pass-through University of Washington	47.041	2103085-02,UWSC13721	103,023	—	—	103,023	—
Total Engineering			386,485	—	—	386,485	—
Computer and Information Science and Engineering:						—	
National Science Foundation	47.070		462,501	—	—	462,501	—
Biological Science:							
National Science Foundation	47.074		90,487	—	—	90,487	—
Pass-through University of California, San Diego	47.074	122085731	955	—	—	955	—
Total Biological Science			91,442	—	—	91,442	—
Social, Behavioral, and Economic Sciences							
National Science Foundation	47.075		1,471	—	—	1,471	—
Pass-through University of California, Davis	47.075	A24-0907-S001	78,172	—	—	78,172	—
Total Social, Behavioral, and Economic Sciences			79,643	—	—	79,643	—
STEM Education (formerly Education and Human Resources)	47.076		39,001	—	—	39,001	—
Office of International Science and Engineering:							
COVID-19 Pass-through University of Wisconsin	47.079	0000001034	775	—	—	775	—

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
National Science Foundation (NSF) (continued)							
NSF Technology, Innovation, and Partnerships	47.084		\$ 189,449	\$ —	\$ —	\$ 189,449	\$ —
Total National Science Foundation			1,249,296	—	—	1,249,296	—
Department of Veterans Affairs							
Direct Contracts	64.RD		16,969	—	—	16,969	—
Total Department of Veterans Affairs			16,969	—	—	16,969	—
Environmental Protection Agency							
Science To Achieve Results (STAR) Research Program							
Pass-through Drexel University	66.509	950052	160,611	—	—	160,611	—
Total Environmental Protection Agency			160,611	—	—	160,611	—
Department of Energy							
Office of Science Financial Assistance Program:							
Pass-through Research Foundation of CUNY	81.049	CM00009609-00	121,922	—	—	121,922	—
Total Department of Energy			121,922	—	—	121,922	—
Department of Education (ED)							
Federal Work-Study Program	84.033		—	124,387	—	124,387	—
Federal Perkins Loan Program	84.038		—	666,475	—	666,475	—
Federal Direct Student Loans	84.268		—	20,315,635	—	20,315,635	—
Total Department of Education			—	21,106,497	—	21,106,497	—

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
Blood Disorder Program: Prevention, Surveillance, and Research:							
Pass-through American Thrombosis and Hemostasis Network	93.080	ATHN2020CDC-II-02, ATHN2020CDC-II-04, ATHN2020CDC-II-05/ ATHN2015001-II-5, ATHN2020CDC-II-01	\$ —	\$ —	\$ 319,123	\$ 319,123	\$ 213,760
Food and Drug Administration Research:							
Pass-through Columbia University	93.103	1(GG015230-01)	11,643	—	—	11,643	—
Comprehensive Community Mental Health Services for Children with Serious Emotional Disturbances (SED):							
Pass-through Missouri Coalition for Community Behavioral Healthcare	93.104	5 H79 SM63402-02	3,170	—	—	3,170	—
Maternal and Child Health Federal Consolidated Programs:							
Direct Grant – DHHS – Health Resources and Services Administration	93.110		—	—	502,615	502,615	374,671
Environmental Health:							
Direct Grant – NIH – National Institute of Environmental Health Sciences	93.113		20,074,726	—	—	20,074,726	3,059,643
Pass-through Brown University	93.113	00002589	70,739	—	—	70,739	—
Pass-through Children's Hospital, Los Angeles	93.113	000013899-A	120	—	—	120	—

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through Colorado State University	93.113	G-20594-01	\$ 39,154	\$ —	\$ —	\$ 39,154	\$ —
		5(GG008446-12),1(GG008502-01),2(GG018891-01),1(GG017455-01),1 GG015212-01,1 GG017471-01,1(GG020065-01)					
Pass-through Columbia University	93.113	A025046,A025046,	755,832	—	—	755,832	—
Pass-through Emory University	93.113	A692682,A857263	82,521	—	—	82,521	—
Pass-through Feinstein Institute for Medical Research	93.113	AWD00001815 MSSM	111,567	—	—	111,567	—
Pass-through Harvard School of Public Health	93.113	113116-5111919	817	—	—	817	—
Pass-through Henry Ford Health System	93.113	1 R01 ES028235-01	1,623	—	—	1,623	—
		2005345004,2004667874,					
Pass-through Johns Hopkins University	93.113	2005429475	214,107	—	—	214,107	—
		22-A0-00-1008820,					
Pass-through New York University	93.113	20-00-001004434	179,118	—	—	179,118	—
Pass-through Queens College Of CUNY	93.113	CM00000654-00	2,087	—	—	2,087	—
Pass-through Seattle Children's Hospital	93.113	12946SUB	148,949	—	—	148,949	—
Pass-through University of Massachusetts	93.113	017789-9165	9,399	—	—	9,399	—
Pass-through University of North Carolina	93.113	5120767,5129587	146,644	—	—	146,644	—
Pass-through University of Southern California	93.113	122873373,133419509	25,940	—	—	25,940	—
Total Environmental Health			21,863,343	—	—	21,863,343	3,059,643
Preventive Medicine Residency:							
Direct Grant – DHHS – Health Resources and Services Administration	93.117		176,267	—	—	176,267	—

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Oral Diseases and Disorders Research:							
Direct Grant – NIH – National Institute of Dental & Craniofacial Research	93.121		\$ 3,742,619	\$ –	\$ –	\$ 3,742,619	\$ 351,551
Pass-through Case Western Reserve University	93.121	RES516500,RES603447	14,104	–	–	14,104	–
Pass-through Johns Hopkins University	93.121	2005598144	15,131	–	–	15,131	–
Pass-through Massachusetts General Hospital	93.121	241910	922	–	–	922	–
Pass-through Mayo Clinic	93.121	ICA-325916,ICA-332513	245,827	–	–	245,827	–
Pass-through Pennsylvania State University	93.121	S002741-DHHS	52,224	–	–	52,224	–
Pass-through SUNY at Buffalo	93.121	R1229593	99,680	–	–	99,680	–
Pass-through University of Puerto Rico	93.121	2019-001938	36,652	–	–	36,652	–
Pass-through Virginia Commonwealth University	93.121	FP00021429_SA001	5,906	–	–	5,906	–
Total Oral Diseases and Disorders Research			4,213,065	–	–	4,213,065	351,551
Injury Prevention and Control Research and State and Community Based Programs:							
Pass-through Fund for Public Health New York	93.136	85287	118,339	–	–	118,339	–
NIEHS Superfund Hazardous Substances_Basic Research and Education:							
Direct Grant – NIH – National Institute of Environmental Health Sciences	93.143		440,142	–	–	440,142	–
Pass-through Research Foundation of CUNY	93.143	CM00007073-00	52,233	–	–	52,233	–
Pass-through University of Pittsburgh	93.143	AWD00007420(139184-2	47,411	–	–	47,411	–
Total NIEHS Superfund Hazardous Substances_Basic Research and Education			539,786	–	–	539,786	–

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
HIV-Related Training and Technical Assistance: Pass-through Trustees of Columbia University	93.145	Unknown	\$ —	\$ —	\$ 115,808	\$ 115,808	\$ —
Coordinated Services and Access to Research for Women, Infants, Children, and Youth	93.153		—	—	1,027,810	1,027,810	—
Health Program for Toxic Substances and Disease Registry: COVID-19 Pass-through American Academy of Pediatrics	93.161	1012132 – ISMMS,100804 1012032 ISMMS,PO100752,	51,549	—	—	51,549	—
Pass-through American Academy of Pediatrics	93.161	100804,101029,101478	289,946	—	—	289,946	—
Pass-through Karolinska Institute	93.161	4-2735/2021	102,482	—	—	102,482	—
Total Health Program for Toxic Substances and Disease Registry			443,977	—	—	443,977	—
Human Genome Research:							
Direct Grant – NIH – National Human Genome Research Institute	93.172		6,263,688	—	—	6,263,688	403,557
Pass-through Case Western Reserve University	93.172	RES515853	14,224	—	—	14,224	—
Pass-through Massachusetts General Hospital	93.172	239465	134,280	—	—	134,280	—
Pass-through Rutgers University	93.172	1207	39,259	—	—	39,259	—
Pass-through The Broad Institute	93.172	5001165-5500001661	67,499	—	—	67,499	—
Pass-through University of California, Los Angeles	93.172	1460 G YA565	83,246	—	—	83,246	—
Pass-through University of California, Santa Cruz	93.172	A20-0450-S003	187,067	—	—	187,067	—
Pass-through University of Colorado	93.172	FY21.1067.002	586,856	—	—	586,856	—
Total Human Genome Research			7,376,119	—	—	7,376,119	403,557

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Research Related to Deafness and Communication Disorders:							
Direct Grant – NIH – National Institute on Deafness and Other Communication Disorders	93.173		\$ 1,856,233	\$ –	\$ –	\$ 1,856,233	\$ 186,911
Pass-through New York University	93.173	F1490-001	13,207	–	–	13,207	–
Pass-through University of Delaware	93.173	UDR0000559	58,432	–	–	58,432	–
Pass-through University of Illinois	93.173	18720	238,575	–	–	238,575	–
Total Research Related to Deafness and Communication Disorders			2,166,447	–	–	2,166,447	186,911
Nursing Workforce Diversity	93.178		–	–	758,136	758,136	–
Research and Training in Complementary and Integrative Health:							
Direct Grant – NIH – National Center for Complementary & Integrative Health	93.213		2,921,618	–	–	2,921,618	225,175
Pass-through New York University	93.213	17-A0-00-008501	8,657	–	–	8,657	–
Total Research and Training in Complementary and Integrative Health:			2,930,275	–	–	2,930,275	225,175
Research on Healthcare Costs, Quality and Outcomes:							
Direct Grant – DHHS – Agency for Healthcare Research and Quality	93.226		1,370,367	–	–	1,370,367	53,815
Pass-through University of California, San Francisco	93.226	176386341	1,521	–	–	1,521	–
Pass-through University of Pittsburgh	93.226	AWD00004483 136450-5	104,262	–	–	104,262	–
Pass-through University of Texas	93.226	171072/171070	30,358	–	–	30,358	–
Total Research on Healthcare Costs, Quality and Outcomes			1,506,508	–	–	1,506,508	53,815

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
National Center on Sleep Disorders Research:							
Direct Grant – NIH – National Heart, Lung, and Blood Institute	93.233		\$ 2,280,183	\$ –	\$ –	\$ 2,280,183	\$ 266,834
Pass-through Brigham and Women’s Hospital	93.233	125876	344,248	–	–	344,248	–
Pass-through University of Texas Health Science	93.233	SA0002320	14,187	–	–	14,187	–
Total National Center on Sleep Disorders Research			2,638,618	–	–	2,638,618	266,834
Mental Health and Research Grants:							
Direct Grant – NIH – National Institute of Mental Health	93.242		45,426,477	–	–	45,426,477	5,269,185
Pass-through Brigham and Women’s Hospital	93.242	124050,126096,131186 1(GG14236-01),2(GG017166-01), 5(GG017014-01),	614,101	–	–	614,101	–
Pass-through Columbia University	93.242	2(GG017430-02)	436,916	–	–	436,916	–
Pass-through Duke University	93.242	A034316,A034615	41,454	–	–	41,454	–
Pass-through Duke University/Case Western Reserve University	93.242	RES515974,303-000052	436,705	–	–	436,705	–
Pass-through Emory University	93.242	A403741	78,552	–	–	78,552	–
Pass-through Foundation for Mental Hygiene	93.242	154664,160961,1018999/6/28470	70,395	–	–	70,395	–
Pass-through Georgia State University	93.242	SP00011004-01	1,488	–	–	1,488	–
Pass-through Harvard University	93.242	132748-5118112	17,726	–	–	17,726	–
Pass-through Johns Hopkins University	93.242	20,048,861,532,005,900,000	124,078	–	–	124,078	–
Pass-through Nathan Kline Institute	93.242	1019082/1/28500	67,686	–	–	67,686	–
Pass-through New York Genome Center	93.242	2021-0076-MSSM-01	414,755	–	–	414,755	–
Pass-through New York University	93.242	19-A0-00-1003397	204,650	–	–	204,650	–
Pass-through Research Foundation for Mental Hygiene	93.242	26738/134481,162461,165397	102,682	–	–	102,682	–

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through Rutgers University	93.242	SUB00002649	\$ 154,769	\$ —	\$ —	\$ 154,769	\$ —
Pass-through Scripps Research Institute	93.242	5-27216	229	—	—	229	—
Pass-through Thomas Jefferson University	93.242	080-19250-S36601	138,498	—	—	138,498	—
Pass-through Trustees of Boston University	93.242	4500004602	31,483	—	—	31,483	—
Pass-through University of California, San Diego	93.242	67274747/S9001213, KR 705321,11076SC	268,906	—	—	268,906	—
Pass-through University of Louisville	93.242	ULRF 21-0442-01	275	—	—	275	—
Pass-through University of Maryland	93.242	21690	153,868	—	—	153,868	—
Pass-through University of Miami	93.242	OS00001628	2,440	—	—	2,440	—
Pass-through University of Southern California	93.242	SCON-00006811	41,547	—	—	41,547	—
Pass-through University of Utah	93.242	10054948-01	65,602	—	—	65,602	—
Pass-through University of Virginia	93.242	GR102867.SUB00000952	50,148	—	—	50,148	—
Pass-through Virginia Commonwealth University	93.242	FP00012312 SA002	43,095	—	—	43,095	—
Pass-through Virginia Polytechnic Institute	93.242	412600-19C70	132,265	—	—	132,265	—
Pass-through Washington University	93.242	WU-21-393	45,059	—	—	45,059	—
Pass-through Yale University	93.242	GR110980-CON80002767, CON-80005236GR124872, CON-80005372 (GR125675), 7 R01 MH121074-02	790,272	—	—	790,272	—
Total Mental Health and Research Grants			49,956,121	—	—	49,956,121	5,269,185

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Substance Abuse and Mental Health Services Projects of Regional and National Significance							
Direct Grant – DHHS – Substance Abuse and Mental Health Services Administration	93.243		\$ 448,783	\$ –	\$ –	\$ 448,783	\$ –
Substance Abuse and Mental Health Services: Projects of Regional National Significance	93.243		–	–	751,640	751,640	–
Pass-through Research Foundation for Mental Hygiene	93.243	140280	64	–	–	64	–
Total Substance Abuse and Mental Health Services Projects of Regional and National Significance			448,847	–	751,640	1,200,487	–
Geriatric Academic Career Awards Department of Health and Human Services:							
Direct Grant – DHHS – Health Resources and Services Administration	93.250		65,663	–	–	65,663	–
Occupational Safety and Health Program:							
Direct Grant – DHHS – Centers for Disease Control and Prevention	93.262		6,174,310	–	–	6,174,310	1,309,439
Direct Grant – DHHS – National Institute for Occupational Safety and Health	93.262		96,841	–	–	96,841	12,591
COVID-19 Pass-through Research Foundation of SUNY	93.262	1170038/2/92119	187,058	–	–	187,058	–
Pass-through New York University	93.262	22-A0-00-006508	49,513	–	–	49,513	–
Pass-through Queens College of CUNY	93.262	CM00009228-00	182,497	–	–	182,497	–
Pass-through Rutgers University	93.262	3149	300,020	–	–	300,020	–
Total Occupational Safety and Health Program			6,990,239	–	–	6,990,239	1,322,030

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Viral Hepatitis Prevention and Control:							
Pass-through Public Health Solutions	93.270	23-IAH-294P,24-IAH-294P,25-IAH-294P	\$ 261,476	\$ —	\$ —	\$ 261,476	\$ 116,595
Alcohol Research Programs:							
Direct Grant – NIH – National Institute on Alcohol Abuse and Alcoholism	93.273	100-1009189-89135,100-1176770-95076,100-1009189-54-10195	1,295,613	—	—	1,295,613	50,889
Pass-through Research Foundation of SUNY	93.273	95076,100-1009189-54-10195	644,139	—	—	644,139	—
Pass-through University of California, San Diego	93.273	KR 704447	931	—	—	931	—
Total Alcohol Research Programs			1,940,683	—	—	1,940,683	50,889
Drug Abuse and Addiction Research Programs:							
Direct Grant – NIH – National Institute of Mental Health	93.279		786,128	—	—	786,128	—
Direct Grant – NIH – National Institute of Neurological Disorders and Stroke	93.279		248,904	—	—	248,904	—
Direct Grant – NIH – National Institute on Drug Abuse	93.279		28,061,307	—	—	28,061,307	2,410,184
Pass-through Albert Einstein College of Medicine	93.279	310258, 312258	567,216	—	—	567,216	—
Pass-through Allen Institute for Brain Science	93.279	2022-0458	1,885,445	—	—	1,885,445	—
Pass-through American Academy of Child and Adolescent Psychiatry	93.279	K12 DA000357	1,208	—	—	1,208	—
Pass-through Cornell University	93.279	223312-3	3,538	—	—	3,538	—
Pass-through New York University	93.279	F2367-27,F1363-13,22-A0-S2-003671,F2367-28	160,184	—	—	160,184	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through Research Foundation for Mental Hygiene	93.279	1 R34 DA055151-01	\$ 48,257	\$ —	\$ —	\$ 48,257	\$ —
Pass-through Research Foundation of CUNY	93.279	CM00011899	302,814	—	—	302,814	—
Pass-through Research Triangle Institute	93.279	7-312-0218755-67098L	326,805	—	—	326,805	—
Pass-through Sanford Burnham Prebys Medical Discovery Institute	93.279	70761-13677-ISMMS	130,694	—	—	130,694	—
Pass-through The Miriam Hospital	93.279	7147185UGK	3,762	—	—	3,762	—
Pass-through University of Kentucky	93.279	3200002286-19-177	23,636	—	—	23,636	—
Pass-through University of Minnesota	93.279	P008325102	41,415	—	—	41,415	—
Pass-through University of Oklahoma	93.279	2023-52	80,115	—	—	80,115	—
Pass-through University of Pennsylvania	93.279	578704	486,227	—	—	486,227	—
Pass-through University of Pittsburgh	93.279	0047895(127700-01), AWD00006009 138057-1	127,139	—	—	127,139	—
Pass-through University of Texas Medical Branch	93.279	16-067/UOSPC0000001442, 2R01DA04062106,23-86216-06	117,923	—	—	117,923	—
Pass-through University of Texas Southwestern	93.279	GMO230118	913,647	—	—	913,647	—
Pass-through University of Washington	93.279	PO00000029,GMO230118	481,157	—	—	481,157	—
Pass-through Wake Forest University	93.279	UWSC15375,UWSC10781	70,914	—	—	70,914	—
Pass-through Weill Cornell University	93.279	21984512011000002163	12,411	—	—	12,411	—
Pass-through Wistar Institute	93.279	192337-2	44,810	—	—	44,810	—
Pass-through Yale University	93.279	25631-04-324	112	—	—	112	—
Total Drug Abuse and Addiction Research Programs		GR111905(CON80002885, CON-80003370, CON-80004350GR120228	34,925,768	—	—	34,925,768	2,410,184

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Discovery and Applied Research for Technological Innovations to Improve Human Health:							
Direct Grant – NIH – National Institute of Biomedical Imaging and Bioengineering	93.286		\$ 1,747,837	\$ –	\$ –	\$ 1,747,837	\$ 320,917
Pass-through Massachusetts General Hospital	93.286	240715	22	–	–	22	–
Pass-through University of Colorado	93.286	FY20.958.001	277	–	–	277	–
Pass-through University of Pennsylvania	93.286	582310	37,929	–	–	37,929	–
Pass-through University of Pittsburgh	93.286	AWD00003848(136584-2	26,460	–	–	26,460	–
Total Discovery and Applied Research for Technological Innovations to Improve Human Health			1,812,525	–	–	1,812,525	320,917
Minority Health and Health Disparities Research:							
Direct Grant – NIH – National Institute of Mental Health	93.307		109,892	–	–	109,892	–
Direct Grant – NIH – National Institute on Minority Health and Health Disparities	93.307		2,522,030	–	–	2,522,030	–
Pass-through Columbia University	93.307	1(GG017383-01),2(GG019054-01),9(GG017227-02)	250,305	–	–	250,305	–
Pass-through NYU Long Island School of Medicine	93.307	22-A0-00-1008788	4,250	–	–	4,250	–
Pass-through Pennsylvania State University	93.307	S002977-DHHS	113,875	–	–	113,875	–
Total Minority Health and Health Disparities Research			3,000,352	–	–	3,000,352	–
Trans-NIH Research Support:							
Direct Grant – NIH – National Institute of Arthritis and Musculoskeletal and Skin Diseases	93.310		335,186	–	–	335,186	–
Direct Grant – NIH – National Institute of Diabetes and Digestive and Kidney Diseases	93.310		1,445,834	–	–	1,445,834	158,564

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Direct Grant – NIH – National Institute of Environmental Health Sciences	93.310		\$ 8,836	\$ –	\$ –	\$ 8,836	\$ –
Direct Grant – NIH – Office of the Director	93.310		10,164,284	–	–	10,164,284	3,963,739
Direct Grant – NIH – National Cancer Institute	93.310		4,649,941	–	–	4,649,941	–
Pass-through Albert Einstein College of Medicine	93.310	1 R01MH094774-01,312228	797	–	–	797	–
Pass-through Ball State University	93.310	21-0124-006	705	–	–	705	–
Pass-through Columbia University	93.310	1(GG014988-01)	117,691	–	–	117,691	–
Pass-through Drexel University	93.310	1008611-3-25001	1,414	–	–	1,414	–
		303002906,303002822,					
Pass-through Duke University	93.310	303002995,303002997,A03-3804	1,702,324	–	–	1,702,324	–
Pass-through Harvard University	93.310	41270632/S9000350	5,257	–	–	5,257	–
Pass-through Northwestern University	93.310	60066242 ISMMS	87,508	–	–	87,508	–
Pass-through University of Arizona	93.310	684052	657,279	–	–	657,279	–
Pass-through University of Chicago	93.310	AWD100257SUB00000236	11,567	–	–	11,567	–
Pass-through University of North Carolina	93.310	5108561	8,503	–	–	8,503	–
Pass-through University of Pittsburgh	93.310	AWD0000481413941210	70,459	–	–	70,459	–
Pass-through University of Washington	93.310	UWSC10692	29,789	–	–	29,789	–
Pass-through University of Wisconsin	93.310	0000000446	661	–	–	661	–
Pass-through Van Andel Research Institute	93.310	40530A-3	784	–	–	784	–
Total Trans-NIH Research Support			19,298,819	–	–	19,298,819	4,122,303
Rare Disorders: Research, Surveillance, Health Promotion, and Education:							
Pass-through National Fragile X Foundation	93.315	5 U01 DD001189-03/PROJECT 5	8,135	–	–	8,135	–

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Paralysis Resource Center:							
COVID-19 Pass-through Christopher & Dana Reeve Foundation	93.325	20-1164-01001-01-PD	\$ 6,732	\$ –	\$ –	\$ 6,732	\$ –
Health Professions Student Loans, Including Primary Care Loans/Loans for Disadvantaged Students	93.342		–	58,904	–	58,904	–
National Center for Advancing Translational Sciences							
Direct Grant – NIH – National Center for Advancing Translational Sciences	93.350		13,014,832	–	–	13,014,832	352,561
Pass-through Brigham and Women's Hospital	93.350	125846	122,342	–	–	122,342	–
Pass-through Duke University	93.350	203-7853	3,729	–	–	3,729	–
Pass-through National Jewish Health	93.350	20115901,20115903,20115902	227,733	–	–	227,733	–
Pass-through New York University	93.350	21-A0-00-106881	17,080	–	–	17,080	–
Total National Center for Advancing Translational Sciences			13,385,716	–	–	13,385,716	352,561
Research Infrastructure Programs							
Direct Grant – NIH – Office of the Director	93.351		238	–	–	238	–
Pass-through Oregon Health & Science University	93.351	1014252-018 MTSINAI	1,321	–	–	1,321	–
Total Research Infrastructure Programs			1,559	–	–	1,559	–

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
21st Century Cures Act – Beau Biden Cancer Moonshot:							
Direct Grant – NIH – National Institute of Diabetes and Digestive and Kidney Diseases	93.353		\$ 677,486	\$ –	\$ –	\$ 677,486	\$ 118,885
Direct Grant – NIH – National Cancer Institute	93.353		2,966,676	–	–	2,966,676	–
Total 21st Century Cures Act – Beau Biden Cancer Moonshot			3,644,162	–	–	3,644,162	118,885
Nursing Research:							
Direct Grant – NIH – National Institute of Nursing Research	93.361		747,669	–	–	747,669	348,608
Pass-through Massachusetts Institute of Technology	93.361	S6044,S6045,S6046	557,446	–	–	557,446	–
Pass-through University of Pittsburgh	93.361	AWD00004758(136886-3	135,379	–	–	135,379	–
Total Nursing Research			1,440,494	–	–	1,440,494	348,608
Nursing Student Loans	93.364		–	213,758	–	213,758	–
21st Century Cures Act – Precision Medicine Initiative:							
Direct Grant – NIH – Office of the Director	93.368		1,585,809	–	–	1,585,809	305,224
Cancer Cause and Prevention Research:							
Direct Grant – NIH – National Cancer Institute	93.393		6,711,432	–	–	6,711,432	1,249,201
Pass-through Be Cancer	93.393	2023-0534	85,739	–	–	85,739	–
Pass-through Cedars-Sinai Medical Cr. Los Angeles	93.393	CSPO425903	28,273	–	–	28,273	–
Pass-through Columbia University	93.393	G15505,2(GG015946-01),E(GG017711-01)	269,504	–	–	269,504	–
Pass-through Fred Hutchinson Cancer Research Center/Cornell University	93.393	0001110398	138,737	–	–	138,737	–

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through Hackensack Meridian Health Hospitals Corp.	93.393	G00017-2229	\$ 37,016	\$ —	\$ —	\$ 37,016	\$ —
Pass-through Massachusetts General Hospital	93.393	237365	659	—	—	659	—
Pass-through Sloan-Kettering Cancer Center	93.393	PO# C22465829	160,017	—	—	160,017	—
Pass-through University of Michigan	93.393	SUBK00014706	1,021	—	—	1,021	—
Pass-through University of Pittsburgh	93.393	AWD00004708 1368061	67,996	—	—	67,996	—
Pass-through University of Texas MD Anderson	93.393	3002278759,3002258264,	71,734	—	—	71,734	—
Pass-through Yale University	93.393	GR107781(CON80002140	86,068	—	—	86,068	—
Total Cancer Cause and Prevention Research			7,658,196	—	—	7,658,196	1,249,201
Cancer Detection and Diagnosis Research:							
Direct Grant – NIH – National Cancer Institute	93.394		8,058,817	—	—	8,058,817	1,393,958
Pass-through Emory University	93.394	A896520	26,324	—	—	26,324	—
		1 U01 CA214114-					
		01,993374,0001045443,0001130560					
		,0001110270,0001109986,00011584					
Pass-through Fred Hutchinson Cancer Research Center	93.394	92	137,675	—	—	137,675	—
Pass-through Research Foundation Of CUNY, Brooklyn College	93.394	CM00003663-00	99,566	—	—	99,566	—
Total Cancer Detection and Diagnosis Research			8,322,382	—	—	8,322,382	1,393,958
Cancer Treatment Research:							
Direct Grant – NIH – National Cancer Institute	93.395		40,739,922	—	—	40,739,922	24,713,435
Pass-through Albert Einstein College of Medicine	93.395	311649	58,151	—	—	58,151	—
Pass-through Cornell University	93.395	PO #C22493811	107,615	—	—	107,615	—
Pass-through Duke University	93.395	303002342	88,457	—	—	88,457	—

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Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through Duke University/University of North Carolina	93.395	303002850/5123528	\$ 239,662	\$ —	\$ —	\$ 239,662	\$ —
Pass-through Ecog-Acrin Medical Research	93.395	U10CA18082008MTSN1A	22,550	—	—	22,550	—
Pass-through Emmes Corporation	93.395	5 UM1 CA121947-16	25,362	—	—	25,362	—
Pass-through Health Research, Inc	93.395	391-01,391-02	49,913	—	—	49,913	—
Pass-through Massachusetts Institute of Technology	93.395	S5930	52,093	—	—	52,093	—
Pass-through National Children's Cancer Foundation	93.395	16723/95002080220-01/	100	—	—	100	—
Pass-through New York University	93.395	19-A0-00-1002964	36,626	—	—	36,626	—
		14-0715-00400-05,14-0715-00200-05,14-0715-00004-05,NRG-					
Pass-through Nrg Oncology Foundation, Inc	93.395	Zamarin-GY11	50,160	—	—	50,160	—
		1013080 MTSINAI,1013080 SWOG_MTSINAIHOSPITAL,					
Pass-through Oregon Health & Science University	93.395	2 U10 CA180888-06/1013080	32,839	—	—	32,839	—
Pass-through Public Health Institute	93.395	AR66789	6,880	—	—	6,880	—
Pass-through Research Foundation of SUNY/Cedars-Sinai Medical Center	93.395	100732/1190302/2/0002149072	44,242	—	—	44,242	—
		CALGB091159/07140019439,AWD					
Pass-through University of Chicago	93.395	103338SUB00001009	80,336	—	—	80,336	—
Pass-through Van Andel Research Institute	93.395	40502A-2	12,287	—	—	12,287	—
Total Cancer Treatment Research			41,647,195	—	—	41,647,195	24,713,435
Cancer Biology Research:							
Direct Grant – NIH – National Heart, Lung, and Blood Institute	93.396		658,573	—	—	658,573	—
Direct Grant – NIH – National Cancer Institute	93.396		15,455,637	—	—	15,455,637	1,129,528

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS) (continued)							
Pass-through Albert Einstein College of Medicine	93.396	310245/P0669769,312061, 312517/PO: P1022969	\$ 68,756	\$ —	\$ —	\$ 68,756	\$ —
Pass-through Dana-Farber Cancer Institute	93.396	11,701,051,170,103	49,188	—	—	49,188	—
Pass-through Duke University	93.396	303004094	742	—	—	742	—
Pass-through H. Lee Moffitt Cancer Center	93.396	10-22849-99-01-G1, 10-22397-99-G1	179,571	—	—	179,571	—
Pass-through Northwestern University	93.396	60059423 ISMMS	36,397	—	—	36,397	—
Pass-through Rutgers University	93.396	0089/527882	7,415	—	—	7,415	—
Pass-through Sloan-Kettering Cancer Center	93.396	BD527215	98,986	—	—	98,986	—
Pass-through The City of Hope	93.396	63339.2009836.669301	28,885	—	—	28,885	—
Pass-through University of Cincinnati	93.396	015546-00002	159,329	—	—	159,329	—
Pass-through University of Pittsburgh	93.396	AWD00002714 134960-1,AWD00004705	258,258	—	—	258,258	—
Total Cancer Biology Research			17,001,737	—	—	17,001,737	1,129,528
Cancer Centers Support Grants:							
Direct Grant – NIH – National Cancer Institute	93.397		3,372,718	—	—	3,372,718	—
Cancer Research Manpower:							
Direct Grant – NIH – National Institute of Mental Health	93.398		38,571	—	—	38,571	—
Direct Grant – NIH – National Cancer Institute	93.398		3,944,632	—	—	3,944,632	6,480
Pass-through Hunter College, CUNY	93.398	CM00008965-00	19,228	—	—	19,228	—
Pass-through University of Colorado	93.398	FY24.1305.001	34,532	—	—	34,532	—
Total Cancer Research Manpower			4,036,963	—	—	4,036,963	6,480

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Cancer Control:							
Pass-through Northwestern University	93.399	60057828 ISMMS	\$ 5,436	\$ –	\$ –	\$ 5,436	\$ –
		NCORP PILOT,NRH-BLANK-					
Pass-through NRG Oncology Foundation, Inc.	93.399	GY10	13,358	–	–	13,358	–
Pass-through University of Texas Medical Branch	93.399	0014326B	5,068	–	–	5,068	–
Total Cancer Control			23,862	–	–	23,862	–
Strengthening Public Health Systems and Services through National Partnerships to Improve and Protect the Nation's Health:							
COVID-19 Pass-through Task Force for Global Health	93.421	NU38OT000316-03-02	93,075	–	–	93,075	–
Pass-through National Network for Public Health Institutes	93.421	G2081 AG-0687	43,205	–	–	43,205	–
Total Strengthening Public Health Systems and Services through National Partnerships to Improve and Protect the Nation's Health			136,280	–	–	136,280	–
ACL National Institute on Disability, Independent Living, and Rehabilitation Research:							
Direct Grant – DHHS – Administration for Community Living	93.433		1,124,080	–	–	1,124,080	69,762
Direct Grant – National Institute of Disability, Independent Living and Rehabilitation Research	93.433		27,928	–	–	27,928	–
Pass-through Indiana University	93.433	8693	3,069	–	–	3,069	–
Total ACL National Institute on Disability, Independent Living, and Rehabilitation Research			1,155,077	–	–	1,155,077	69,762

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Family Violence Prevention and Services/ Sexual Assault/Rape Crisis Services and Supports: Pass-through NYS Office of Children & Family Services	93.497	2202NYFSC6	\$ —	\$ —	\$ 5,164	\$ 5,164	\$ —
Affordable Care Act Initiative to Reduce Avoidable Hospitalizations among Nursing Facility Residents: Pass-through Greater New York Hospital Association Foundation	93.621	CMS-1E1-14-001	100	—	—	100	—
Ending the HIV Epidemic: A Plan for America — Ryan White HIV/AIDS Program Parts A and B: Pass-through Public Health Solutions	93.686	24-DTC-602P/UT8 AH33949	—	—	466,381	466,381	—
Mental and Behavioral Health Education and Training Grants: Direct Grant – DHHS – Health Resources and Services Administration	93.732		679,168	—	—	679,168	—
Opioid STR: Direct Grant – DHHS – Substance Abuse and Mental Health Services Administration	93.788		178,473	—	—	178,473	—
Health Careers Opportunity Program (HCOP) Direct Grant – DHHS – Health Resources and Services Administration	93.822		579,287	—	—	579,287	215,629

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Cardiovascular Diseases Research:							
Direct Grant – NIH – National Heart, Lung, and Blood Institute	93.837		\$ 27,183,069	\$ –	\$ –	\$ 27,183,069	\$ 3,398,270
COVID-19 Pass-through New York University	93.837	22-A0-00-1008035	469,381	–	–	469,381	–
Pass-through Albert Einstein College of Medicine	93.837	312275	93,598	–	–	93,598	–
		OS00000635/313945,OS00000533/313945,OS00000861/313945/313945					
Pass-through Children's Hospital Medical Center/Cincinnati Children's Hospital	93.837	PCGC CO-CHAIR,313945/OS0001612	200,730	–	–	200,730	–
Pass-through City of Hope	93.837	63331.2010702.669301	187,268	–	–	187,268	–
		1(GG019665-01),1 (GG015296-01),3(GG010998-01),2(GG018855-01),2(GG012850-01)	602,059	–	–	602,059	–
Pass-through Columbia University	93.837	227143,212515	92,038	–	–	92,038	–
Pass-through Cornell University	93.837	1040872,0001110491,					
Pass-through Fred Hutchinson Cancer Research Center	93.837	0001129920,0001163904	224,903	–	–	224,903	–
Pass-through Geisinger Clinic	93.837	625650ISMMS01	207,835	–	–	207,835	–
Pass-through Indiana University	93.837	9633,9859	38,970	–	–	38,970	–
		1 U01 HL23336-01/225715,243119,					
Pass-through Massachusetts General Hospital	93.837	230000/239330,235115	302,489	–	–	302,489	–
Pass-through New England Research Institute	93.837	10253228-002	6,152	–	–	6,152	–
		18-A0-00-001801,22-A0-00-					
Pass-through New York University	93.837	1007792,17-A1-00-005849-01	87,430	–	–	87,430	–
Pass-through Ohio State University	93.837	5UH3HL140144-03	1,433	–	–	1,433	–

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through University of Arizona/Georgia State University	93.837	728446/SP00013920-01 OS00000778,OS00001504,	\$ 92,157	\$ —	\$ —	\$ 92,157	\$ —
Pass-through University of Miami	93.837	OS00001396	144,310	—	—	144,310	—
Pass-through University of North Carolina	93.837	5120339,5110503,5127669 CNVA00062504-1324381,	60,402	—	—	60,402	—
Pass-through University of Pittsburgh	93.837	AWD00000026 132575-2	39,935	—	—	39,935	—
Pass-through University of Texas	93.837	SA0000632	73,134	—	—	73,134	—
Pass-through University of Utah	93.837	10067841-01-ICAHN	239,909	—	—	239,909	—
Pass-through University of Virginia	93.837	GB10709,AWD-003268,GR100418 VUMC116222,VUMC114909,VUM	109,681	—	—	109,681	—
Pass-through Vanderbilt University	93.837	C98562,VUMC80816	232,302	—	—	232,302	—
Pass-through Washington University	93.837	WU-22-0003,WU-23-0596	13,871	—	—	13,871	—
Pass-through Yale University	93.837	CON-80005071GR123684	2,891	—	—	2,891	—
Total Cardiovascular Diseases Research			30,705,947	—	—	30,705,947	3,398,270
Lung Diseases Research:							
Direct Grant – NIH – National Heart, Lung, and Blood Institute	93.838		6,715,163	—	—	6,715,163	698,353
COVID-19 Pass-through Cornell University	93.838	220367-3	26,889	—	—	26,889	—
COVID-19 Pass-through New York University	93.838	ADU-11-21,21-A0-00-1006779 5-312-0217571-66037L,5-312- 0217571-66037L,5-312-0217571-	3,600,568	—	—	3,600,568	—
COVID-19 Pass-through Research Triangle Institute	93.838	66037L	895,078	—	—	895,078	—
Pass-through Albert Einstein College of Medicine	93.838	310750/,333483	57,339	—	—	57,339	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS) (continued)							
Pass-through Columbia University	93.838	3(GG017998-01), 4(GG017828-01) 383001137,383001465,383001279/3 83001278/WBSE: 383000909; SPS:	\$ 39,587	\$ —	\$ —	\$ 39,587	\$ —
Pass-through Duke University	93.838	280176/383001980/383001739	1,164,196	—	—	1,164,196	—
Pass-through Kaiser Foundation	93.838	RNG211577-ISMMMS AUT-06-21,PATHO-PH1-SUB 15 22,PATHO-PH2-SUB 10 23,ADU-	9,244	—	—	9,244	—
Pass-through New York University	93.838	11-21,06-B-01	645,448	—	—	645,448	—
Pass-through Research Foundation of SUNY	93.838	100-1177856-95503 8-312-0216670-66931L,30-312- 0217571-66347,52-312-0217571-	20,988	—	—	20,988	—
Pass-through Research Triangle Institute	93.838	67001	1,194,704	—	—	1,194,704	—
Pass-through Society of Critical Care Medicine	93.838	1 R01 HL162954	21,649	—	—	21,649	—
Pass-through Stanford Junior University	93.838	63064464-270244	63,940	—	—	63,940	—
Pass-through University of North Carolina	93.838	5125917-H3MNTSINAI,5109053	134,105	—	—	134,105	—
Pass-through Virginia Polytechnic Institute and State University	93.838	412762-19C70	81,698	—	—	81,698	—
Total Lung Diseases Research			14,670,596	—	—	14,670,596	698,353
Blood Diseases and Resources Research:							
Direct Grant – NIH – National Heart, Lung, and Blood Institute	93.839		5,965,890	—	—	5,965,890	1,308,778
Pass-through Beth Israel Medical Deaconess Hospital	93.839	GRT66780	13,649	—	—	13,649	—
Pass-through Children's Hospital of Philadelphia	93.839	GRT-00000907	4,354	—	—	4,354	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through National Marrow Donor Program	93.839	RFQ #C21-0025/1919032,C10205	\$ 116,115	\$ —	\$ —	\$ 116,115	\$ —
Pass-through NYU Grossman School of Medicine	93.839	21-A2-00-1004666	2,625	—	—	2,625	—
Pass-through Rockefeller University	93.839	SUB00000254	244,739	—	—	244,739	—
Pass-through University of California, Irvine	93.839	2023-1929	101,454	—	—	101,454	—
Pass-through University of Maryland	93.839	20798	62,296	—	—	62,296	—
Pass-through University of Michigan	93.839	1102	2,334	—	—	2,334	—
Pass-through University of Virginia	93.839	GB10798	66	—	—	66	—
Total Blood Diseases and Resources Research			6,513,522	—	—	6,513,522	1,308,778
Translation and Implementation Science Research for							
Heart, Lung, Blood Diseases, and Sleep Disorders							
Pass-through University of Utah	93.840	10057254-03-ICAH	44,230	—	—	44,230	—
Arthritis, Musculoskeletal and Skin Diseases Research:							
Direct Grant – NIH – National Human Genome Research Institute	93.846		51,935	—	—	51,935	—
Direct Grant – NIH – National Institute for Allergy and Infectious Diseases	93.846		620,183	—	—	620,183	—
Direct Grant – NIH – National Institute of Arthritis and Musculoskeletal and Skin Diseases	93.846		9,940,881	—	—	9,940,881	533,375
		1(GG012250-01)/G12264,1(GG017737-01),1(GG011317-01),1(GG012738-01)/SAPO G12013/,2(GG015978-01),2(GG017881-01)	85,781	—	—	85,781	—
Pass-through Columbia University	93.846		15,842	—	—	15,842	—
Pass-through Cornell University	93.846	213967					

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through Feinstein Institute for Medical Research	93.846	AWD00001214-MSSM	\$ 2,386	\$ —	\$ —	\$ 2,386	\$ —
Pass-through New York University	93.846	21-A0-00-1006836	261,258	—	—	261,258	—
Pass-through Rush University	93.846	5P30AR079206 02-Pilot Award2	9,197	—	—	9,197	—
Pass-through University of Pennsylvania	93.846	572781	9,446	—	—	9,446	—
Total Arthritis, Musculoskeletal and Skin Diseases Research			10,996,909	—	—	10,996,909	533,375
Diabetes, Digestive, and Kidney Diseases Extramural Research:							
Direct Grant – NIH – National Institute of Diabetes and Digestive and Kidney Diseases	93.847		40,320,119	—	—	40,320,119	3,866,128
Direct Grant – NIH – National Institute on Aging	93.847		45,535	—	—	45,535	—
COVID-19 Pass-through University of Minnesota	93.847	N009112302	5,492	—	—	5,492	—
Pass-through Adventist Health System	93.847	1329760-MSSM	230,855	—	—	230,855	—
		31220B,31220A,312244,31220C,312363,312545,31165E,31607H,31249E ,31219H,333702	948,161	—	—	948,161	—
Pass-through Albert Einstein College of Medicine	93.847			—	—		—
Pass-through Ann & Robert H. Lurie Children's Hospital of Chicago	93.847	901628-MOUNT SINAI	8,447	—	—	8,447	—
Pass-through Baylor College of Medicine	93.847	2 P01 DK056492-17A1	1,151	—	—	1,151	—
		PO#GENFD0001888480,		—	—		—
Pass-through Boston Children's Hospital	93.847	PO#GENFD0001913899	246,960	—	—	246,960	—
Pass-through City of Hope	93.847	64329.2011887.669302	9,540	—	—	9,540	—
		5(GG019638-01),1(GG017456-01),1(GG017389-01)	197,798	—	—	197,798	—
Pass-through Columbia University	93.847			—	—		—
Pass-through Cornell University	93.847	85317-11148,2213307-2	14,332	—	—	14,332	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through Duke University	93.847	303001194	\$ 8,184	\$ —	\$ —	\$ 8,184	\$ —
Pass-through Harvard University	93.847	168037-5115306	188,242	—	—	188,242	—
Pass-through Johns Hopkins University	93.847	2004008228/GRT-00003464	16,021	—	—	16,021	—
Pass-through Massachusetts General Hospital	93.847	238587	117,879	—	—	117,879	—
Pass-through Northwestern University	93.847	60050284 ISMMS	15,175	—	—	15,175	—
Pass-through Research Foundation of SUNY	93.847	89194/2/1163728,1 R01 DK112984-01	137,825	—	—	137,825	—
Pass-through Rockefeller University	93.847	SUB00000384,SUB000000449	189,428	—	—	189,428	—
Pass-through Saint Louis University	93.847	A01697-14696	338,519	—	—	338,519	—
Pass-through University of Alabama	93.847	000524405-SC002,000537127-SC003	607,993	—	—	607,993	—
Pass-through University of California, Davis	93.847	A21-3549-S002	829	—	—	829	—
Pass-through University of Florida	93.847	TRIALNET 3487	11,982	—	—	11,982	—
Pass-through University of Maryland	93.847	21661	77,207	—	—	77,207	—
Pass-through University of Michigan	93.847	SUBK00022342,UWSC10092,SUB K00016268	226,258	—	—	226,258	—
Pass-through University of Nebraska	93.847	34-5170-2097-001	17,611	—	—	17,611	—
Pass-through University of Pennsylvania	93.847	574566	2,658	—	—	2,658	—
Pass-through University of Pittsburgh	93.847	9015304(131759-1),AWD00002684(135261),AWD00005603(1376121),AWD00008104(8014301),AWD00002999(135642-2),AWD00004225 136883-1,AWD00004225 138386-1	1,195,613	—	—	1,195,613	—
Pass-through University of Southern California	93.847	SCON-00006292,SCON-00004032	33,508	—	—	33,508	—
Pass-through University of Texas	93.847	169763/169761,171929/171926	52,202	—	—	52,202	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through University of Virginia	93.847	GR104121.SUB00001154	\$ 74,785	\$ —	\$ —	\$ 74,785	\$ —
Pass-through Vanderbilt University	93.847	VUMC 74635,VUMC80652,	155,492	—	—	155,492	—
Pass-through Weill Cornell University	93.847	VUMC104236,VUMC126265	87,281	—	—	87,281	—
Pass-through Yale University	93.847	204852/212416/223325	218,927	—	—	218,927	—
Total Diabetes, Digestive, and Kidney Diseases Extramural Research			45,802,009	—	—	45,802,009	3,866,128
Extramural Research Programs in the Neurosciences and Neurological Disorders:							
Direct Grant – NIH – National Cancer Institute	93.853		63,363	—	—	63,363	—
Direct Grant – NIH – National Institute of Neurological Disorders and Stroke	93.853		24,197,449	—	—	24,197,449	4,856,660
Direct Grant – NIH – National Institute on Aging	93.853		66,492	—	—	66,492	—
Direct Grant – NIH – National Institute on Drug Abuse	93.853		5,742	—	—	5,742	—
Pass-through Allen Institute	93.853	2023-0606	270,007	—	—	270,007	—
Pass-through Boston University	93.853	4500003974,4500005055,45000047	395,297	—	—	395,297	—
Pass-through Buck Institute for Research on Age	93.853	14	108,976	—	—	108,976	—
Pass-through Carnegie Mellon University	93.853	SA15008-MS	476,570	—	—	476,570	—
Pass-through Children's Hospital, Boston	93.853	1090697,1090736-463768,1090784-477502	71,735	—	—	71,735	—
Pass-through Columbia University	93.853	GENFD0001773961,GENFD00021	46,126	—	—	46,126	—
Pass-through Cornell University	93.853	17225	2,719	—	—	2,719	—
		2(GG017706-01)					
		170051					

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through Indiana University	93.853	10094-MS	\$ 152,747	\$ —	\$ —	\$ 152,747	\$ —
Pass-through John Hopkins University	93.853	2005524120	9,332	—	—	9,332	—
		244025,U01NS114001,235486,1					
		OT2 NS122680-					
		01,239655,239654,235400					
Pass-through Massachusetts General Hospital	93.853	,243984,244090	1,017,650	—	—	1,017,650	—
Pass-through Massachusetts Institute of Technology	93.853	S5550	280,925	—	—	280,925	—
Pass-through Mayo Clinic Jacksonville	93.853	ICA-224063-09	106	—	—	106	—
		21A0-00-1005982,21-A1-00-					
		1005982,23-A1-00-1008936,23-A0-					
Pass-through New York University	93.853	00-1009359	570,404	—	—	570,404	—
Pass-through Ohio State University	93.853	GR125695SPC100000585	245,661	—	—	245,661	—
Pass-through Pennsylvania State University	93.853	S004882-DHHS,S001403-DHHS	78,584	—	—	78,584	—
Pass-through Sloan-Kettering Cancer Center	93.853	BD526923	10,804	—	—	10,804	—
Pass-through Stanford University	93.853	62790670-208926	446,707	—	—	446,707	—
Pass-through University of California, Davis	93.853	A21-1324-S032	275,307	—	—	275,307	—
Pass-through University of California, Los Angeles	93.853	1580 G ZD691	158,248	—	—	158,248	—
		012043-135724,1 U01 NS110772					
		01-02,1U01NS110772-01,012044-					
Pass-through University of Cincinnati	93.853	135724,013888-135724	50,775	—	—	50,775	—
Pass-through University of Miami	93.853	OS00000072	208,612	—	—	208,612	—
Pass-through University of Michigan	93.853	SUBK00020153	195,396	—	—	195,396	—
Pass-through University of Pennsylvania	93.853	578474,500320	270,443	—	—	270,443	—
Pass-through University of South Carolina	93.853	A23-0066-S004	88,824	—	—	88,824	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through University of Southern California	93.853	100922174,SCON-00003845,SCON-00005078	\$ 347,029	\$ —	\$ —	\$ 347,029	\$ —
Pass-through University of Texas at Dallas	93.853	1806761	303	—	—	303	—
Pass-through University of Texas Health Science	93.853	SA0000222	365,011	—	—	365,011	—
Pass-through University of Texas Southwestern	93.853	GMO211016	110,289	—	—	110,289	—
Pass-through University of Utah	93.853	U01 NS120901	37,094	—	—	37,094	—
Pass-through University of Wisconsin	93.853	0000003433	216,091	—	—	216,091	—
Pass-through Wistar Institute	93.853	25810-04-381	125,133	—	—	125,133	—
Total Extramural Research Programs in the Neurosciences and Neurological Disorders			30,965,951	—	—	30,965,951	4,856,660
Allergy and Infectious Diseases Research:							
Direct Grant – NIH – National Institute For Allergy and Infectious Diseases	93.855		40,243,250	—	—	40,243,250	8,893,541
COVID-19 Pass-through John Hopkins University	93.855	2005285928	21,982	—	—	21,982	—
COVID-19 Pass-through Palo Alto Veterans Institute for Research	93.855	MAL0008-01	126,044	—	—	126,044	—
COVID-19 Pass-through University of Colorado	93.855	FY22.1050.005	64,153	—	—	64,153	—
		FY21ITN379,FY23ITN346,FY17ITN232,FY21ITN232,FY22ITN232,FY23ITN232,FY23ITN480,FY22ITN583,FY22ITN561,BRI#					
Pass-through Benaroya Research Institute	93.855	FY19ITN319,FY22ITN589	849,977	—	—	849,977	—
Pass-through Boston Children's Hospital	93.855	GENFD0001903588	111,564	—	—	111,564	—
		0002117571,CSPO538312,					
Pass-through Cedars-Sinai Medical Center	93.855	0002319139	571,041	—	—	571,041	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through Children's Hospital Medical Center	93.855	OS00000721/308342,					
Pass-through Children's Hospital of Philadelphia	93.855	OS00000798/314219	\$ 962,969	\$ —	\$ —	\$ 962,969	\$ —
Pass-through Columbia University	93.855	GRT-00002464	32,204	—	—	32,204	—
Pass-through Drexel University	93.855	3(GG019684-01)	20,448	—	—	20,448	—
Pass-through Emory University	93.855	900326	34,715	—	—	34,715	—
Pass-through Georgia State University	93.855	A887391.A683372	611,915	—	—	611,915	—
Pass-through Harvard University	93.855	SP00013377-02,SP00012813	1,915	—	—	1,915	—
Pass-through Hospital for Special Surgery	93.855	160510-1854	30	—	—	30	—
Pass-through Immune Deficiency Foundation	93.855	001	84,971	—	—	84,971	—
Pass-through Indiana University	93.855	2 U24 AI086037-10	1,374	—	—	1,374	—
		IN4688966MS,8985 MSSM	115,986	—	—	115,986	—
		2003638041,2004124856,20059093					
		04,2006074489,2004247207,200497					
		4093,2006476411,2006476454,2006					
Pass-through Johns Hopkins University	93.855	476453,2004199184	1,705,121	—	—	1,705,121	—
		20013-03-171-284,200012-03-134-					
		284,20012-02-164-284,20012-01-					
Pass-through La Jolla Institute for Allergy	93.855	134-284	264,550	—	—	264,550	—
		1 U01 AL136816 01-					
Pass-through Massachusetts General Hospital	93.855	05,241954,232560,239761,243015	681,087	—	—	681,087	—
Pass-through National Jewish Health	93.855	20123804-Mt Sinai	26,734	—	—	26,734	—
		19-A1-00-1001999,23-A0-00-					
Pass-through New York University	93.855	1010606,11-02237/101790	86,206	—	—	86,206	—
		60060177 ISMMS,60062888					
		ISMMS,60046387					
Pass-through Northwestern University	93.855	ICAHN,60063098 ISM	911,901	—	—	911,901	—
Pass-through Research Foundation of CUNY	93.855	CM00012826-00	78,694	—	—	78,694	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS) (continued)							
		1 R01 AI143295-01A1, 1R01H145870-01,SUB00000247, SUB00000302,SUB00000301, SUB00000451,SUB00000246	\$ 661,739	\$ —	\$ —	\$ 661,739	\$ —
Pass-through Rockefeller University	93.855						
Pass-through Rutgers University	93.855	SUB00002786	295,118	—	—	295,118	—
Pass-through Sanford Burnham Prebys Medical Discovery Institute	93.855	60088-12934-MSSM/ 5-54729,5-54813,5-54836,5- 54828,5-54847	2,260	—	—	2,260	—
Pass-through Scripps Research Institute	93.855	REQ. #2018865,MSKSUB00000090,MSK	537,320	—	—	537,320	—
Pass-through Sloan-Kettering Cancer Center	93.855	SUB00000165	2,029,007	—	—	2,029,007	—
Pass-through St Jude Children's Research Hospital	93.855	112525010-7883639,GR-0003551- 003766	858,452	—	—	858,452	—
Pass-through The Jackson Laboratory	93.855	210310-0320-07,210311-0120- 03,210310-0221-15,210384-1222- 03,2 U19 AI142733/210310-0225- 08,210310-0225-09,210310-0225-10	285,738	—	—	285,738	—
Pass-through The Miriam Hospital	93.855	7147154RV,7147200RCV	118,631	—	—	118,631	—
Pass-through University of Alabama	93.855	000520244-SP008	148,754	—	—	148,754	—
Pass-through University of Arizona	93.855	694508	650,603	—	—	650,603	—
Pass-through University of California, Davis	93.855	A23-0293-S002	61,740	—	—	61,740	—
Pass-through University of California, Irvine	93.855	2023-1984,2023-1910	193,490	—	—	193,490	—
Pass-through University of California, Los Angeles	93.855	1560 B LA618	126,498	—	—	126,498	—
Pass-through University of California, San Diego	93.855	9251SC	39,889	—	—	39,889	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through University of California, San Francisco	93.855	14624SC,9455SC,13618SC,13620S					
Pass-through University of Chicago	93.855	C,13631SC,14168SC	\$ 1,523,719	\$ —	\$ —	\$ 1,523,719	\$ —
Pass-through University of Cincinnati	93.855	AWD102905	16,930	—	—	16,930	—
	93.855	011266-135724,015688-00002	154,540	—	—	154,540	—
		22010,87592-Z0296201,108346-					
Pass-through University of Maryland	93.855	Z0447205,143820-Z0733202	106,522	—	—	106,522	—
Pass-through University of Michigan	93.855	SUBK00010468,SUBK00015137	545,607	—	—	545,607	—
Pass-through University of Pennsylvania	93.855	569199,577385	115,915	—	—	115,915	—
Pass-through University of Pittsburgh	93.855	AWD000059944(137861)	25,022	—	—	25,022	—
		SUB00000604,SUB00000603/GR53					
Pass-through University of Rochester	93.855	3691	613,990	—	—	613,990	—
		21-85494-01,21-85493-01,23-					
Pass-through University of Texas	93.855	85493-03,24-86926-02	552,931	—	—	552,931	—
Pass-through University of Wisconsin	93.855	0000001302,203,0000001550	352,665	—	—	352,665	—
Pass-through Vanderbilt University	93.855	OSA00000404,VUMC116785	123,884	—	—	123,884	—
		WU-24-0046,WU-22-0369,WU-24-					
		0205,WU-23-0539,WU-23-					
Pass-through Washington University	93.855	0538,WU-23-0572,WU-23-0541	1,299,985	—	—	1,299,985	—
Total Allergy, Immunology and Transplantation Research Program			59,049,780	—	—	59,049,780	8,893,541
Biomedical Research and Research Training:							
Direct Grant – NIH – National Institute of General Medical Sciences	93.859		14,932,096	—	—	14,932,096	387,530
Pass-through Northwestern University	93.859	60040806 ICAHN	8,094	—	—	8,094	—
Pass-through St Jude Children's Research Hospital	93.859	112563010-7911627	84,730	—	—	84,730	—
Pass-through University of Toledo	93.859	N-2020-31	17,508	—	—	17,508	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through Vanderbilt University	93.859	OSA00000432	\$ 9,083	\$ —	\$ —	\$ 9,083	\$ —
Pass-through Washington State University	93.859	127105- WSU001183	261,527	—	—	261,527	—
Total Biomedical Research and Research Training			15,313,038	—	—	15,313,038	387,530
Child Health and Human Development Extramural Research:							
Direct Grant – NIH – National Institute of Child Health & Human Development	93.865		10,875,802	—	—	10,875,802	1,415,873
Pass-through Boston Children's Hospital	93.865	GENFD0002445051	30,931	—	—	30,931	—
Pass-through Children's Research Institute	93.865	30005547-10,30005550-07	59,068	—	—	59,068	—
Pass-through Columbia University	93.865	8(GG017754-01)	8,848	—	—	8,848	—
Pass-through Emory University	93.865	A830766	403,258	—	—	403,258	—
Pass-through Michigan State University	93.865	RC115789MSSM	491,538	—	—	491,538	—
Pass-through New York University	93.865	S0684-01	3,800	—	—	3,800	—
Pass-through Research Foundation of CUNY	93.865	CM00004996	6,730	—	—	6,730	—
Pass-through SUNY at Buffalo	93.865	R1305788	7,951	—	—	7,951	—
Pass-through University of Florida	93.865	SUB00001725	14,501	—	—	14,501	—
Pass-through University of Georgia	93.865	SUB00002798	270	—	—	270	—
Pass-through University of Pittsburgh	93.865	AWD00003188	12,049	—	—	12,049	—
Pass-through Wake Forest University	93.865	19874510811000001766	28,878	—	—	28,878	—
Pass-through Yale University	93.865	GR108780	498	—	—	498	—
Total Child Health and Human Development Extramural Research			11,944,122	—	—	11,944,122	1,415,873
Aging Research:							
Direct Grant – NIH – National Institute on Aging	93.866		61,684,159	—	—	61,684,159	9,892,491
Pass-through Albert Einstein College of Medicine	93.866	31166B,311868,311712	595,132	—	—	595,132	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through Arizona State University	93.866	ASUB00000302	\$ 1,419	\$ —	\$ —	\$ 1,419	\$ —
Pass-through Banner Health	93.866	0435-06-132021	9,458	—	—	9,458	—
Pass-through Boston University	93.866	4500004058,4500004925	221,179	—	—	221,179	—
Pass-through Brigham and Women's Hospital	93.866	125768	31,523	—	—	31,523	—
Pass-through Bronx Veterans Medical Res Inc	93.866	5 R01 AG060341-03-04	2,052	—	—	2,052	—
Pass-through Brown University	93.866	00001692,00002079	380,048	—	—	380,048	4,622
Pass-through California Institute of Technology	93.866	S536572	166,936	—	—	166,936	—
Pass-through California Pacific Medical Center	93.866	280201030-S315	926,508	—	—	926,508	—
Pass-through Case Western Reserve University	93.866	RES515739	27,394	—	—	27,394	—
		2(GG012937-01)/G12221/,5 (GG013437-02),3(GG012955-05),5(GG013437-01),G19424					
Pass-through Columbia University	93.866	6(GG017543-05)	276,501	—	—	276,501	—
Pass-through Duke University	93.866	303-001875	64,625	—	—	64,625	—
Pass-through Fordham University	93.866	FORD0065-30315	620,615	—	—	620,615	—
Pass-through John Hopkins University	93.866	2004956780	38,380	—	—	38,380	—
Pass-through Kennedy Krieger, Inc.	93.866	113581-1124-01	45,940	—	—	45,940	—
Pass-through Massachusetts Institute of Technology	93.866	S5077	354,226	—	—	354,226	—
Pass-through Mayo Clinic Jacksonville	93.866	IMS-284424	721,616	—	—	721,616	—
Pass-through Mayo Clinic Rochester	93.866	ICA-263134-01	4,966	—	—	4,966	—
Pass-through New York Stem Cell Foundation	93.866	1001-001	248,571	—	—	248,571	—
		19A0001003584,21-A1-00-1004969,22-A0-00-1009111,23-A0-00-1010128					
Pass-through New York University	93.866	24-A1-00-1011311/ CON80004266 GR119221	585,455	—	—	585,455	—
Pass-through New York University/Yale University	93.866		36,255	—	—	36,255	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through Northern California Institute for Research & Education	93.866	STE2196-09, WEI2990-14 60055920 ISMMS,60059638	\$ 1,179,566	\$ —	\$ —	\$ 1,179,566	\$ 29,925
Pass-through Northwestern University	93.866	ISM,60062199 ISM	253,360	—	—	253,360	—
Pass-through Rensselaer Polytechnic Institute	93.866	A24-0024-001,A19-0033-S001	10,002	—	—	10,002	—
Pass-through Rutgers University	93.866	3301	22,584	—	—	22,584	—
Pass-through Sage Bionetworks	93.866	AMPT2017MSSM	2,138	—	—	2,138	—
Pass-through Stanford University	93.866	62019200-116920	1,478	—	—	1,478	—
Pass-through Trustees of Boston University	93.866	4500004601	31,496	—	—	31,496	—
Pass-through University of California, Davis	93.866	A20-0263-S001,A19-1025-S001,A22-0257-S001, A21-1701-S001	78,285	—	—	78,285	—
Pass-through University of California, San Diego	93.866	37336570-001/37336570/37336570-004,KR 704802,SCON-00004458,SCON-00005652,SCON-00005698	790,749	—	—	790,749	—
Pass-through University of California, San Francisco	93.866	14025SC,14093SC,14526SC,14417 SC	1,627,561	—	—	1,627,561	—
Pass-through University of Colorado	93.866	FY23.1204.002	147,541	—	—	147,541	—
Pass-through University of Miami	93.866	OS00000575	183,438	—	—	183,438	—
Pass-through University of Minnesota	93.866	N011256801	205,852	—	—	205,852	—
Pass-through University of North Carolina	93.866	5125419	119,442	—	—	119,442	—
Pass-through University of Pennsylvania	93.866	587757,579279	201,086	—	—	201,086	—
Pass-through University of Rochester	93.866	SUB00001045	10,842	—	—	10,842	—
Pass-through University of Southern California	93.866	105637233,140789934,129644062,1 29644591,SCON-0004791	271,099	—	—	271,099	—
Pass-through University of Washington	93.866	762186/UWSC13294	24,223	—	—	24,223	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through University of Wisconsin	93.866	0000000689,0000003354, 0000001238	\$ 636,669	\$ —	\$ —	\$ 636,669	\$ —
Pass-through University of Missouri System	93.866	C00081736-1,C00090131-1	412,885	—	—	412,885	—
Pass-through Visiting Nurse Service of NY	93.866	4664 2023 MSSM 142-101720-	11,312	—	—	11,312	—
		117741,25283366411000001795,11 0-101720-551083,1435-33664-					
Pass-through Wake Forest University	93.866	110000006 WU-21-323,WU-21-45,WU-25-	207,745	—	—	207,745	—
Pass-through Washington University	93.866	0004,WU-15-336	98,073	—	—	98,073	—
Pass-through Yale University	93.866	CON-80002962 GR11256	1,031	—	—	1,031	—
Total Aging Research			73,571,415	—	—	73,571,415	9,927,038
Vision Research:							
Direct Grant – NIH – National Eye Institute	93.867		2,461,929	—	—	2,461,929	556,346
Pass-through Massachusetts Eye and Ear Infirmary	93.867	530913	42,343	—	—	42,343	—
Pass-through Penn State University College of Medicine	93.867	MTSIEY023533	9,232	—	—	9,232	—
Pass-through Research Foundation Of SUNY	93.867	1160942 – 87961	33,989	—	—	33,989	—
Pass-through Schepens Eye Research Institute	93.867	533387	9,694	—	—	9,694	—
Pass-through University of California, Los Angeles	93.867	095453-17527,0125 G YA540	5,196	—	—	5,196	—
Pass-through University of Maine	93.867	UMS2236	191,099	—	—	191,099	—
Pass-through University of Tennessee	93.867	24-2823	38,126	—	—	38,126	—
Total Vision Research			2,791,608	—	—	2,791,608	556,346

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Medical Library Assistance:							
Direct Grant – NIH – National Library of Medicine	93.879		\$ 1,035,959	\$ –	\$ –	\$ 1,035,959	\$ 39,589
National Bioterrorism Hospital Preparedness Program:							
Pass-through Health Research Inc.		1989-19, 1989-20	–	–	36,500	36,500	–
Pass-through Public Health Solutions	93.889	19-MSH 01-02/P076622 /AMD 3/PO76623/AMD 5/ PO76623	–	–	149,285	149,285	–
Total National Bioterrorism Hospital Preparedness Program			–	–	185,785	185,785	–
HIV Emergency Relief Project Grants:							
Pass-through Public Health Solutions	93.914	24-CCR-602/H89HA00015/24- SCX-602P/6 H89 HA00015-31/09- MCM-110/16-HRM-110/19-CCR- 110/18-CCR-175/20-FNS-175	–	–	4,489,369	4,489,369	–
HIV Care Formula Grants:							
Pass-through New York State Department of Health	93.917	C36282GG	–	–	181,009	181,009	–
Grants to Provide Outpatient Early Intervention Services with Respect to HIV Disease:							
Grants to Provide Outpatient Early Intervention Services with Respect to HIV Disease	93.918		–	–	1,595,732	1,595,732	–

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS) (continued)							
Pass-through New York University	93.918	14-A0-00-003512, 21-A0-00-1000300	\$ 94,407	\$ —	\$ —	\$ 94,407	\$ —
Total Grants to Provide Outpatient Early Intervention Services with Respect to HIV Disease			94,407	—	1,595,732	1,690,139	—
Ryan White HIV/AIDS Dental Reimbursement	93.924		—	—	996,165	996,165	—
Special Projects of National Significance	93.928		—	—	339,951	339,951	—
HIV Prevention Activities Health Department Based							
Pass-through Public Health Solutions	93.940	6 NU62 PS924626-02-01/13-SBH-602/21-SBH-602PL/11-HTR-110	—	—	355,580	355,580	—
Block Grants for Community Mental Health Services							
Pass-through New York State Department of Health	93.958	OMH01-C22157GG-3650000/C23230GG	—	—	137,291	137,291	—
Pass-through NY State Office of Mental Health	93.958	PO21640/PO22143/P022179	—	—	5,600	5,600	—
Total Block Grants for Community Mental Health Services			—	—	142,891	142,891	—
Block Grants for Prevention and Treatment of Substance Abuse:							
Pass-through New York State Office of Addiction Services and Supports	93.959	CFA0348MDS	—	—	29,933	29,933	—

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Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
The Zika Health Care Services Program:							
Direct Grant – NIH – National Institute on Aging	93.966		192,333	–	–	192,333	
International Research and Research Training:							
Direct Grant – NIH – Fogarty International Center	93.989		659,458	–	–	659,458	219,657
Maternal and Child Health Services Block Grant to the States:							
Pass-through NYS Department of Health	93.994	DOH01-C36785GG-3450000	33,392	–	–	33,392	–
Pass-through New York State Department of Health	93.994	C022478/C32404GG/C32404GG/C36282GG/C37186GG	–	–	738,229	738,229	–
Total Maternal and Child Health Services Block Grant to the States			33,392	–	738,229	771,621	–
Direct Contract – DHHS – Food & Drug Administration	93.RD		517	–	–	517	
Direct Contract – DHHS – National Institute for Occupational Safety and Health	93.RD		42,447,863	–	–	42,447,863	–
Direct Contract – General Services Administration	93.RD		112,722	–	–	112,722	–
Direct Contract – NIH – National Cancer Institute	93.RD		4,362	–	–	4,362	–
Direct Contract – NIH – National Institute for Allergy and Infectious Diseases	93.RD		35,185,079	–	–	35,185,079	18,903,727
Direct Contract – NIH – National Institute on Drug Abuse	93.RD		6,093,795	–	–	6,093,795	–
Pass-through Children’s Research Institute	93.RD	30002363-13-03	7,210	–	–	7,210	–
Pass-through Georgia Institute of Technology	93.RD	C8H9CNG1VBD9	2,834,840	–	–	2,834,840	–
Pass-through Institute for Clinical Research	93.RD	M77-MS-081-2011-10	70,685	–	–	70,685	–

Mount Sinai Health System, Inc.

Schedule of Expenditures of Federal Awards (continued)

Federal Department Program Title	Assistance Listing No.	Pass-Through Entity Identifying Number	Research and Development Cluster	Student Financial Assistance Cluster	Other Expenditures	Total Expenditures	Expenditures to Subrecipients
Department of Health and Human Services (DHHS)							
(continued)							
Pass-through International AIDS Vaccine Initiative	93.RD	HHSN272201800010I 21X092F3/75N91019D00024,21X0 92,21X092F1,HHSN261201500003I	\$ 399	\$ —	\$ —	\$ 399	\$ —
Pass-through Leidos Biomedical Research	93.RD	/21X093Q	2,491,505	—	—	2,491,505	—
Pass-through Northeastern University	93.RD	80ARC023CA005/505282-78050	51,347	—	—	51,347	—
Pass-through Oregon Health & Science University	93.RD	1004086-001 Mount Sinai S1900I SUB00002166,SUB00002166-	2,265	—	—	2,265	—
Pass-through University of Georgia	93.RD	3A,SUB00002166-3U	5,279	—	—	5,279	—
Pass-through University of Minnesota	93.RD	P010232203	784,900	—	—	784,900	—
Pass-through University of Nebraska	93.RD	36-5420-2057-201	313,130	—	—	313,130	—
Pass-through Wayne State University	93.RD	WSU22012-A2	155,300	—	—	155,300	—
Total from Contracts			90,561,198	—	—	90,561,198	18,903,727
Total Department of Health and Human Services			662,497,969	272,662	13,001,321	675,771,952	103,656,156
Department of Homeland Security							
Disaster Grants – Public Assistance (Presidentially Declared Disasters)							
COVID-19 Pass-through New York State Division of Homeland Security and Emergency Services	97.036	UNKNOWN	—	—	205,524,270	205,524,270	—
Total Department of Homeland Security			—	—	205,524,270	205,524,270	—
U.S. Agency for International Development (USAID)							
USAID Foreign Assistance for Programs Overseas							
Pass-through Brigham and Women's Hospital	98.001	122066	2,579	—	—	2,579	—
Total U.S. Agency for International Development			2,579	—	—	2,579	—
Total Federal Expenditures			\$ 687,568,017	\$ 21,379,159	\$ 342,853,420	\$ 1,051,800,596	\$ 108,989,341

Mount Sinai Health System, Inc.

Notes to Schedule of Expenditures of Federal Awards

Year Ended December 31, 2024

1. Basis of Presentation

The accompanying Schedule of Expenditures of Federal Awards (the Schedule) includes the federal grant activity of Mount Sinai Health System (the System) and is presented on the accrual basis of accounting. The information on the Schedule is presented in accordance with the requirements of Title 2 U.S. *Code of Federal Regulations* Part 200, *Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards* (the Uniform Guidance). For purposes of the Schedule, federal awards include any assistance provided by a federal agency directly or indirectly in the form of grants, contracts, cooperative agreements, loan and loan guarantees, or other non-cash assistance. In accordance with applicable requirements, certain programs may be presented in a fiscal period based on the program-specific guidance. Therefore, some amounts presented in the Schedule may differ from amounts presented in, or used in the preparation of, the consolidated financial statements of the System.

Change in Reporting Entity

As disclosed in the Notes to the Consolidated Financial Statements, the System transitioned to reporting consolidated financial statements of the System and its controlled subsidiaries and affiliates for the year ended December 31, 2024. Prior to the transition to a consolidated financial statement report, separate Schedules of Expenditures of Federal Awards were reported on a standalone basis. Accordingly, the accompanying Schedule is presented on a consolidated basis for the year ended December 31, 2024 and includes information for the following controlled subsidiaries and affiliates for which standalone reports were previously issued: The Mount Sinai Hospital, Icahn School of Medicine at Mount Sinai, Beth Israel Medical Center and Affiliates, South Nassau Communities Hospital and Subsidiaries, The St. Luke's-Roosevelt Hospital Center and Affiliates, and The New York Eye and Ear Infirmary and Affiliates. A full listing of controlled subsidiaries and affiliates comprising the System, by Employer Identification Number and by Unique Entity Identifier, is included with this consolidated report that accompanies the Data Collection Form submitted to the Federal Audit Clearinghouse.

2. Indirect Cost Rate

The Uniform Guidance provides for a 10% de minimis indirect cost rate election; however, the System did not make this election and uses a negotiated indirect cost rate.

Mount Sinai Health System, Inc.

Notes to Schedule of Expenditures of Federal Awards (continued)

3. U.S. Department of Housing and Urban Development Mortgage Insurance: Hospitals Program

The System has a mortgage loan insured under the provisions of the Department of Housing and Urban Development – Federal Housing Administration (FHA) Section 223 Mortgage Insurance Program, pursuant to Section 242 of the National Housing Act. At January 1, 2024 and December 31, 2024, the outstanding balance of the loan totaled approximately \$119.2 million and \$102.4 million, respectively. The Department of Housing and Urban Development has determined that the mortgage insurance program is to be considered a federal award for purposes of compliance with the Uniform Guidance.

As a condition to insuring the FHA 223 Loan, the FHA requires that the System maintain a Mortgage Reserve Fund (MRF) whose balance is fully funded at the agreed-upon two full years' debt service on the FHA 223 Loan. At December 31, 2024, approximately \$24.4 million is on deposit in the fund, which represents the required balance. Pursuant to these borrowings, the System is required to maintain certain financial and nonfinancial covenants. For the year ended December 31, 2024, the System was in compliance with all covenant requirements.

4. Federal Emergency Management Agency: Disaster Grants

The System incurred certain expenditures related to the COVID-19 pandemic which are eligible for reimbursement from the Federal Emergency Management Agency (FEMA) Disaster Grants – Public Assistance (Presidentially Declared Disasters) (Assistance Listing No. 97.036). In 2024, the System recognized approximately \$205.5 million of obligated reimbursement payments from FEMA. Certain costs were incurred prior to January 1, 2024 and approved by FEMA in 2024. These amounts have been included in the accompanying Schedule for the year ended December 31, 2024, in accordance with the guidance specific to Assistance Listing No. 97.036.

5. Loan Programs

The System administers the Federal Perkins Loan Program – Federal Capital Contributions (Assistance Listing No. 84.038). The outstanding loan balance was \$666,475 at January 1, 2024 and there were no loan disbursements for the year ended December 31, 2024. The outstanding loan balance of \$666,475 at January 1, 2024 is considered current-year federal expenditures and is reported in the accompanying Schedule. There is no outstanding loan balance at December 31, 2024 as the System completed the liquidation of the Perkins Loan Program including all end-of-participation procedures in 2024.

Mount Sinai Health System, Inc.

Notes to Schedule of Expenditures of Federal Awards (continued)

5. Loan Programs (continued)

The System participates in the Federal Direct Student Loan Program (Assistance Listing No. 84.268) (which includes the Stafford and GradPlus Loans). During the year ended December 31, 2024, the System processed \$20.3 million in new loans. Since this program is administered by outside financial institutions, new loans made in the year ended December 31, 2024 relating to this program are considered current period federal expenditures, whereas the outstanding balances are not. The new loans made during the year ended December 31, 2024 are reported in the accompanying Schedule.

The System administers the Health Professions Student Loans, Including Primary Care Loans/Loans for Disadvantaged Students (Assistance Listing No. 93.342). There were no loan disbursements for the year ended December 31, 2024 and the outstanding loan balance of \$58,904 at January 1, 2024 is considered current-year federal expenditures. This amount is reported in the accompanying Schedule. The outstanding loan balance at December 31, 2024 is \$43,508.

The System administers the Nursing Student Loans Program (Federal Assistance Listing No. 93.364). The outstanding loan balance of \$172,670 at January 1, 2024 and loan disbursements of \$41,088 for the year ended December 31, 2024 are considered current-year federal expenditures and are reported in the accompanying Schedule. The outstanding loan balance at December 31, 2024 was \$178,889.

6. Food and Nutrition Awards

During the year ended December 31, 2024, the System participated in the New York State Department of Health, Special Supplemental Nutrition Program for Women, Infants, and Children (WIC) through the receipt and distribution of food instruments (Assistance Listing No. 10.557). The U.S. Department of Agriculture has determined that such WIC food instruments are considered “property in lieu of money” and, therefore, should be considered part of the subgrant received by the System.

Mount Sinai Health System, Inc.

Notes to Schedule of Expenditures of Federal Awards (continued)

6. Food and Nutrition Awards (continued)

The total amount reported as federal awards on the accompanying Schedule represents the value of food instruments redeemed in the amount of \$1.4 million, plus administrative costs of \$697,000 for the year ended December 31, 2024. As New York State funds are commingled with federal funds, percentages were applied to determine the total amount of federal funds to be reported on the accompanying Schedule. These percentages were supplied by the New York State Department of Health as follows:

	<u>Federal Percentage</u>
Administrative costs	96%
Food instruments	100%



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Report of Independent Auditors on Internal Control Over Financial Reporting and on Compliance and Other Matters Based on an Audit of Financial Statements Performed in Accordance with *Government Auditing Standards*

Management and the Board of Trustees
Mount Sinai Health System, Inc.

We have audited, in accordance with auditing standards generally accepted in the United States of America and the standards applicable to financial audits contained in *Government Auditing Standards* issued by the Comptroller General of the United States (*Government Auditing Standards*), the consolidated financial statements of the Mount Sinai Health System, Inc. (the System), which comprise the consolidated statement of financial position as of December 31, 2024, and the related consolidated statements of operations, changes in net assets, and cash flows for the year then ended, and the related notes (collectively referred to as the “financial statements”), and have issued our report thereon dated May 30, 2025.

Report on Internal Control Over Financial Reporting

In planning and performing our audit of the financial statements, we considered the System’s internal control over financial reporting (internal control) as a basis for designing audit procedures that are appropriate in the circumstances for the purpose of expressing our opinion on the financial statements, but not for the purpose of expressing an opinion on the effectiveness of the System’s internal control. Accordingly, we do not express an opinion on the effectiveness of the System’s internal control.

A *deficiency in internal control* exists when the design or operation of a control does not allow management or employees, in the normal course of performing their assigned functions, to prevent, or detect and correct misstatements on a timely basis. A *material weakness* is a deficiency, or a combination of deficiencies, in internal control, such that there is a reasonable possibility that a material misstatement of the entity’s financial statements will not be prevented, or detected and corrected on a timely basis. A *significant deficiency* is a deficiency, or a combination of deficiencies, in internal control that is less severe than a material weakness, yet important enough to merit attention by those charged with governance.

Our consideration of internal control was for the limited purpose described in the first paragraph of this section and was not designed to identify all deficiencies in internal control that might be material weaknesses or significant deficiencies and therefore, material weaknesses or significant deficiencies may exist that were not identified. Given these limitations, during our audit we did not identify any deficiencies in internal control that we consider to be material weaknesses. We identified certain



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deficiencies in internal control, described in the accompanying schedule of findings and questioned costs as items 2024-001 and 2024-002, that we consider to be significant deficiencies.

Report on Compliance and Other Matters

As part of obtaining reasonable assurance about whether the System's financial statements are free of material misstatement, we performed tests of its compliance with certain provisions of laws, regulations, contracts and grant agreements, noncompliance with which could have a direct and material effect on the financial statements. However, providing an opinion on compliance with those provisions was not an objective of our audit, and accordingly, we do not express such an opinion. The results of our tests disclosed no instances of noncompliance or other matters that are required to be reported under *Government Auditing Standards*.

The System's Response to Findings

Government Auditing Standards requires the auditor to perform limited procedures on the System's response to the findings identified in our audit and described in the accompanying schedule of findings and questioned costs. The System's response was not subjected to the other auditing procedures applied in the audit of the financial statements and, accordingly, we express no opinion on the response.

Purpose of This Report

The purpose of this report is solely to describe the scope of our testing of internal control and compliance and the results of that testing, and not to provide an opinion on the effectiveness of the entity's internal control or on compliance. This report is an integral part of an audit performed in accordance with *Government Auditing Standards* in considering the entity's internal control and compliance. Accordingly, this communication is not suitable for any other purpose.

Ernst & Young LLP

May 30, 2025



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Report of Independent Auditors on Compliance for Each Major Federal Program and Report on Internal Control Over Compliance Required by the Uniform Guidance

Management and the Board of Trustees
Mount Sinai Health System, Inc.

Report of Independent Auditors on Compliance for Each Major Federal Program

Opinion on Each Major Federal Program

We have audited Mount Sinai Health System, Inc.'s (the System) compliance with the types of compliance requirements identified as subject to audit in the U.S. Office of Management and Budget (OMB) *Compliance Supplement* that could have a direct and material effect on each of the System's major federal programs for the year ended December 31, 2024. The System's major federal programs are identified in the summary of auditor's results section of the accompanying schedule of findings and questioned costs.

In our opinion, the System complied, in all material respects, with the compliance requirements referred to above that could have a direct and material effect on each of its major federal programs for the year ended December 31, 2024.

Basis for Opinion on Each Major Federal Program

We conducted our audit of compliance in accordance with auditing standards generally accepted in the United States of America (GAAS); the standards applicable to financial audits contained in *Government Auditing Standards*, issued by the Comptroller General of the United States (*Government Auditing Standards*); and the audit requirements of Title 2 U.S. *Code of Federal Regulations* Part 200, *Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards* (Uniform Guidance). Our responsibilities under those standards and the Uniform Guidance are further described in the Auditor's Responsibilities for the Audit of Compliance section of our report.

We are required to be independent of the System and to meet our other ethical responsibilities in accordance with the relevant ethical requirements relating to our audit. We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our audit opinion on



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compliance for each major federal program. Our audit does not provide a legal determination of the System's compliance with the compliance requirements referred to above.

Responsibilities of Management for Compliance

Management is responsible for compliance with the requirements referred to above and for the design, implementation, and maintenance of effective internal control over compliance with the requirements of laws, statutes, regulations, rules, and provisions of contracts or grant agreements applicable to the System's federal programs.

Auditor's Responsibilities for the Audit of Compliance

Our objectives are to obtain reasonable assurance about whether material noncompliance with the compliance requirements referred to above occurred, whether due to fraud or error, and express an opinion on the System's compliance based on our audit. Reasonable assurance is a high level of assurance but is not absolute assurance and therefore is not a guarantee that an audit conducted in accordance with GAAS, *Government Auditing Standards*, and the Uniform Guidance will always detect material noncompliance when it exists. The risk of not detecting material noncompliance resulting from fraud is higher than for that resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control. Noncompliance with the compliance requirements referred to above is considered material, if there is a substantial likelihood that, individually or in the aggregate, it would influence the judgment made by a reasonable user of the report on compliance about the System's compliance with the requirements of each major federal program as a whole.

In performing an audit in accordance with GAAS, *Government Auditing Standards*, and the Uniform Guidance, we:

- Exercise professional judgment and maintain professional skepticism throughout the audit.
- Identify and assess the risks of material noncompliance, whether due to fraud or error, and design and perform audit procedures responsive to those risks. Such procedures include examining, on a test basis, evidence regarding the System's compliance with the compliance requirements referred to above and performing such other procedures as we considered necessary in the circumstances.
- Obtain an understanding of the System's internal control over compliance relevant to the audit in order to design audit procedures that are appropriate in the circumstances and to test and report on internal control over compliance in accordance with the Uniform



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Guidance, but not for the purpose of expressing an opinion on the effectiveness of the System's internal control over compliance. Accordingly, no such opinion is expressed.

We are required to communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit and any significant deficiencies and material weaknesses in internal control over compliance that we identified during the audit.

Other Matters

The results of our auditing procedures disclosed instances of noncompliance, which are required to be reported in accordance with the Uniform Guidance and which are described in the accompanying schedule of findings and questioned costs as items 2024-007, related to the Research and Development Cluster for I. Procurement and Suspension and Debarment, and 2024-008, related to the Research and Development Cluster for I. Procurement and Suspension and Debarment. Our opinion on the major federal program is not modified with respect to these matters.

Government Auditing Standards requires the auditor to perform limited procedures on the System's response to the noncompliance finding identified in our audit described in the accompanying schedule of findings and questioned costs. The System's response was not subjected to the other auditing procedures applied in the audit of compliance and, accordingly, we express no opinion on the response.

The System is responsible for preparing a corrective action plan to address each audit finding included in our auditor's report. The System's corrective action plan was not subjected to the auditing procedures applied in the audit of compliance and, accordingly, we express no opinion on it.

We performed audit procedures with respect to the Student Financial Assistance Cluster major federal program related to compliance requirements in Special Tests and Provisions N11, *Gramm-Leach Bliley Act – Student Information Security* as outlined in the *OMB Compliance Supplement* for the year ended December 31, 2024. We determined whether the System has designated a "Qualified Individual," as described in the *OMB Compliance Supplement*, to be responsible for implementing and monitoring the information security program in accordance with 16 CFR 314.4(a). We determined whether the System has a written information security program and that the written information security program documents the seven minimum elements required by 16 CFR 314.4(a)-(g). Our audit procedures did not evaluate whether the designated Qualified Individual is competent to oversee the implementation and monitoring of the information security program nor whether the Qualified Individual possessed the adequate authority to carry out those duties. Our audit procedures also did not determine whether the written information security program



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sufficiently and completely addressed the seven minimum elements required by 16 CFR 314.4(a)-(g). Our opinion on the Student Financial Assistance Cluster major federal program is not modified with respect to this matter.

Report on Internal Control Over Compliance

Our consideration of internal control over compliance was for the limited purpose described in the Auditor's Responsibilities for the Audit of Compliance section above and was not designed to identify all deficiencies in internal control over compliance that might be material weaknesses or significant deficiencies in internal control over compliance, and therefore, material weaknesses or significant deficiencies may exist that were not identified. However, as discussed below, we did identify certain deficiencies in internal control over compliance that we consider to be a material weakness and significant deficiencies.

A deficiency in internal control over compliance exists when the design or operation of a control over compliance does not allow management or employees, in the normal course of performing their assigned functions, to prevent, or detect and correct noncompliance with a type of compliance requirement of a federal program on a timely basis. *A material weakness in internal control over compliance* is a deficiency, or a combination of deficiencies, in internal control over compliance, such that there is a reasonable possibility that material noncompliance with a type of compliance requirement of a federal program will not be prevented, or detected and corrected on a timely basis. We consider the deficiency in internal control over compliance described in the accompanying schedule of findings and questioned costs as item 2024-008 related to the Research and Development Cluster for I. Procurement and Suspension and Debarment to be a material weakness.

A significant deficiency in internal control over compliance is a deficiency, or a combination of deficiencies, in internal control over compliance with a type of compliance requirement of a federal program that is less severe than a material weakness in internal control over compliance, yet important enough to merit attention by those charged with governance. We consider the following deficiencies in internal control over compliance described in the accompanying schedule of findings and questioned costs to be significant deficiencies:

Finding No.	Assistance Listing No.	Program or Cluster Name	Compliance Requirement
2024-003	Various	Student Financial Assistance Cluster	N. Special Tests and Provisions – Verification
2024-004	Various	Student Financial Assistance Cluster	N. Special Tests and Provisions – Enrollment Reporting
2024-005	Various	Student Financial Assistance Cluster	N. Special Tests and Provisions – Disbursement to or on Behalf of Students



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2024-006	Various	Student Financial Assistance Cluster	L. Reporting
2024-007	Various	Research and Development Cluster	I. Procurement and Suspension and Debarment

Our audit was not designed for the purpose of expressing an opinion on the effectiveness of internal control over compliance. Accordingly, no such opinion is expressed.

Government Auditing Standards requires the auditor to perform limited procedures on the System’s response to the internal control over compliance findings identified in our audit described in the accompanying schedule of findings and questioned costs. The System’s response was not subjected to the other auditing procedures applied in the audit of compliance and, accordingly, we express no opinion on the response.

The purpose of this report on internal control over compliance is solely to describe the scope of our testing of internal control over compliance and the results of that testing based on the requirements of the Uniform Guidance. Accordingly, this report is not suitable for any other purpose.

Ernst & Young LLP

September 30, 2025

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs

For the Year Ended December 31, 2024

Section I—Summary of Auditor’s Results

Financial Statements

Type of report the auditor issued on whether the financial statements audited were prepared in accordance with GAAP:

Unmodified

Internal control over financial reporting:

Material weakness(es) identified?

 Yes X No

Significant deficiency(ies) identified?

 X Yes None reported

Noncompliance material to financial statements noted?

 Yes X No

Federal Awards

Internal control over major federal programs:

Material weakness(es) identified?

 X Yes No

Significant deficiency(ies) identified?

 X Yes None reported

Type of auditor’s report issued on compliance for major federal programs:

Unmodified

Any audit findings disclosed that are required to be reported in accordance with 2 CFR 200.516(a)?

 X Yes No

Identification of major federal programs:

Assistance Listing number(s)

Various

Various

Name of federal program or cluster

Student Financial Assistance Cluster

Research and Development Cluster

Dollar threshold used to distinguish between Type A and Type B programs:

\$3,155,401

Auditee qualified as low-risk auditee?

 Yes X No

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section II—Financial Statement Findings

Finding 2024-001 Financial Statement Close Process

Criteria:

Mount Sinai Health System, Inc. (the System) has a responsibility for the design, implementation, and maintenance of internal control relevant to the preparation and fair presentation of the financial statements to provide reasonable assurance against the possibility of misstatements that are material to the financial statements.

Condition:

The System experienced significant delays in its financial statement close process and was not able to produce complete and accurate account reconciliations in a timely manner.

Cause:

On October 7, 2024, the System completed its implementation of an Enterprise Resource Planning (ERP) system. Various system conversion issues and limited refinement of historical close processes resulted in a delayed close process and, along with the change to a consolidated reporting structure, exacerbated issues within the historical accounting processes that were not updated to accommodate the new financial reporting model.

Effect or Potential Effect:

The lack of effective controls in place for the financial statement close process impacted the ability to close the accounting records and produce accurate financial statements in a timely manner.

Recommendation:

We recommend management implement an enterprise-wide accounting function to reduce inconsistencies in accounting processes across the System including consistent account analyses and timely reconciliations, subject to appropriate levels of review. In addition, we recommend that management continue to refine and evaluate the functionality of the accounting modules within the ERP as the post-implementation environment is stabilized.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section II—Financial Statement Findings (continued)

Views of Responsible Officials:

In 2024, Finance implemented three major enhancements which are designed to improve the finance function. These enhancements include the implementation of EPIC to modernize and standardize patient billings and collections, the implementation of Oracle to modernize financial reporting, accounts payable, supply chain management and other areas, and the transition to consolidated financial statements to improve external financial reporting.

Management has established an implementation team to enhance the financial statement close process and the financial reporting process. A major focus of this team will be implementing Oracle's Financial Consolidation and Close Module which will automate the consolidation process. In addition, the implementation team will focus on standardizing financial reporting across all entities, standardizing the timeliness and manner in which reconciliations are performed, and standardizing the use of the chart of accounts.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section II—Financial Statement Findings (continued)

Finding 2024-002 Financial Reporting

Criteria or Specific Requirement:

The System has a responsibility for the design, implementation, and maintenance of internal control relevant to the preparation and fair presentation of the financial statements to provide reasonable assurance against the possibility of misstatements that are material to the financial statements.

Condition:

The System elected a change in reporting entity from reporting standalone financial statements of its subsidiaries to consolidated financial statements of the System for the year ended *December 31, 2024*. This represented a significant change from past practices and required significant effort by management to compile and present consolidated information across numerous entities for which the reporting function is decentralized.

Effect or Potential Effect:

Management had not established an adequate process to effectuate this significant change in reporting entity which resulted in a significant deficiency in the controls over consolidated financial reporting and the related preparation, review and approval process.

Recommendation:

We recommend that management enhance internal controls to ensure consistency in financial reporting, including accounting policies across the entities. In addition, we recommend that a centralized financial reporting function be implemented with review controls to validate the consistency, completeness and accuracy of the reported amounts within the consolidated financial statements and the accompanying footnote disclosures.

Views of Responsible Officials:

The Oracle implementation was designed to report separate financial statements for each entity. Due to the timing of the decision to prepare consolidated financial statements, a consistent reporting process was not part of the implementation plan. As a result, the 2024 consolidation process was a manual process performed in excel.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section II—Financial Statement Findings (continued)

Finding 2024-002 Financial Reporting (continued)

The implementation team discussed above will work to address these issues. Management is in process of creating a centralized financial reporting function. The centralized function will reduce the inconsistencies noted in the financial statements, reconciliations, other key accounting processes, as well as align external and internal financial reporting.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs

Finding 2024-003 N. Special Tests and Provisions – Verification

Identification of the Federal Program:

Grantor: Department of Education

Program Name: Student Financial Assistance Cluster

Assistance Listing No.: Various

Criteria or Specific Requirement:

Section 200.303 of the Uniform Guidance states the following regarding internal control: “The non-Federal entity must: (a) Establish and maintain effective internal control over the Federal award that provides reasonable assurance that the non-Federal entity is managing the Federal award in compliance with Federal statutes, regulations, and the terms and conditions of the Federal award. These internal controls should be in compliance with guidance in “Standards for Internal Control in the Federal Government” issued by the Comptroller General of the United States or the “Internal Control Integrated Framework” issued by the Committee of Sponsoring Organizations of the Treadway Commission (COSO).”

Title 34, Subtitle B, Chapter VI, Part 668, Subpart E 668.51 through 668.61 describes procedures for verifying applicant information for those applicants selected for verification by the Department of Education (DOE), including requiring each applicant whose Free Application for Federal Student Aid (FAFSA) is selected for verification by the DOE to verify the information specified by the DOE.

Condition:

Mount Sinai Phillips School of Nursing (PSON), did not retain all documentation to evidence the review as required for the verification process as outlined in the Federal Student Aid Handbook. As a result, information required to be verified was not retained or available as evidence that the control was operating effectively.

Cause:

PSON did not have sufficient internal controls to ensure that the required evidence to be obtained and reviewed was retained.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-003 N. Special Tests and Provisions – Verification (continued)

Effect or Potential Effect:

The lack of effective review controls over verification has the potential to result in applicant information reported on the applicant's FAFSA to be incorrect.

Questioned Costs:

None identified.

Context:

The testing performed for this compliance requirement is only required for subsidized loans. A sample of five applicants with subsidized loans during the fiscal year who were selected for verification by the DOE were selected for testing. There was one instance in which insufficient documentation was retained to evidence all verification requirements were met.

This compliance requirement was not applicable to Icahn School of Medicine at Mount Sinai (ISMMS) as none of the loans are subsidized.

Recommendation:

Management should enhance its existing controls to ensure all required documentation to verify a selected student's information is retained for those applicants selected for verification by the DOE.

Views of Responsible Officials:

Management agrees with the finding described above. PSN's Office of Student Financial will implement a process that ensures all required documentation is retained. Employees of the Student Financial Aid Office will be trained and PSN will be in compliance with the requirements in the Federal Student Aid Handbook.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-004 N. Special Tests and Provisions – Enrollment Reporting

Identification of the Federal Program:

Grantor: Department of Education

Program Name: Student Financial Assistance Cluster

Assistance Listing No.: Various

Criteria or Specific Requirement:

Section 200.303 of the Uniform Guidance states the following regarding internal control: “The non-Federal entity must: (a) Establish and maintain effective internal control over the Federal award that provides reasonable assurance that the non-Federal entity is managing the Federal award in compliance with Federal statutes, regulations, and the terms and conditions of the Federal award. These internal controls should be in compliance with guidance in “Standards for Internal Control in the Federal Government” issued by the Comptroller General of the United States or the “Internal Control Integrated Framework”, issued by the Committee of Sponsoring Organizations of the Treadway Commission (COSO).”

Title 34, Subtitle B, Chapter VI, Part 685, Subpart C 685.309 describes that an institution is required to update the status of a student on the National Student Loans Data System (“NSLDS”) website if they graduate, withdraw or have an increase/decrease in attendance level during the year within 60 days of the date the institution becomes aware of the change in enrollment status, and the institution is responsible for accurately reporting all data elements.

Condition:

Management’s review process did not ensure significant data elements were accurately reported to the NSLDS website for certain students whose enrollment status changed during the year and/or did not report the status changes within the required timeframe.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-004 N. Special Tests and Provisions – Enrollment Reporting (continued)

Cause:

For PSON and ISMMS, the completeness of the enrollment reporting roster was not properly reviewed resulting in recently graduated students being excluded from the roster, which resulted in the NSLDS system defaulting the status to withdrawn. Further, an enrollment reporting roster for ISMMS was submitted late.

Effect or Potential Effect:

Lack of internal controls over timely and accurate enrollment reporting resulted in inaccurate enrollment status. Enrollment reporting in a timely and accurate manner is important for effective management of the programs.

Questioned Costs:

None identified

Context:

For PSON, of a sample of 11 students receiving Pell and Direct Student Loans during the fiscal year, 8 were reported to the NSLDS with an incorrect status change.

For ISMMS, of a sample of 14 students receiving Pell and Direct Student Loans during the fiscal year, 3 were reported to the NSLDS with an incorrect status change and one status change was submitted beyond the 60 day requirement.

For each of the instances of incorrect status changes reported to NSLDS, the system defaulted the status of the student as withdrawn as opposed to graduated. It was noted that all of the students completed 100% of the course work and, therefore, no Title IV funds were required to be returned for these students.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-004 N. Special Tests and Provisions – Enrollment Reporting (continued)

Recommendation:

Management should enhance its existing controls to ensure all status changes reported to the NSLDS accurately reflect the correct status of the student, and that any changes are submitted within the 60 day requirement.

Views of Responsible Officials:

Management agrees with the finding described above. PSON and ISSMS' Offices of Student Financial will ensure that all NSLDS submissions are made timely and with the correct status of each student.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-005 N. Special Tests and Provisions – Disbursement to or on Behalf of Students

Identification of the Federal Program:

Grantor: Department of Education

Program Name: Student Financial Assistance Cluster

Assistance Listing No.: Various

Criteria or Specific Requirement:

Section 200.303 of the Uniform Guidance states the following regarding internal control: “The non-Federal entity must: (a) Establish and maintain effective internal control over the Federal award that provides reasonable assurance that the non-Federal entity is managing the Federal award in compliance with Federal statutes, regulations, and the terms and conditions of the Federal award. These internal controls should be in compliance with guidance in “Standards for Internal Control in the Federal Government” issued by the Comptroller General of the United States or the “Internal Control Integrated Framework”, issued by the Committee of Sponsoring Organizations of the Treadway Commission (COSO).”

Title 34, Subtitle B, Chapter VI, Part 668, Subpart K 668.164(h) describes that when a student who received Title IV funds has total loans exceeding the amount of tuition and fees, food and housing, and other authorized charges assessed the student, a credit balance is created. The institution must pay the resulting credit balance directly to the student or parent borrower within 14 days after (1) the first day of class of a payment period if the credit balance occurred on or before that day, or (2) the balance occurred if that was after the first day of class.

Condition:

Management has a review process in place to process and issue refunds to students for credit balances. However, this process did not ensure refunds were issued within the required timeframe. As a result, there were instances of refunds issued beyond the 14 day requirement.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-005 N. Special Tests and Provisions – Disbursement to or on Behalf of Students (continued)

Cause:

The internal controls in place for the issuance of credit refunds to students did not ensure refunds were issued to students within the required timeframe of 14 days after (1) the first day of class of a payment period if the credit balance occurred on or before that day, or (2) the balance occurred if that was after the first day of class.

Effect or Potential Effect:

The lack of effective controls over the credit refund process has the potential to result in ISMMS and PSON not returning funds owed to the student within the required timeframe.

Questioned Costs:

None identified

Context:

For ISMMS, of a sample of 40 credit refunds issued during the fiscal year, there were 12 instances of refunds issued beyond the 14 day requirement. ISMMS issued 798 refunds during the year totaling \$7,503,258.

For PSON, of a sample of 40 credit refunds issued during the fiscal year, there were 9 instances of refunds issued beyond the 14 day requirement. PSON issued 395 refunds during the year totaling \$2,139,327.

Recommendation:

ISMMS and PSON should enhance controls to ensure all credit balances are refunded to students within 14 days of the creation of the credit balance.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-005 N. Special Tests and Provisions – Disbursement to or on Behalf of Students (continued)

Views of Responsible Officials:

Management agrees with the finding that certain credit refunds were not issued timely. PSON and ISMMS' Offices of Student Financial will implement a control that ensures communication related to refunds is enhanced when a student withdraws. The control will ensure that all credit refunds are issued timely.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-006 L. Reporting

Identification of the Federal Program:

Grantor: Department of Education

Program Name: Student Financial Assistance Cluster

Assistance Listing No.: Various

Criteria or Specific Requirement:

Section 200.303 of the Uniform Guidance states the following regarding internal control: “The non-Federal entity must: (a) Establish and maintain effective internal control over the Federal award that provides reasonable assurance that the non-Federal entity is managing the Federal award in compliance with Federal statutes, regulations, and the terms and conditions of the Federal award. These internal controls should be in compliance with guidance in “Standards for Internal Control in the Federal Government” issued by the Comptroller General of the United States or the “Internal Control Integrated Framework”, issued by the Committee of Sponsoring Organizations of the Treadway Commission (COSO).”

Title 34, Subtitle B, Chapter VI, Part 674, Subpart A 674.19(h) describes that each year an institution shall submit a Fiscal Operations Report and Application to Participate (FISAP) in addition to other information the DOE requires. The institution shall ensure that the information reported is accurate and shall submit it on the form and at the time specified by the DOE.

Condition:

Management has a review process in place to ensure the FISAP is completed and submitted within the required deadline. However, for PSON, this review process did not ensure the information within the FISAP was accurate. As a result, the FISAP for the report year ended *June 30, 2024* was submitted with inaccurate statistics for one section.

Cause:

Due to a lack of precision of the review control, certain differences between the FISAP and the underlying schedules were not identified and corrected.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-006 L. Reporting (continued)

Effect or Potential Effect:

The lack of effective controls over the FISAP preparation and review and approval process has the potential to result in inaccuracies in the submission which could result in reductions of fund allocations.

Questioned Costs:

None identified

Context:

For the report year ended June 30, 2024, the FISAP for PSON was submitted by the organization with certain inaccurate student statistics. A corrected FISAP was subsequently submitted during the audit process. It was noted that, although certain categories of students were corrected in the resubmission, the total number of students and tuition received was accurate in the original submission.

No such inaccuracies were noted in the FISAP for report year ended June 30, 2024 for ISMMS.

Recommendation:

Management should enhance its review controls to ensure the FISAP is correctly populated prior to submission.

Views of Responsible Officials:

Management agrees with the finding that FISAP was not correctly populated. Management has since corrected the data and submitted a revised FISAP. Management notes there was turnover in the PSON's Office of Student Financial Aid during the year and an employee was not properly trained on the FISAP preparation. Training has since been implemented and new employees in the department will be trained accordingly.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-007 I. Procurement and Suspension and Debarment

Identification of the Federal Program:

Grantor: Department of Agriculture, Department of Defense, Department of Labor, National Aeronautics and Space Administration, Promotion of the Arts Grants to Organizations and Individuals, National Science Foundation, Department of Veteran Affairs, Environment Protection Agency, Department of Energy, Department of Health and Human Services, U.S. Agency for International Development

Program Name: Research and Development Cluster

Assistance Listing No.: Various

Criteria or Specific Requirement:

Section 200.303 of the Uniform Guidance states the following regarding internal control: “The non-Federal entity must: (a) Establish and maintain effective internal control over the Federal award that provides reasonable assurance that the non-Federal entity is managing the Federal award in compliance with Federal statutes, regulations, and the terms and conditions of the Federal award. These internal controls should be in compliance with guidance in “Standards for Internal Control in the Federal Government” issued by the Comptroller General of the United States or the “Internal Control Integrated Framework”, issued by the Committee of Sponsoring Organizations of the Treadway Commission (COSO).”

Title 2, Subtitle A, Chapter II, Part 200, Subpart D 200.320 – Procurement methods – outlines the acceptable methods of procurement. Purchases below the simple acquisition threshold, but above the micro-purchase threshold, require price and rate quotations to be obtained from an adequate number of qualified sources.

Condition:

The System did not follow its purchasing policy requirements for certain purchases during the period October 7, 2024 through December 31, 2024, which required competitive bids or documentation of sole source justification.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-007 I. Procurement and Suspension and Debarment (continued)

Cause:

Due to the change in the ERP system on October 7, 2024, the previous process that directed purchase orders made by certain designated individuals to the purchasing department for verification of appropriate procurement procedures for purchases exceeding the micro-purchase threshold was no longer in effect. This resulted in the System not completing applicable procurement requirements prior to entering into a transaction for these purchases.

Effect or Potential Effect:

The lack of effective controls in place to ensure the proper purchasing policies are followed has the potential to result in noncompliance with the requirements.

Questioned Costs:

Known questioned costs of \$44,178. Based on the analysis provided by management of all purchases made by certain designated individuals above the micro-purchase threshold from October 7, 2024 to December 31, 2024, the likely questioned costs are \$939,712.

Context:

Of a sample of 60 purchase orders within the Research and Development Cluster above the micro-purchase threshold during the fiscal year, there were 3 instances in which required procurement procedures were not appropriately completed.

Recommendation:

Management should enhance the process for reviewing purchases made using federal funds to ensure the appropriate procurement requirements are completed prior to entering into a transaction.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-007 I. Procurement and Suspension and Debarment (continued)

Views of Responsible Officials:

Management agrees with the finding described above. In October 2024, Mount Sinai transitioned to Oracle Cloud for its Enterprise Resource Planning (ERP) and general ledger system. During this transition, Deputy Buyer transactions of \$10,000 and above were not routed to the Purchasing Team for review, allowing purchase orders to be completed without the required documentation, including Sole Source Justifications, Directed Source Justifications, or multiple quotes as required under Uniform Guidance.

Corrective actions, including a system enhancement, have been implemented to remediate this issue. Mount Sinai's enhanced process ensures Deputy Buyer transactions of \$10,000 and above are routed appropriately to the Purchasing Team for validation and documentation review. In addition, a quarterly manual review process, supported by reports developed in partnership with Mount Sinai's technology team, will be fully implemented as a compensating control in the fourth quarter of 2025.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-008 I. Procurement and Suspension and Debarment

Identification of the Federal Program:

Grantor: Department of Agriculture, Department of Defense, Department of Labor, National Aeronautics and Space Administration, Promotion of the Arts Grants to Organizations and Individuals, National Science Foundation, Department of Veteran Affairs, Environment Protection Agency, Department of Energy, Department of Health and Human Services, U.S. Agency for International Development

Program Name: Research and Development Cluster

Assistance Listing No.: Various

Criteria or Specific Requirement:

Section 200.303 of the Uniform Guidance states the following regarding internal control: “The non-Federal entity must: (a) Establish and maintain effective internal control over the Federal award that provides reasonable assurance that the non-Federal entity is managing the Federal award in compliance with Federal statutes, regulations, and the terms and conditions of the Federal award. These internal controls should be in compliance with guidance in “Standards for Internal Control in the Federal Government” issued by the Comptroller General of the United States or the “Internal Control Integrated Framework”, issued by the Committee of Sponsoring Organizations of the Treadway Commission (COSO).”

Title 2, Subtitle A, Chapter II, Part 200, Subpart C 200.214 – Suspension and debarment – Non-Federal entities are subject to the non-procurement debarment and suspension regulations that restrict awards, subawards, and contracts with certain parties that are debarred, suspended, or otherwise excluded from or ineligible for participation in Federal assistance programs or activities.

Condition:

Management was unable to provide evidence of a control being consistently performed throughout the audit period to address the risk that the System may enter into a transaction with an entity that is suspended, debarred, or otherwise excluded.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-008 I. Procurement and Suspension and Debarment (continued)

We noted the following matters during our testing of suspension and debarment:

- (a) The System did not submit all vendors to the third-party service provider for on-going monitoring for the months of January 2024 through September 2024.
- (b) The System did not obtain a detailed listing of all vendors screened with related results from the third-party service provider. The System receives a summarized results report with a total number of vendors screened for suspension and debarment by the third-party service provider. However, a reconciliation was not performed and retained between the vendors provided by the System and the service provider's summarized results report to evidence that all vendors submitted were screened.
- (c) The System did not retain evidence of review of the results provided by the third-party service provider, including evidence that unverified vendors or those that were indicated as excluded were investigated and evaluated by the System.
- (d) The System did not retain evidence of review of new vendors being screened for suspension and debarment prior to entering into a transaction.

Cause:

The System did not have a process in place to review the results of screened vendors to verify that all vendors are screened.

Effect or Potential Effect:

The lack of effective controls over suspension and debarment has the potential to result in noncompliance with the requirement. By not performing a reconciliation between the listings submitted by the System and the results provided by the third-party service provider, there is a risk that vendors are not screened by the third-party service provider and such vendors used for federally funded activities may be suspended or debarred.

Questioned Costs:

Known questioned costs: \$56,000.

Mount Sinai Health System, Inc.

Schedule of Findings and Questioned Costs (continued)

Section III—Federal Award Findings and Questioned Costs (continued)

Finding 2024-008 I. Procurement and Suspension and Debarment (continued)

Context:

Management was unable to provide evidence of a control being consistently performed throughout the audit period, as described in the Condition section above. Further, of a sample of 60 current year expenditure transactions within the Research and Development Cluster, there was 1 instance in which no evidence could be provided to validate that the vendor was neither suspended nor debarred prior to entering into the transaction.

Recommendation:

Management should enhance the design of the System's controls related to the screening of new vendors prior to entering into transactions, as well as the on-going monitoring and screening of its vendors for suspension and debarment. In addition, management should perform and retain documentation to support reconciliation of the vendor list submitted to the third-party service provider and the results obtained to support that the screening process is completed for all vendors and that no suspended or debarred vendors are utilized by the System prior to entering into transactions.

Views of Responsible Officials:

Management agrees with the finding described above. Due to a system failure, vendor payment files from January through September 2024 were not transmitted to Mount Sinai's sanction screening vendor.

Mount Sinai has taken corrective actions to address this issue. All current vendors in our vendor master file, as well as all new vendors added through our new vendor credentialing process, are now processed through our sanction screening vendor on a monthly basis. In addition, a new manual control has been implemented to review, confirm, and reconcile to ensure that the vendor master file has been transmitted successfully, and all vendors are screened for sanctions, and a report thereof is provided each month. Our process will also document and maintain evidence that unverified vendors or those that were indicated as excluded were investigated and evaluated by the Health System. We will also periodically perform independent sanction screening checks on a sample of our vendors to validate the accuracy of the results of our third-party sanction screening vendor.